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Psychosocial support for people living with hiv

The design of a handbook for managing and mitigating the impact of an hiv diagnosis

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Psykosocialt stöd för personer som lever med hiv

Utvecklingen av en handbok för att hantera och avdramatisera en hiv-diagnos

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Sammanfattning

Denna rapport beskriver ett examensarbete på masternivå i Teknisk Design vid Kungliga Tekniska Högskolan i Stockholm, utfört för Venhälsan på Södersjukhuset i samarbete med Sös Innovation. Syftet med projektet var att bidra till att öka det psykosociala välmåendet hos personer som lever med hiv, och som följs av Infektionsmottagning 2 på Södersjukhuset i Stockholm.

Projektet följde en tjänstedesignmetodik, där aktörer involverades i alla stadier med hjälp av olika människocentrerade metoder. En litteraturstudie inledde arbetet, där tidigare forskning om hiv och stigma undersöktes. Det följdes av en förstudie som involverade forskare, patientorganisationer och arbetare på Venhälsan för att skapa en förståelse för vårdkontexten, och det psykosociala stödet som erbjuds av olika aktörer idag. Huvudstudien involverade personer som lever med hiv och bestod av en serie djupintervjuer, som efter en mängd analysmetoder ledde till insikter som låg till grund för det efterföljande idégenereringsarbetet. Idégenereringen bestod av en workshop med personal på Venhälsan, brainstorming och feedbacksessioner med personer från patientgruppen, där triggermaterial användes.

Projektet resulterade i framtagningen av en Handbok i hiv som patienten får i samband med deras hiv-diagnos. Handboken innehåller sex olika teman: Om hiv, Venhälsan, Kunskap, Igenkänning, Berättande och Acceptans, och ämnar att behandla och avdramatisera dessa ämnen för patienten, och följaktligen hjälpa en patient i en krissituation. Utöver handboken sammanställdes alla idéer och insikter från projektet och levererades till Venhälsan för eventuellt framtida arbete.



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Abstract

This report describes a masters' thesis in Industrial Design Engineering at the Royal Institute of Technology in Stockholm, conducted for Venhälsan at Södersjukhuset in Stockholm, in collaboration with Sös Innovation. The purpose of the project was to contribute to improving the psychosocial well-being of people living with hiv, who are monitored by the Infektionsmottagning 2 at Södersjukhuset.

The project followed a service design methodology, involving stakeholders at all stages using various human-centered methods. The work began with a literature study, examining previous research on hiv and stigma. This was followed by a pre-study involving researchers, patient organizations, and staff at Venhälsan to create an understanding of the healthcare context and the psychosocial support offered by different actors today. The main study involved people living with hiv and consisted of a series of in-depth interviews, which, after a variety of analysis methods, led to insights that formed the basis for the subsequent concept generation. The concept generation involved a workshop with Venhälsan staff, brainstorming, and feedback sessions with members of the patient group, using trigger materials.

The project resulted in the development of an Hiv Handbook that patients receive in connection with their hiv diagnosis. The handbook contains six different themes: About hiv, Venhälsan, Knowledge, Recognition, Disclosure, and Acceptance, and aims to address and mitigate these topics for the patient, thereby helping a patient in a crisis situation. In addition to the handbook, all ideas and insights from the project were compiled and delivered to Venhälsan for potential future work.

Acknowledgements

We are absolutely humbled by the amount of contribution, feedback and support that we have received throughout this project. There are many to thank.

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Furthermore, we would like to thank all the wonderful people at Venhälsan for your warm welcome, your contribution with recruiting respondents and your input during the workshop. Thank you to our contact persons Ida Östlund and Emy Åstrand for all the feedback sessions and help during the project. We were always happy to talk to you.

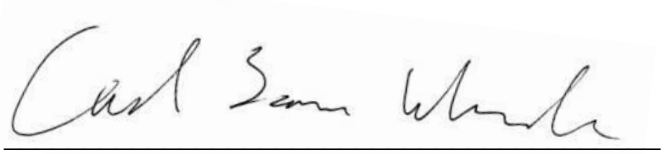
Thank you to all respondents from Sös, Karolinska Institutet, Karolinska Sjukhuset Huddinge, Positiva Gruppen and Noaks Ark for your contribution and insights.

Lastly, thank you to all the respondents living with hiv. Without your time and effort, this project would have never been realized.

Sanna Ericsson



Carl Jonas Wiklander



Stockholm, June 2024.

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1. Introduction

This section will elaborate on the background, the purpose, the delimitations and the research questions that were discussed in this project, as well as the ethical considerations that were taken.

1.1. Background

In Sweden today, there are roughly 8000 people diagnosed with hiv, a virus that weakens the immune system. During 2022, around 450 new cases of hiv were reported in Sweden (Folkhälsomyndigheten 2022). Infektionsmottagning 2 at Venhälsan at Södersjukhuset in Stockholm follows and treats nearly 1600 of the 8000 people living with hiv. Even though 96% of people living with hiv in Sweden are on antiretroviral therapy, meaning the virus is not transmittable, and the medical complications are close to none, there is still stigma, bad attitudes, lacking knowledge and discrimination towards them. This results in negative consequences for their physical, social and mental well-being (Kanon and Lindberg 2022). Further, Venhälsan also reports that many living with hiv experience anxiety regarding different interactions, bad self-esteem, and loneliness (Venhälsan n.d). Even so, few of the patients utilize the counselors at Venhälsan. Folkhälsomyndigheten (2016) also reports that only 15% of people living with hiv have contact with patient organizations or hiv-organizations even though research suggests that peer-to-peer support is beneficial for reducing internalized stigma.

In 2013, the world's leading organization in combating hiv, UNAIDS, set a global goal called 90-90-90. The target was to by 2020 make 90% of people living with hiv aware of their hiv status, to make 90% of all people diagnosed with an hiv infection receive antiretroviral therapy, and 90%f of all people receiving antiretroviral therapy having viral suppression (UNAIDS 2014). In 2020, UNAIDS updated the goal to by 2030 instead have corresponding levels of 95-95-95 (UNAIDS, 2020). Sweden was the first country to achieve these goals, and now has a corresponding level of 90-98-94. To further increase the well-being of people living with hiv in Sweden, a fourth 90-goal has been discussed by many authorities in Sweden to also include the experienced quality of life of people living with hiv (RFSU 2021).

Through working with the quality of life for people living with hiv in Sweden, this project would contribute to the third UN Sustainable Development Goal, *Good Health and Well-being* (UNAIDS 2024). Although SDG Indicator 3.3.1 directly addresses hiv, with the aim to end the aids-epidemic world-wide by 2030, the focus of this project would be on psychosocial health, as that is the main problem for people living with hiv in Sweden (WHO 2024). The word 'psychosocial' is used to describe the interrelation between people's social environment and the individuals' thoughts and behaviors (Psykologiguiden 2024).

This project originated from an initiative by Venhälsan, in collaboration with Sös Innovation, Posithiva Gruppen and Noaks Ark, to work towards creating action plans to increase the

long-term psychosocial support for people living with hiv, who are treated at Venhälsan. Their solution was to offer group sessions for people living with hiv, as a way to offer more peer-to-peer support. For this project, the scope was re-framed to focus more on the underlying structures and needs to contribute to increasing the psychosocial well-being for people living with hiv. The work was done by the two master´s students, together with and aided by Service Designers at Sös Innovation, on behalf of Venhälsan and Infektionsmottagning 2 at Södersjukhuset in Stockholm.

1.2. Purpose and research questions

The overall aim of this thesis was to:

Contribute to increasing the psychosocial well-being of people living with hiv.

To approach this, it was important to first examine how Venhälsan today tries to facilitate a better situation for their patients in their everyday life. Therefore, the first research question of this thesis was phrased:

RQ1: How does Venhälan provide psychosocial support for their patients?

As with all offerings, understanding the impact of the support provided was impossible without also investigating how it was received and experienced by the target group. This lead to the second research question:

RQ2: How do people living with hiv experience existing psychosocial support and what are they lacking?

When receiving an hiv diagnosis, the patient needs to prepare for living with it for the rest of their life. It was therefore important to review the first period after the diagnosis, to see what needs the patients have, and how Venhälsan can modify their work to further support the patients during this time. The last research question was therefore phrased:

RQ3: How might Venhälsan modify their work with supporting patients during the period after a diagnosis?

1.3. Delimitations

The thesis will only consider and act in a Swedish context. Since the project was first brought to life by counselors at Venhälsan, it revolves around the context of Venhälsan. Venhälsan specifies in care for men who have sex with men (MSM), which does not correspond to the patient group nationwide which is more heterogeneous. It is therefore important to see this project as heavily influenced by the target group of Venhälsan.

1.4. Ethical considerations

Usually, before a study of people is done, an ethical review is required to ensure the protection of the people involved in the study. For this thesis, an ethical review was not deemed necessary since the project was not a research project, but rather about increasing the function of Venhälsan. It was therefore classified as “business development”, which does not need a formal ethical review.

Nonetheless, ethical considerations had to be taken into account during the project. People living with hiv already witness of stigma and alienation, which the project should not contribute to. To avoid that, the approach of the project, the interview content and general attitudes towards the patient groups were always discussed with counselors at Venhälsan. It was also important that the information obtained was up-to-date and that the project group was respectful and willing to change their ways and perceptions.

2. Background research

The following chapter presents the background research for the thesis. A comprehensive study of hiv was conducted: how the virus works and the treatment, how aids is developed, the historical and contemporary context for hiv and what test and obligations surround hiv today. This information was gathered through books, and through using search words such as: *hiv, psychosocial, stigma, review* on websites such as Google Scholar, PubMed and Researchgate. Additionally, the chapter presents Venhälsan and its history. Finally, the chapter explores the different kinds of stigma: anticipated, enacted and internalized, while also presenting the intersectional perspective and stigma reducing interventions.

2.1. Hiv - what is it?

Hiv (which stands for human immunodeficiency virus) is a retrovirus that weakens the immune system by infecting white blood cells, CD4 cells or helper cells, in the blood. It is stored in the genome and the person infected by it can never be cleared of it (Folkhälsomyndigheten 2024). An hiv infection today is treatable through antiretroviral therapy. If the person infected has an effective treatment, the virus levels are suppressed to a point where the virus can no longer be detected or be transmitted to another person.

During the aids conference in Amsterdam in 2018, the so-called “Partner II”-study was presented (RFSL 2020). The Partner II study involved 783 homosexual male couples from 14 different European countries, where one of the partners lived with a well-treated hiv infection, and the other did not. The study involved around 77 000 unprotected, condomless anal intercourses, and did not report of a single transmission of the virus. This was a breakthrough in changing the understanding of what living with hiv means today and sprouted the, now well-used, slogan “U=U, undetectable = untransmittable” (UNAIDS 2024).

If hiv is transmitted, it is done so through body fluids such as blood, semen, vaginal secretions and breast milk, but it does not last for long outside the body, meaning it can not be transferred for example by sharing a toilet, a drinking glass, shaking hands or kissing (Folkhälsomyndigheten 2024). The infection is often asymptomatic, but during the first few weeks the infected person might get a fever, a sore throat, swollen or tender lymph nodes, muscle or headache, a rash, or diarrhea. After the initial symptoms, there might be several years before more serious symptoms appear, such as shingles, lymph node cancer, tuberculosis, fungus in the mouth or other infections, which is caused by the weakened immune system (1177 2023).

2.1.1. Aids

Untreated, hiv will lead to aids (acquired immune deficiency syndrome), a state or condition that covers all serious consequences of the weakened immune system, for example pneumonia and tumors (1177 2023).

Other symptoms of aids include “minor mucocutaneous manifestations and recurrent upper respiratory tract infections, unexplained chronic diarrhea for longer than a month, severe bacterial infections and pulmonary tuberculosis or toxoplasmosis of the brain, candidiasis of the oesophagus, trachea, bronchi or lungs and Kaposi’s sarcoma.” (UNAIDS n.d.). As shown in Figure 1 , an untreated hiv-infection can show itself in many forms, beginning with minor symptoms that increase in severity the longer the infection lasts.

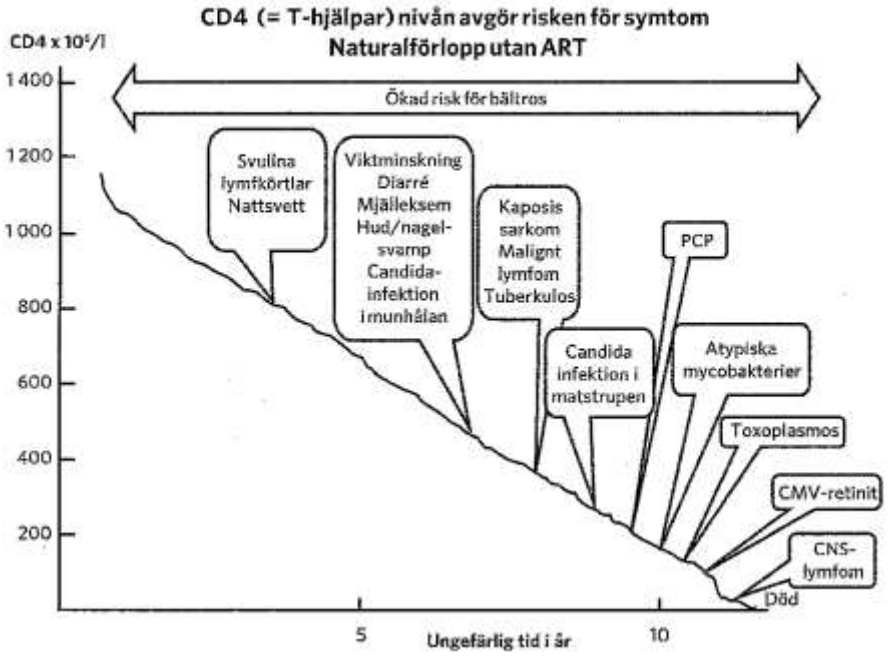


Figure 1. Graph of common symptoms as the CD4-cell count decreases (Kanon and Lindeberg 2022).

It differs from person to person, but untreated, most people will develop aids within 8-10 years after an hiv infection. According to United States Centers for Disease Control and Prevention, a person living with hiv has developed aids if the CD4-positive T-cell count is under 200 per mm^3 of blood (UNAIDS n.d.). When the infection has developed into aids, the infection is more contagious. The risk of infecting someone else with hiv is therefore larger the longer the infection has gone untreated (Folkhälsomyndigheten 2024).

2.1.2. Tests and obligations

The virus for hiv is tested by a routine laboratory-based combination blood test that is sent to a laboratory. This test can detect levels of the virus already after two to six weeks after a person has been exposed to hiv. Nevertheless, it is recommended to take another test if the first test was negative and taken before 6 weeks after exposure. Some places also offer rapid response tests that give an answer directly, but these tests have lower accuracy, as there is a period where the tests can not prove any infection, which means it is recommended to take a new one after 8 weeks (Folkhälsomyndigheten 2023). All tests are free and the person

wanting to be tested does not have to disclose why they want to be tested. They can also choose to be anonymous until the test is positive.

As hiv is classified as a public health disease in Sweden, it falls under the infection protection act, or "Smittskyddslagen" (Riksdagen 2022). This means that a positive hiv-test makes the patient obliged to help the healthcare co-workers with infection tracking, to map out where they might have been exposed to the virus and who they might have exposed themselves. The mapping is done by healthcare personnel with a certain education. The patient is also obliqued to have regular contact with the healthcare system, and are offered meetings with counselors for psychosocial support. Additionally, in short, they are also obliged to minimize the risk of infecting other people through using active protection (such as condoms) during sex, and also informing the partner about their hiv status. If they follow their treatment plan and provide two consequent negative hiv-tests, the doctor responsible for that person can remove the obligations to disclose their hiv status and do not need to use a condom during sex (Folkhälsomyndigheten 2023).

2.1.3. Historical context

Hiv was first discovered in the 1980s in the USA and after that an epidemic followed. Since many of the infected belonged to marginalized groups, for example migrants, homosexual men, people actively injecting drugs, and people who have sex for compensation, a stigma arose around hiv (Kanon and Lindberg 2022). Figure 2 shows an overview of the timeline since the discovery of hiv.



Figure 2. Timeline from the discovery of hiv until effective treatment.

The first person diagnosed with aids in Sweden was during autumn of 1982 (Halldin 1994). The media coverage was minimal, as hiv and aids was seen as something that did not concern

the public, only minorities, and majority of the first cases belonged to marginalized groups such as bi- and homosexuals, people who were injecting drugs, and people who sold sex for compensation. The non-profit hbtqi-organization RFSL received 25 000 SEK the first year to deal with the question and give the public information.

As more and more people got diagnosed with aids and the media coverage grew, the fear increased. Since it was known that aids was first discovered in homosexual men it was referred to as "gay-cancer" or "gay-plauge" in everyday speech. To control the outbreak, doctors and debaters suggested that infected people would tattoo disclosure that they were infected on their body, or be put in isolated camps (Forum för levande historia n.d.).

Hiv was discovered as the cause for aids in 1984, as well as blood and sperm as the main route of transmission (Gallo et.al 2003). In 1985, the Swedish government decided to include hiv and aids in the infection protection act. In short, it meant that infected people needed to disclose all sexual partners to doctors and follow all their rules, go to regular check ups, and always disclose their infection to healthcare co-workers. If they failed to follow this, they could be put into isolation or jail indefinitely without a trial.

Following this decision, the amount of men who went and got tested for hiv or aids declined, as RFSL had predicted. RFSL advocated for safer sex and regular check-ups instead of no sex, which the government wanted. Following "Bastuklubbslagen" in 1987, where all public establishments where men could have sex with men were forbidden, there was a harsh debate between RFSL and Aidsdelegationen who reported directly under the government. The government wanted to confine the infection to minorities, and thus created the strictest aids legislation in the world, together with South Africa. Other European countries focused more on better information and trust instead of prohibition, and thus had a smaller outbreak.

Apart from the obvious results with sick and dead people, the epidemic also resulted in other negative consequences, mainly for minorities such as hbtqi. The movement for hbtqi rights were halted and homosexual men were viewed as a risk. The movement for a more free sexuality was also halted, and society started to view a person that had sex with many different persons in a negative way. A new form of radicalism started, for example Act Up that acted both in the USA and in Sweden. Literature and art tried to illuminate hiv to the public, but relatively few pieces were released in Sweden. However, one example was Jonas Gardells' trilogy *Torka aldrig tårar utan handskar,* that was released in 2010 and presented a rather dark reality for an hiv-infected in the 1980s.

In 1996, effective treatment was discovered, hindering people living with hiv to develop aids. This greatly reduced the amount of people needing healthcare for aids-symptoms to practically zero within a few years, and hiv was no longer a death sentence (Forum för levande historia n.d.).

2.1.4. Swedish context today

The Swedish healthcare system uses the quality register InfCareHiv to keep track of statistics of hiv tracking and treatment in Sweden (InfCareHiv 2024). It involves all hiv clinics in Sweden, and tracks 99% of all people diagnosed with hiv.

As mentioned in the introduction, there are over 8000 registered people living with hiv in Sweden. The majority live in Sweden's larger cities, Stockholm, Göteborg and Malmö. See Figure 3 for the distribution of patients across different regions.

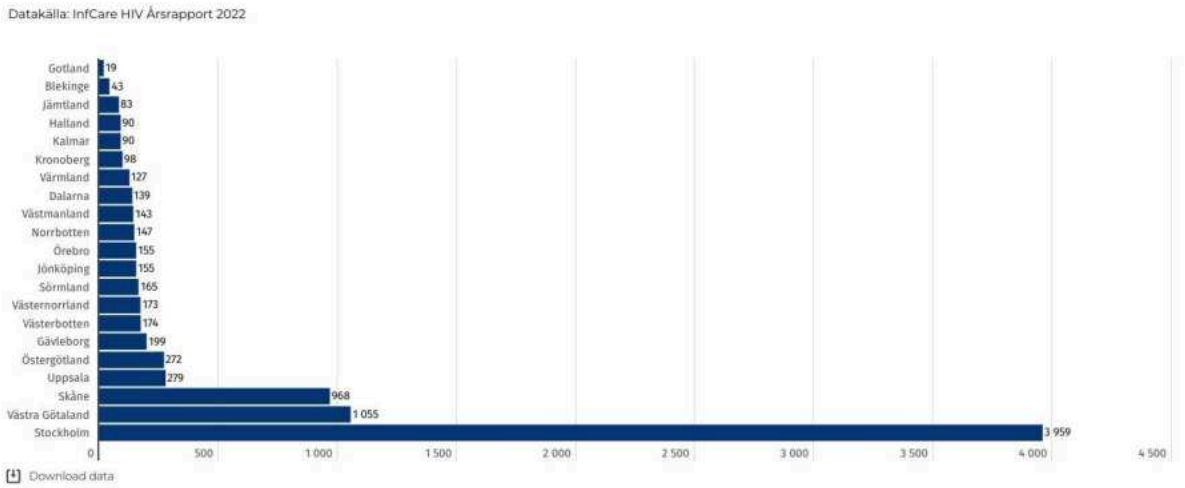


Figure 3. Distribution of patients across different regions of Sweden (InfCareHiv 2022).

The demographic of people living with hiv in Sweden is very heterogeneous. It is quite different from the context of hiv in the early stages of the disease's history. 51% of newly infected are actually transmitted by heterosexual relationships, with the majority being transmitted from male to female. 32% of the sexual transmissions are between men who have sex with men, and 4% of total transmissions are caused by the sharing of needles during the injection of drugs (InfCareHiv, 2022).

The majority of the patient group are between 40-59 years old (4734 people). 33% of newly reported infections are among people born in Sweden, and the rest are migrants, the largest group of 36% coming from often high-endemic Sub-Saharan African countries.

As stated in the introduction, the well-being of people living with hiv in Sweden is relatively high in comparison to the rest of the world, and was the first country to reach the 90-90-90 goals set by UNAIDS. According to the study *To live with hiv* by Folkhälsomyndigheten (2016), the people living with hiv in Sweden generally view their quality of life as high. Although this is a part of the new 95-95-95-95 goals, there are no official statistics of the level of well-being among people living with hiv in Sweden.

2.1.5. Treatment

Today, hiv is treatable but not curable. As stated previously, a well-treated infection reduces the virus levels until they are no longer detectable, which leads to the infection not being transmittable, once again U=U. A well-treated infection does not result in any somatic symptoms.

The antiretroviral therapy today consists of taking daily pills, often 1-2. These pills are often combinations of six main drug substance groups with different purposes:

1. Fusion prohibitors which prohibits the fusion of the virus and the cell.
2. Entry inhibitors prevent the HIV virus from entering the cell by blocking receptors on the cell surface.
3. Nucleoside analogues block the enzyme reverse transcriptase and prevent RNA from being converted into DNA.
4. NNRTIs block or alter the reverse transcription process and prevent the formation of new DNA.
5. Integrase inhibitors prevent the virus's genetic material from being integrated into human genetic material. In this way, no new virus particles can be formed.
6. Protease inhibitors block the enzyme protease and prevent the formation of new virus particles (Folkhälsomyndigheten 2023).

It is common for people to experience side effects during the initial stages of their treatment (often for the first three months), but these are often mild and transient, for example head- and stomach ache, nausea and insomnia. These reactions are very common, but can be stronger for some, and for specific substances (Venhälsan n.d).

Helena Berggren, infection doctor at Akademiska Sjukhuset in Uppsala explains that the medication and treatment always is about balancing effectiveness, side effects and cost. There is a wide array of accessible medication, in different combinations from many suppliers. This enables doctors to tailor the medicine to the reaction and needs of each patient. Should any specific medication cause side effects for a patient, there are many other suppliers and substances to be tried instead.

Berggren also points out that compliance with the treatment is important, that the medicine should be taken at the same time every day, to avoid the virus getting resistant towards the medication. She explains however, that if a patient forgets their medicine once, it will not affect their treatment. The medication is strong enough (Posithiva Gruppen 2022).

2.2. Venhälsan at Sös

Venhälsan is a medical ward at Södersjukhuset in Stockholm that targets everyone who identifies as MSM or transgender people. The main operation involves testing of STDs (sexually transmitted diseases), counseling, advice, and preventing the spread of hiv (Venhälsan n.d).

Venhälsan was opened in 1982 to prepare to take on hiv-patients that were expected to show up in Sweden. It started as an initiative by young activist doctors from RFSL and was initially a non-profit care center for people living with hiv. There were many that wanted to come to Venhälsan, and they had to expand their opening hours from one night a week to two, and still had patients waiting to come. In 1983 they started getting paid (Unia 2023). Eventually, as hiv was spread and more people started having symptoms of aids, Venhälsan opened a ward to take care of people with aids and it quickly grew in size. That ward could be closed a year after the effective treatment in 1996 (Thorén 2022).

Infektionsmottagning 2 is the part of Venhälsan that treats people who live with hiv.
Today, Infektionsmottagning 2 at Södersjukhuset treats around 1600 patients living with hiv (Venhälsan, 2024). At Infektionsmottagning 2, nurses, doctors, counselors, midwives, nursing assistants, and medical secretaries work. Although Venhälsan specifically targets MSM and trangender people, who make up for 89% of their patients, they treat patients with all genders and gender identities (Venhälsan n.d). For the context of this report, the name "Venhälsan" will be used as an overarching term for both Venhälsan and Infektionsmottagning 2.

2.3. Stigma

Previous research, as explained in the introduction and background, has found that one of the main problems with psychosocial health among people living with hiv is different types of stigma surrounding the disease (Kanon and Lindberg 2022). This chapter aims to map and describe the different types of stigma that are most common regarding hiv, to get a thorough understanding of the term.

The word stigma originates from ancient Greek, and translates roughly to "to carve, to mark as a sign of shame, punishment or disgrace" (Economou 2020). Popularized by Ervin Goffman, the term today refers to the negative attitudes or beliefs that are held against certain groups of people, or more specifically characteristics they possess. Stigma contributes to discrimination, marginalization, and social exclusion of people who are "labeled" as different in some kind of way (Goffman 1963). Stigmatized people often experience feelings of shame, blame, discrimination, isolation, social exclusion, secrecy, stereotypes, and the "black sheep of the family" role (Byrne 2000).

In the context of this project, the main focus and research was done about anticipated stigma, enacted stigma and internalized stigma in regard to hiv and hiv from an Intersectional perspective.

Risks with stigma regarding disease is that it leads to isolation, and people avoiding health care and social interactions. However, the main problem for people living with hiv is the ignorance of what it means to live with hiv today, in comparison to what it meant in the 1980s as explained earlier. In 2016, 97% of respondents in a study by Folkhälsomyndigheten claimed that people living with hiv should inform new sex partners about their diagnosis, even though 96% of those affected in Sweden live with viral suppression, and therefore can't

transmit the disease (Folkhälsomyndigheten 2016). In the same study, 55% of the respondents expressed high levels of concern regarding the public attitudes toward people who live with hiv. They witness aids-jokes, simplifying portrayals of people living with hiv, and neglect. The life of someone living with hiv does not correlate to the public's perception of the disease, which contributes to the societal stigmatization of people living with hiv.

2.3.1. Anticipated stigma

Anticipated stigma revolves around the expectation of being treated differently because of your characteristics (Earnshaw et.al 2013). For people living with hiv, this often affects the expectations of being rejected sexually, not getting health care, or being neglected at work or in your family and by loved ones. According to Folkhälsomyndigheten, 78% of people living with hiv experience concerns regarding the disclosure of their hiv among peers, and 16% have avoided looking for jobs because of their hiv (Folkhälsomyndigheten 2016).

In a study about psychosocial health among people living with hiv at Karolinska Institutet, many of the respondents described different types of anticipated prejudice regarding their hiv (Reinius et.al 2021). It was described in many situations that exist not only among specific individuals but also by the general public. Many witnessed losing control of the spread of their hiv status, and therefore losing control of others' perception of them. In the study, the respondents described how they anticipated stigma differently in different settings. One respondent described the difference in sharing their hiv status with their straight or homosexual friends, where every one of their straight friends knew about their hiv, but that they expected too much judgment and labeling in the "homosexual world" to tell people. Some of the participants who were migrants from sub-Saharan countries, testified of not wanting to disclose their hiv status due to anticipated prejudice and judgment by their countrymen.

Anticipated stigma can be found both in the general public, but also in smaller communities where people living with hiv take part. This results in a heavy load from many directions and scales.

2.3.2. Enacted stigma

Enacted stigma is when a person is subjected to negative prejudice, attitudes, and actions from others. In the context of living with hiv, enacted stigma can take shape in being denied health care, being left or rejected by a partner, or being treated differently because of the disease. In the study by Folkhälsomyndigheten from 2016, 22% of respondents who live with hiv witness negative treatment by others when disclosing their hiv status (Folkhälsomyndigheten 2016).

According to Reinius et.al, many of the respondents in their study often referred to other people living with hiv and their experiences when asked about encounters with stigma (Reinius et.al 2021). Respondents also witnessed how the Swedish gay dating community was harsh and judgmental against people who live with hiv. That it was common to demand

from partners that they are “only 100% healthy”, referring to for example hiv. Since the respondents deemed themselves as healthy, because of their viral suppression, this caused them to become angry.

2.3.3. Internalized stigma

Internalized stigma is thoughts and emotions about yourself that often reflect other peoples’ or societies’ negative attitudes. It is easy for an individual to soak up their surroundings’ negative perception and prejudice which can become truths for the individual. This can result in the person feeling dirty or shameful, that they are a danger to others, that they deserve to get hiv, that their worth is less than others, and that they are not worthy of love. It can result in them undermining themselves and their development, and ultimately lead to them not getting the health care they need (Folkhälsomyndigheten 2020). Internalized stigma or “self-stigma” can also be stigmatizing in itself, as it claims that it is something the person does towards themselves when it in fact is a structure in society that is responsible for prejudice.

Reinius et.al (2021) found in their study that the majority of internalized stigma among the interviewed people living with hiv consisted of shame and embarrassment. One also claimed that they had negative feelings towards people living with hiv before they received their diagnosis, causing a dissonance when receiving the infection themselves.

2.3.4. Intersectional perspective on stigma

Intersectionality was coined by American civil rights advocate and a leading scholar of critical race theory Kimberlé Crenshaw in 1989 and refers to the concept that oppression is linked. Oxford Dictionary defines intersectionality as: *“The interconnected nature of social categorizations such as race, class, and gender, regarded as creating overlapping and interdependent systems of discrimination or disadvantage; a theoretical approach based on such a premise.”* (OED 1989).

An intersectional perspective enables researchers to see people's situations and experiences without reducing them to one group or category, which can cause a hindrance when trying to understand and explain complex structures (Kanon and Lindberg 2022). Intersectionality is a term used to analyze how social categories such as gender, sexuality, age, functionality, and nationality can cross and strengthen each other. This perspective takes these different social categories, to see how combinations of suppressions affect individuals and groups.

Because of the amount of stigma surrounding people living with hiv, they can also be counted as one of these social categories. This adds complexity to the hiv stigma, because of the high level of heterogeneity in the patient group in Sweden. 89% of the patients at Venhälsan are MSM, but 65% of people living with hiv in Sweden are also migrants from other countries. Many come from countries where hiv is much less accepted than in Sweden, and some even migrate because of their hiv status. An intersectional perspective can increase the

understanding of different demographics within the group of people living with hiv in Sweden, specifically the patients at Infektionsmottagning 2 at Södersjukhuset.

2.3.5. Review of stigma-reducing interventions

As the previous chapters explains, there are many sides to the stigma against people living with hiv. It was therefore important to review how stigma has been researched and addressed in connection to other stigmatized groups, as an inspiration or guide for this project.

Weight has long been a topic of discussion in the health and well-being discourse. Weight stigma and weight bias have followed along in the discussion, as there often is a wrong assumption that obesity is the result of a lack of discipline and responsibility. Recent evidence shows that obesity is a widespread, complex chronic disease that is caused by interplay of behavioral, environmental, genetic, and metabolic factors. Much of the stigma is also based on a "weight normative approach", where weight and weight control have been a factor of well-being (Sánchez-Carracedo 2022).

Research conducted in the United Kingdom revealed a notable increase in unhealthy weight-control behaviors among adolescents from 1986 to 2015 (Solmi et.al 2021). Most noteworthy was the rise in such behaviors among girls. Additionally, those who perceived themselves as overweight exhibited more depressive symptoms compared to those who perceived themselves as having a normal weight. Implementations of screening in schools, and the introduction and mapping of body mass index (BMI) have also increased the level of displeasure among your people about their weight (K.A. Madsen et.al 2021).

"Weight-based stereotypes" are also brought up as a factor contributing to weight stigma, as there often are generalizations about overweight people as "lazy, gluttonous, lacking in willpower and self-discipline, incompetent, unmotivated to improve their health, non-compliant with medical treatment, and are personally to blame for their higher body weight." (Sánchez-Carracedo 2022).

While the weight discourse is on-going, there have been many interventions to try to mitigate the weight stigma, which in this report have been categorized into different approaches.

Messages toward ending the stigma

- In 2016, the WHO issued a call to end discrimination in healthcare, publishing one year later a "report with recommendations to address the problem of obesity stigma" (WHO 2017)
- "The call to the media to reduce the stigma of obesity by different British associations" linked to the study of obesity (Flint et.al, 2018).

Narrative-changing interventions

- The implementation of British Parliament guidelines, such as referring to “people living with obesity” instead of “obese people”. Putting the fact that they are people before their characteristics (Flint, 2020).
- Working against stereotypes in media and campaigns, not displaying “weight based stereotypes”

Information spreading

- Recognizing the fact that obesity is not entirely in the individual’s control. The condition is a complex disease based on many factors.
- Changing the direction of obesity prevention campaigns to more focus on adopting a healthy lifestyle and behaviors, instead of focusing on weight loss.

Another area where stigma is present and where much research has been done is regarding mental illness and stigma, where information spreading is discussed as the most common weapon in the fight against stigma. As Peter Byrne (2000) of Cambridge University explains regarding interventions against stigma: “*The starting point for all target groups and at every level is education”.* Byrne writes that the first step to reduce stigma is to educate all different target groups, regardless of their prior knowledge, and gives an example of the internet as a very effective source to reach many people. Moreover, Byrne refer to Wolff et al (1996a,b), which suggests a practical working model for different target groups were targeted intervention can be done, all based on education:

- “Specific target groups, with prior identification of their attitudes
- No evidence of community backlash
- Flexible public education packages
- Small groups work better
- Several interventions over time exceed the sum of their parts
- Continuing contact with the group (keyworker) maintains momentum”

Byrne (2000) continues by writing that the “carrot” of education often also needs a “stick” to reduce stigma. Stigmatizers are often not the target group that is open for education on the matter, and thus might need rules, sanctions, and regulations to drive the process of destigmatization.

The Canadian Centre for Addiction and Mental Health has belief in more than information to fight stigma, and lists seven things to think about to reduce stigma:

1. Know the facts. Educate yourself about mental illness including substance use disorders.
2. Be aware of your attitudes and behaviour. Examine your own judgmental thinking, reinforced by upbringing and society.

3. Choose your words carefully. The way we speak can affect the attitudes of others.
4. Educate others. Pass on facts and positive attitudes; challenge myths and stereotypes.
5. Focus on the positive. Mental illness, including addictions, are only part of anyone's larger picture.
6. Support people. Treat everyone with dignity and respect; offer support and encouragement.
7. Include everyone. It's against the law to deny jobs or services to anyone with these health issues (CAMH n.d.).

3. Methodology

This chapter gives an overview of the design process in this project, its stages and methods, based on a service design methodology and Design Thinking process. Moreover, a summary of associate professor in service design Josina Vinks' study on common assumptions and alternative approaches about service design, and how social innovation can be utilized within the boundaries of the project is presented.

3.1. Design process

Using a human-centered service design methodology, a design process and method overview was created, building on the Design Thinking Process by Stanford d.School (2024). Through diverging and converging through a Double Diamond structure (Design council 2024), the process was iterative, with involvement of actors in several rounds of interviews, research and collaboration.

An overview of the design process is shown in Figure 4.

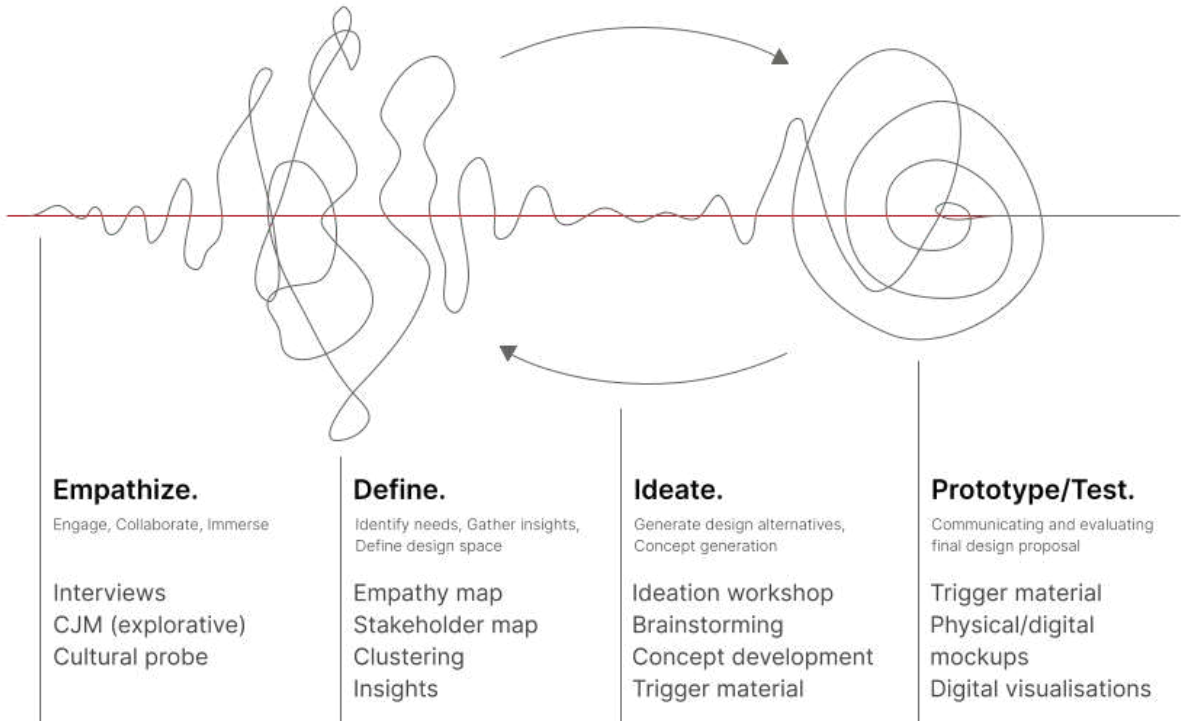


Figure 4. Design process and methods used for each stage.

The **empathize mode** was about empathizing with the patient-group, utilizing, engaging and collaborating with patient organizations and co-workers, and immersing in the patient's situation. Throughout this phase, time was spent at Venhälsan in different settings, for example in staff meetings and briefings about patients, to get a broader understanding of the

context in which the different stakeholders act. The following exploratory methods were used in this phase:

- Open semi-structured in-depth interviews with the stakeholders, for example, people living with hiv, experts, and healthcare co-workers. This aimed to gain a rich understanding of different perspectives and experiences.
- A cultural probe (in this case a diary) was used to understand different perspectives and try to catch normal unbiased everyday emotions in context without the presence of an interviewer which might have affected the outcome (Stickdorn et al. 2018).
- Trigger material, in the shape of customer journey maps, was used to gain a deeper understanding of the respondents experiences while engaging them.

All the methods will be discussed more in detail in the main study chapter, mostly in the pre-study and patient-group study.

The **define mode** unpacked the findings from the empathize mode and translated them into needs. These needs were then used to reframe the project's design space through compiling the findings into major insights. The methods used in this phase were:

- Empathy maps were used after each interview to facilitate the clustering and analysis.
- Clustering was done with the information from the empathize mode to find patterns and common themes among the researched material.
- A why-how ladder was created to visualize and process problems, how they are connected and what consequences they lead to.
- Insights were phrased by gathering different findings, which then acted as a base for the ideation mode.

In the **ideation mode,** both quantitative and qualitative methods were used to generate and evaluate ideas. The following methods were used:

- An ideation workshop was held with some of the healthcare co-workers at Venhälsan, using “How might we …?” questions to generate ideas.
- Several iterative brainstorming sessions were carried through within the research group, using the same “How might we …?” questions form the workshop.
- Ideas were refined, combined and prioritized to develop concepts.
- Desktop research and interviews to develop concepts further.

The last **prototype and test mode** entailed finalizing one idea and visualizing it to be able to evaluate the outcome together with the respondents and healthcare co-workers at Venhälsan. The vision was to finalize the concept enough so that Venhälsan could move forward with the idea. The main methods used were:

- Trigger materials were used to present and trigger people from the patient group to give feedback about presented concepts. The same material were given to counselors at Venhälsan to get their feedback as well.
- Physical and digital mockups were created to finalize the design.

3.2. Service design

This project was based on a service design approach and methodology. Service design can be described in many different ways, however, a popular one, according to Stickdorn et al. (2018) is:

“Service design helps organizations see their services from a customer perspective. It is an approach to designing services that balances the needs of the customer with the needs of the business, aiming to create seamless and quality service experiences. Service design is rooted in design thinking, and brings a creative, human-centered process to service improvement and designing new services. Through collaborative methods that engage both customers and service delivery teams, service design helps organizations gain true, end-to-end understanding of their services, enabling holistic and meaningful improvements.”

To further describe and clarify what service design is, Stickdorn et.al (2018) offer six principles of service design. It is:

Human centered
“Consider the experience of all the people affected by the service.”

Collaborative
“Stakeholders of various backgrounds and functions should be actively engaged in the service design process.”

Iterative
“Service design is an exploratory, adaptive, and experimental approach, iterating toward implementation.”

Sequential
“The service should be visualized and orchestrated as a sequence of interrelated actions.”

Real
“Needs should be researched in reality, ideas prototyped in reality, and intangible values evidenced as physical or digital reality.”

Holistic
“Services should sustainably address the needs of all stakeholders through the entire service and across the business.”

3.2.1. Assumptions about Service Design

To further investigate into the meaning of Service Design and the role it could have in this project, a study of associate professor in service design Josina Vinks’ research on assumptions about Service Design.

In contrast to the popular understanding of service design, Vink (2019) argues that this explanation of service design fails to consider the situated struggles of different actors. Instead, Vink proposes that a more processual, embedded, systemic, and embodied perspective of service design is needed to challenge the underlying assumptions about service design in their research. For the scope of this project, one of their research questions was particularly interesting:

"*What underlying assumptions are getting in the way of a more situated understanding of service design, and what alternative assumptions are needed?*".

For this question, Vink has discovered six underlying assumptions about service design (and design in general), and proposed alternative assumptions to them, see Figure 5.

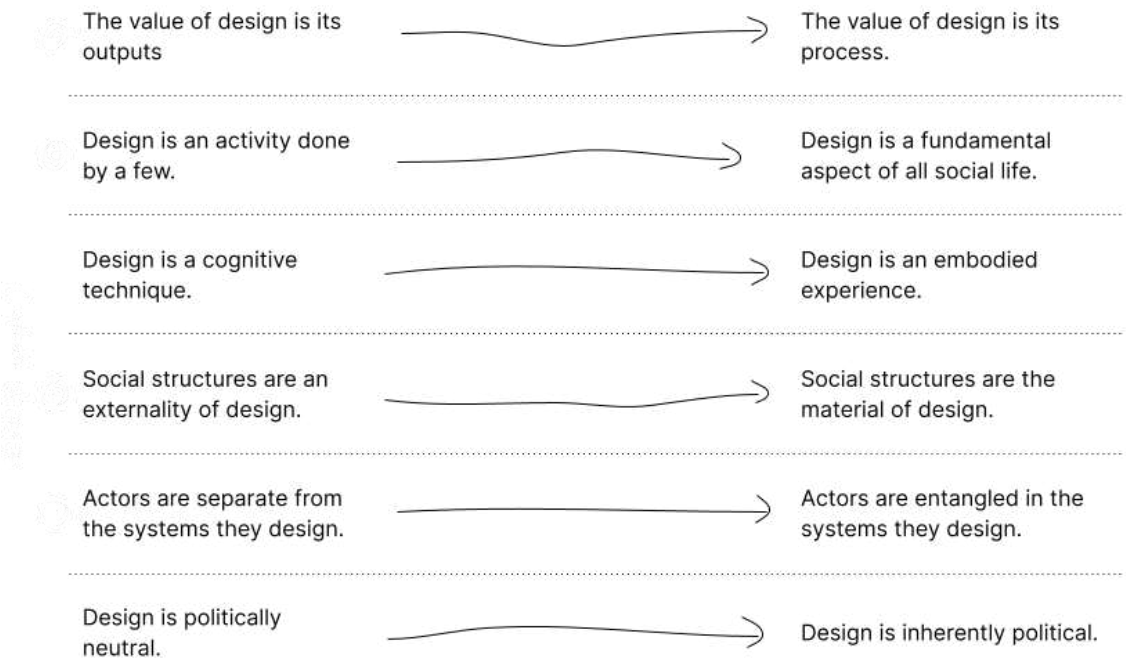


Figure 5. Josina Vinks' identified underlying assumptions and their corresponding alternative assumptions.

The value of design is its outputs. → The value of design is its process.

Historically, design has often been associated with a goods-dominant perspective, and even if that is shifting, Vink claims that a heavy focus is still on what output there will be, thus ensuring that service design only adds value at the end of a process. Instead, Vink argues that the value of service design lies in "a social process of creating, disrupting and maintaining social structures." Although one goal with this project was to create a solution, the process of service design: the insights, how service design could be used in this sort of project, what methods work and do not work, should not be underestimated.

Design is an activity done by a few. → Design is a fundamental aspect of all social life.

Vink further shows that design is seen as something that a few trained people do, while arguing that design could be a collective process by many stakeholders. Vink clarifies by saying "This does not mean that all actors have the same level of skills in design, but rather that all actors intentionally shape social structures through their actions." This is also supported by the collaborative aspect in the six main principles of service design. For this project, following Vinks' assumption, it was important to take into consideration subtle social aspects throughout the project.

Design is a cognitive technique. → Design is an embodied experience.

Popular representations of design thinking have tended to depict design as a process of problem-solving that happens within actors' heads (Vink, 2019). Vink discusses how there is a focus on mental techniques in service design methods, that problem-solving, decision-making, and idea generation is a mental process, ultimately disregarding the body in the design process. Vink argues that disregarding the senses and the embodied experience of different situations can result in a hollowed-out design process. A goal of the project was therefore to include physical environments, trigger material and artefacts during all stages of the design process.

Social structures are an externality of design. → Social structures are the material of design.

Vink states that "social structures, the entrenched, widely-shared norms, rules, roles and beliefs" have a central role in designing and that it is impossible to leave them out. Furthermore, and especially in healthcare, service design can "create, disrupt and maintain social structures through specific practices" and "aid actors in apprehending these taken-for-granted social structures so that they can intentionally shape them.". For this project, where stigma was a big issue, this was important to emphasize and utilize. If the structures are a reason why the stigma is still in place, it was crucial for the actors to be made aware of them.

Actors are separate from the systems they design. → Actors are entangled in the systems they design.

Vink describes how designers see themselves as unrelated to the context they are designing for, for example by often saying they put themselves in the position of the customer, not realizing that they are always related to the context in some way, that no one is context-less. Vink further claims "This work reinforces that even when actors are working to become aware of social structures, they are entangled in, constrained, and partially blinded by those social structures.". Taking this into consideration, it was crucial to not only illuminate the

social structures but to actively try to oppose them throughout the design process. This was important to mediate with all actors involved.

Design is politically neutral. → Design is inherently political.

In the final shift, Vink discusses the political aspect of design. There often is an assumption that design is for the common good and that involving customers and engaging people in the design process contributes to that. Vink however, discusses the power that design has, and the influence it can bring. Their main argument is the intrinsic relationship between social structures and power. That social structures influence actors' behaviors, and that through this, certain actors can affect the social structures to influence political opportunities, and influence service systems toward their agenda.

This was important to keep in mind during a project that heavily relied on existing social structures, that involved stigmatized groups, to work towards reshaping the understanding of these groups.

These shifts affected this project in how the approaches and methods for different phases were chosen. It also discussed the role of service design. How service design could work within a setting of psychosocial problems, the role it could have within the boundaries of the project, and how it could break new ground in areas where social- and healthcare co-workers most commonly are found. This required the involvement and collaboration of actors on different levels. Experts in hiv and stigma, the co-workers at Venhälsan, and most importantly the patient group themselves. Discussing the existing social structures that are in play and affect the patient groups, how the structures could be affected or bent to benefit the project.

3.3. Social Innovation

Given the level of complexity, the combinations of social structures and stigma surrounding hiv, social innovation theory was closely related to the themes of this project. To fulfill the purpose of this thesis, it was paramount to recognize the social resources that were already in play, and to find, utilize or change latent resources and structures that could be used.

Co-founder of the Design for Social Innovation towards Sustainability network, Ezio Manzini describes social innovation as: "*...all that design can do to recognize, stimulate, extend, and sustain, what I call, promising ways of being and doing. This defnition assumes that we can also find people and organizations capable of implementing systemic changes that can lead toward socially and environmentally sustainable ways of living*" (Muratovski, 2022).

An *optimistic* view of social innovation is to view people and society as bearers of opportunity rather than problems, who all can act to drive positive change. In the context of living with hiv, many actors on different levels are involved in working both toward and against the stigma. The main actors are displayed in Figure 6.

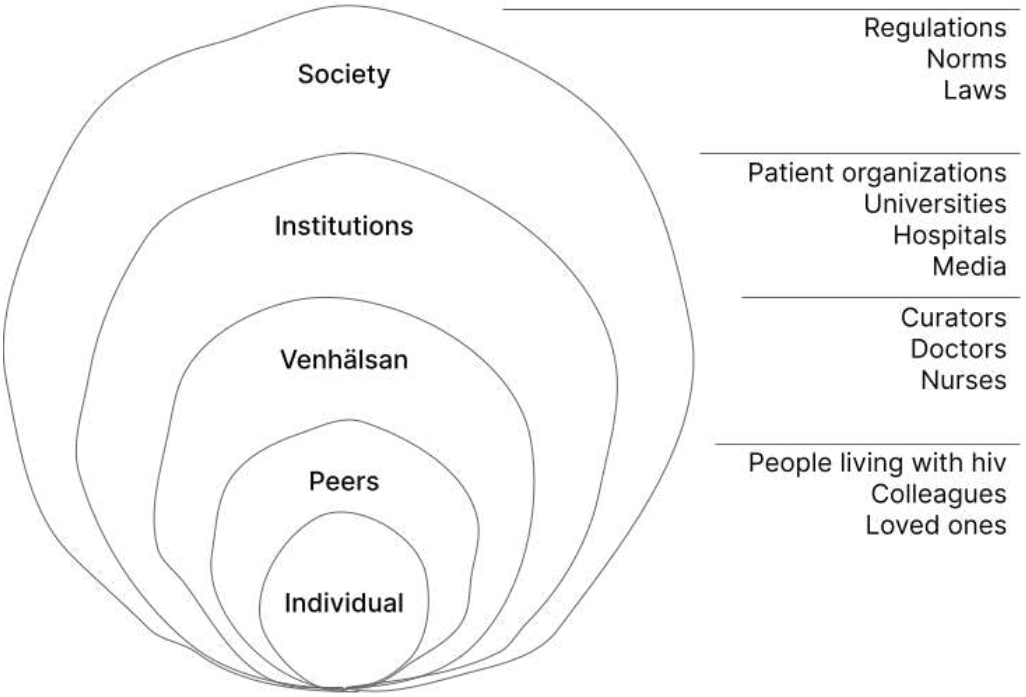


Figure 6. The main actors involved in the hiv discourse.

To design for social innovation within the boundaries of this project required involvement and understanding of the different actors and their context. A thorough understanding of existing problems on different levels was needed to choose where an intervention could have the most impact. This type of social experimentation is a collaborative process, and the social needs diagnosis should be shared by a wide range of stakeholders (European Commission, 2022). The target group would through the human-centered approach be directly involved in the needs study, while other actors such as patient organizations or co-workers at Venhälsan was indirectly involved, serving as intermediaries, representing the target group while adding valuable professional knowledge.

4. Main study

The main study consisted of a pre-study of interviews with different stakeholders to understand the context of Venhälsan, the support that is offered from them and patient organizations, and research on hiv and stigma. This was followed by a qualitative patient-group study, and an analysis phase where data was processed and condensed into insights. A full list of all respondents can be found in Appendix A.

4.1. Pre-study

A pre-study was conducted by first discussing imagined and expected needs of the target group, then interviewing relevant stakeholders, such as healthcare co-workers, patient organizations and researchers.

The interviews with the stakeholders were in-depth and semi structured to make the interviews more flexible and more resemble a real conversation, thus creating a more comfortable environment for the respondent. This also allowed the interviewer to focus more on some areas that were deemed interesting, or that seemed important to the respondent to talk about. The interviews followed a mind-map like structure.

A total of 11 in-depth interviews were conducted in the pre-study. The interviews ranged between 40-60 minutes in duration and the structure and topics that were discussed varied between each stakeholder. All interviews were recorded with consent, and then transcribed. See the complete list of respondents in Table 1.

Table 1. Respondent from the pre-study.

ID	Time [min]	Location of interview	Profession	Description
Respondent 1	60	Venhälsan	Counselor	Working at Venhälsan
Respondent 2	40	Venhälsan	Counselor	Working at Venhälsan
Respondent 3	49	Venhälsan	Nurse	Working at Venhälsan
Respondent 4	60	Venhälsan	Nurse	Working at Venhälsan
Respondent 5	60	Venhälsan	Nurse	Working at Venhälsan
Respondent 6	41	Venhälsan	Doctor	Working at Venhälsan
Respondent 7	47	Digital	Counselor	Working at Infektionsmottagning 2 at Karolinska sjukhuset
Respondent 8	42	Digital	Lecturer and associate professor at KI	Maria Wiklander
Respondent 9	38	Digital	Operations manager at PG	Daniel Suarez
Respondent 10	43	Digital	Team leader for NAs' social support	Emma Olsson
Respondent 11	43	Digital	Sociologist	Kenneth Fjällborg

4.1.1. Imagined and expected needs

Based on the background research, a study of imagined and expected needs was made, following Innovationsguidens' (2022) structure for finding and framing the problems, and to try to capture the relevant stakeholders. This work was done in collaboration with counselors at Venhälsan, to use their knowledge.

Innovationsguiden provided these helpful questions to define the challenge and its' problems:

- What is this about? And why is it important?
- Who is this a problem for? Who is affected by this question?
- What social and cultural factors in society or your everyday life shape this challenge?
- What proof is there that this is a problem?

These questions were important to make sure the research team shared the same idea of the challenge with co-workers at Venhälsan, as well as illustrate the different factors that contribute to the challenge.

A first stakeholder map was created to visualize which stakeholders were involved, and in what way they were. This led to it becoming more clear how the stakeholders were connected to the patient group, and how they should be involved in the project. See Figure 7 for initial stakeholder map.

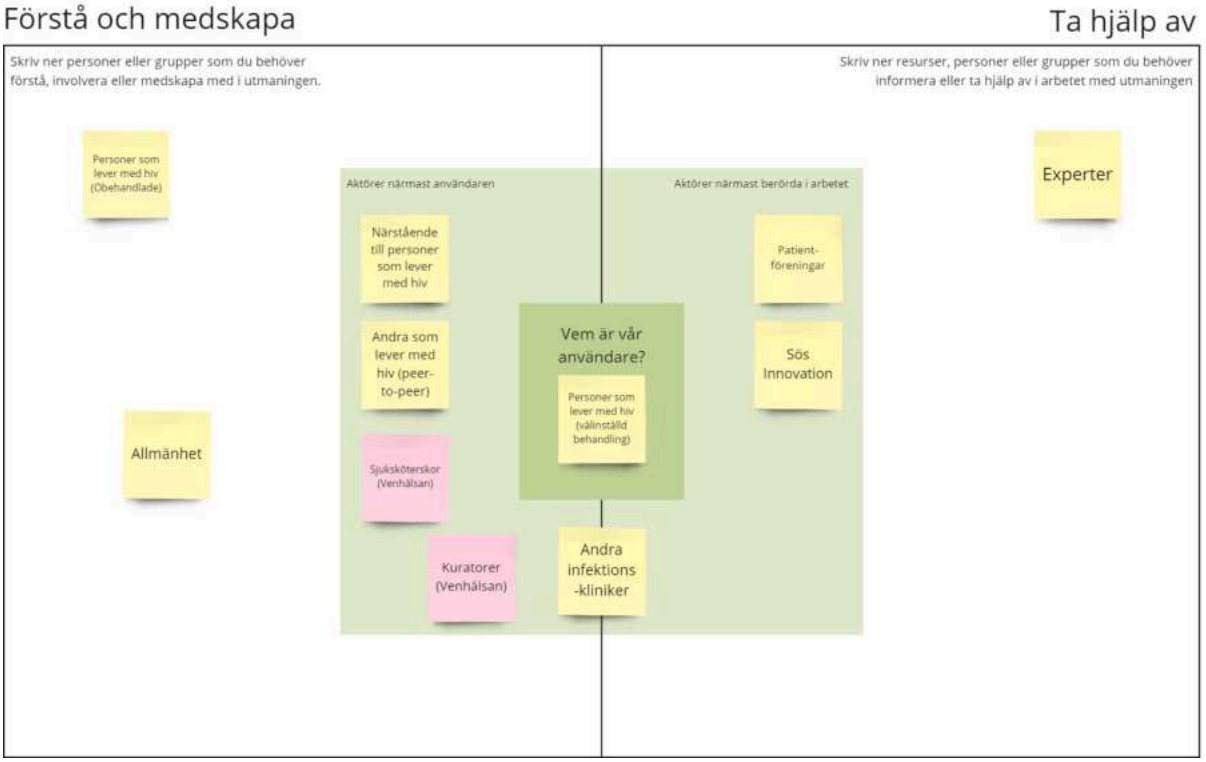


Figure 7. Stakeholder map created in collaboration with counselors at Venhälsan.

4.1.2. Interviews with healthcare co-workers at Venhälsan

To gain an initial understanding of the context of Venhälsan, their co-workers, the care they offer and what needs they find in the patient group, 2 counselors, 3 nurses and a doctor working at Venhälsan were interviewed.

The interviews followed a similar mind-map-like interview guide, with some profession-specific questions and lasted for around 60 minutes. See Appendix B for all interview guides. Some topics that were discussed were:

- Medical treatment
- Structure of appointments
- Existing psychosocial support
- Target group mapping

Takeaways

The interviews offered great insight in the structure of the care at Venhälsan, how the different professions work with the patients, and how they communicate internally. It became evident that all the co-workers have a quite close relationship to their patients, and that they quickly want to establish contact with them. When diagnosed at Venhälsan, the patient should ideally meet both a doctor, a nurse and a counselor on their first visit. They quickly want to establish contact with the patients, and immediately work on creating a safe environment for them, as receiving an hiv-diagnose often is a life-changing event.

During the interview stage, this timeline was offered from Venhälsan, covering the initial visits a newly diagnosed patient goes through the months following a diagnosis. See Figure 8.

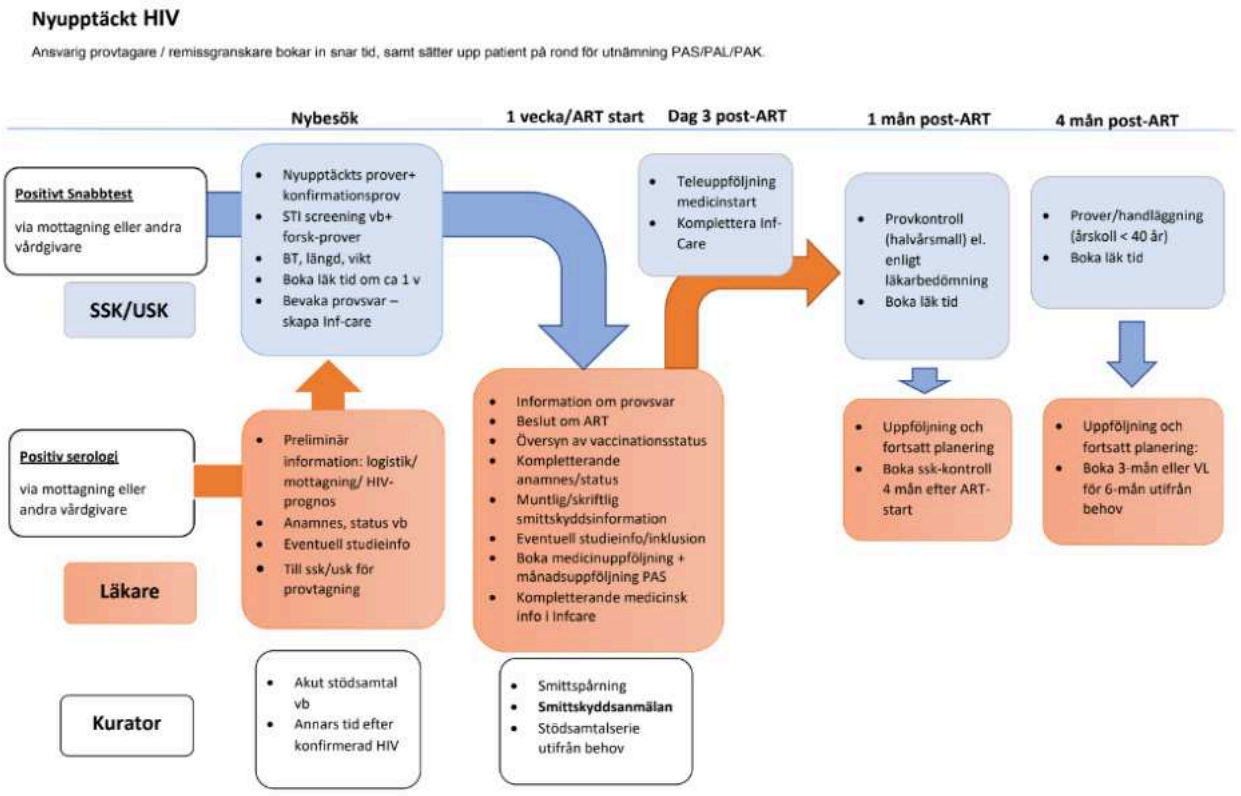


Figure 8. Timeline of visits a newly diagnosed patient goes through, and who they meet during each visit (Venhälsan n.d.).

The doctors and nurses each have patients they are responsible for, so the patients should meet the same people each time they visit Venhälsan. This creates a friendly and safe environment. It also became evident that all the professions work with different types of support for the patients. As the nurses are the ones the patient meets each visit, they do a lot of work on identifying needs of support, and therefore communicate with counselors if any extra support is needed.

The counselors also explained that they sometimes work with issues that are more social, such as economical problems, housing or even assisting patients with legal issues. These issues are more common among people who received a diagnosis in the 80's or 90's, when hiv was more serious. Migrants might face other problems, such as application for asylum.

The doctors meet the patients less often and work questions regarding the medical aspects of the infection, and adjusting medication to reduce side-effects.

4.1.3. Interviews with researcher and organizations

In addition to healthcare co-workers, experts, patient organizations and non-profit organizations were interviewed. Similarly to the previous interviews, they followed a mind-map like structure, see Appendix B for the full interview guide.

Researchers

Maria Wiklander, lecturer and associate professor at Karolinska Institutet, has published many articles related to hiv and stigma. She was interviewed about her work on the article *Towards a new understanding of HIV-related stigma in the era of efficient treatment- A qualitative reconceptualization of existing theory* (2021) as well as hiv in society in general. The interview covered questions about the target group and tips about interviewing patients, challenges in their study, and stigma. Wiklander also shared tips on further research, and the project as a whole, see Appendix B for interview guide.

Patient organization Posithiva Gruppen (PG)

PG is a patient organization, created for and by people living with hiv. They work towards increasing knowledge about hiv in society, for example lectures and workshops, while protecting their rights and acting de-stigmatizing. They provide a place for people living with hiv to gather and socialize, while also offering education and psychosocial support. It is a non-profit organization, and free for all people living with hiv. Historically, they were created for MSM, but welcomes everyone today living with hiv. Additionally, they offer separatistic activities for MSM, women, pregnant people, transpersons, young people between 18 to 30 years old, and older people that are older than 50 years (Posithiva gruppen n.d.).

Daniel Suarez, operations manager at PG, was interviewed to deepen the knowledge about their work, as well as their experience with the patient-group. Questions regarded who they are and are not reaching, how they work together with Venhälsan, what sort of support they offer and how it works, and what needs they see, see Appendix B for full interview guide.

As PG is a patient organization, the people working there all live with hiv. This makes their work revolve much around peer-to-peer interactions in different ways, by offering gatherings and events where peers can meet.

National Association Noaks Ark

Noaks Ark is an national association, working regionally to offer, similarily to PG, support, activities, and a place to meet for people living with hiv. They also offer testing for hiv. They work, among other things, against discrimination and stigmatization, while increasing the knowledge about hiv and how one protects oneself and others. Their goal "is to work for a reduced spread of HIV and its consequences for the individual and for our society as a whole." (Noaks ark n.d.)

Emma Olsson, team leader for NAs' social support, and Kenneth Fjällborg, sociologist, were questioned about their work, as well as their relationship and perception of the target group. They were asked who they are and are not reaching, their work with Venhälsan and what support they offer.

The interview offered insights into the heterogeneity of the patient group, and how the patient organizations work to adapt to needs that might arise for people living with hiv. They also collaborate together with other actors, to try to offer as much and broad support for people living with hiv as possible.

4.2. Patient-group study

After the pre-study, a study of the patient-group was done with people living with hiv, and one partner to a person living with hiv. First, in-depth interviews and customer journey maps were conducted, and the results were analyzed with empathy maps. Cultural probes were used for some of the respondents that were willing. A total of 9 interviews were held, ranging from 40-60 minutes in duration. Most of the interviews were held in Swedish, while some were held in English. For one interview, an interpreter was needed, who joined via telephone. Similarly to the interviews in the pre-study, all interviews were recorded with consent, and then transcribed. See all respondents in Table 2.

Table 2. The respondents from the patient-group study.

ID	Time [min]	Location of interview	Description	Other
Respondent 12	51	Digital	Man, age 68	
Respondent 13	42	Digital	Man, age 47	
Respondent 14	60	Venhälsan	Man, age 50	
Respondent 15	59	Venhälsan	Man, age 45	Interpreter needed
Respondent 16	60	Venhälsan	Man, age 74	
Respondent 17	40	Digital	Man, age 24	
Respondent 18	60	Venhälsan	Man, age 40	
Respondent 19	49	Digital	Woman, age 39	
Respondent 20	23	Digital	Man, age 57	Partner to one respondent

4.2.1. Interviews with the respondents from the patient group

Similar to the in-depth interviews in the pre-study, the interviews with the respondents from the patient-group were semi structured. This created a more relaxing situation for the respondent as the interview was more flexible, and they could lead the conversation to which direction they wanted. The respondents were partly recruited through co-workers at Venhälsan, who asked if they wanted to participate, and through posters that were displayed in Venhälsans' waiting room and on their instagram. These posters were also put up on Noaks Arks' and Positiva Gruppens' premises, see poster in Appendix C.

The interview guide for respondents living with hiv focused on mainly six different areas:

- Their relationship to their hiv
- Disclosure of their hiv status
- Hiv in their everyday life
- Hiv in regard to their relationships to other people
- Venhälsan and other healthcare settings
- Support

See Appendix B for the full interview guide.

Due to the heterogeneous patient group, respondents were of varying age, background, nationality and gender. The interviews were done in two different settings: on location at Venhälsan, or online via either Zoom or WhatsApp.

The interview phase also included interviewing a partner to someone living with hiv. The interview revolved around disclosure of the other persons hiv, their knowledge then and now, if it affected them in their everyday life and relationship, and if they thought it affected the other person. See Appendix B for the full interview guide.

Takeaways

The responses from the interviews were much in-line with the target group, in that their experiences varied a lot. Some respondents were diagnosed in a time before active medication, when hiv was a death sentence, while some were diagnosed a few years ago. What was interesting was the fact that their reaction to the diagnosis was quite similar, although a well-treated diagnosis poses very few problems for the patient.

Most of the respondents had disclosed their hiv status to someone, but it was still a delicate subject. The general standpoint was that the patients were selective when disclosing, to keep the amount of people who know their hiv status under control.

Disclosure was often received in a good way when telling friends, family and for example co-co-workers, but was more of a problem when trying to find new partners.

Many of the respondents expressed a feeling of anticipated stigma more than enacted stigma. There were more instances of either referring to other people's experiences or expecting to be treated in a specific way due to their hiv status, rather than actually being treated differently. Those instances were often in health care settings, or more subtle interactions with people in their surroundings, not wanting to share a coffee mug due to it being contagious, or similar.
Lastly, there was a great belief in the hiv-community and peer-support, that there was a lot of potential in helping people struggling with their hiv status through peer-to-peer initiatives.

What became evident from the interview with the partner to someone living with hiv was that people close to people living with hiv want to be involved in the care they receive, and be updated about any changes in the patients' well-being, if they have a good relationship with the patient. As hiv is still regarded as a serious disease, disclosing their status is viewed as a sign of trust from people close to the patients.

4.2.2. Customer Journey Map

To engage the respondents that could meet at Venhälsan, a simple customer journey map was co-created as a boundary object to recap the respondents' previous experiences, and how they made them feel.

The respondents were shown a straight line with two fixed points, *diagnosis* and *today*, see Figure 9, and were told to mark important events between these points, placing them over or under the line, depending on if it was a positive or negative experience. This allowed the interviewer to focus on those important events, and the timeline also acted as a basis for

further questions. By putting the important events on post-its, the respondents were forced to phrase words that could be associated with said event, and thus deeper reflect about the event.

At the end of the interview, after all the post-its had been placed, the respondent was asked to draw a line of their mental well being in regards to their hiv, to see how it had shifted. This made it easy to notice any highs or lows that did not correspond to any post-its that were important to ask and talk about. A total of three customer journey maps were made. See Figure 10 for a co-created customer journey map.

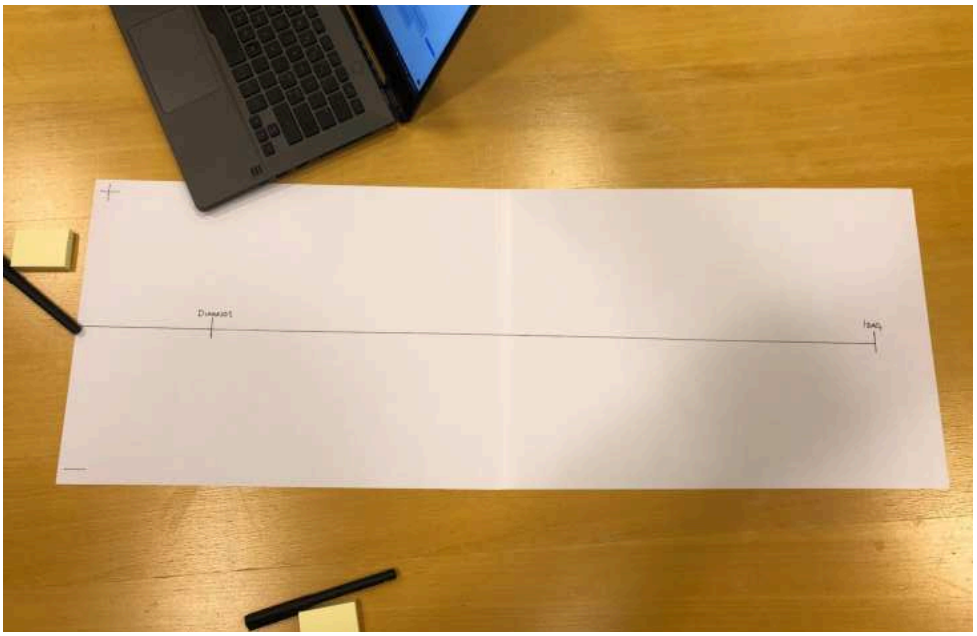


Figure 9. Base of customer journey map

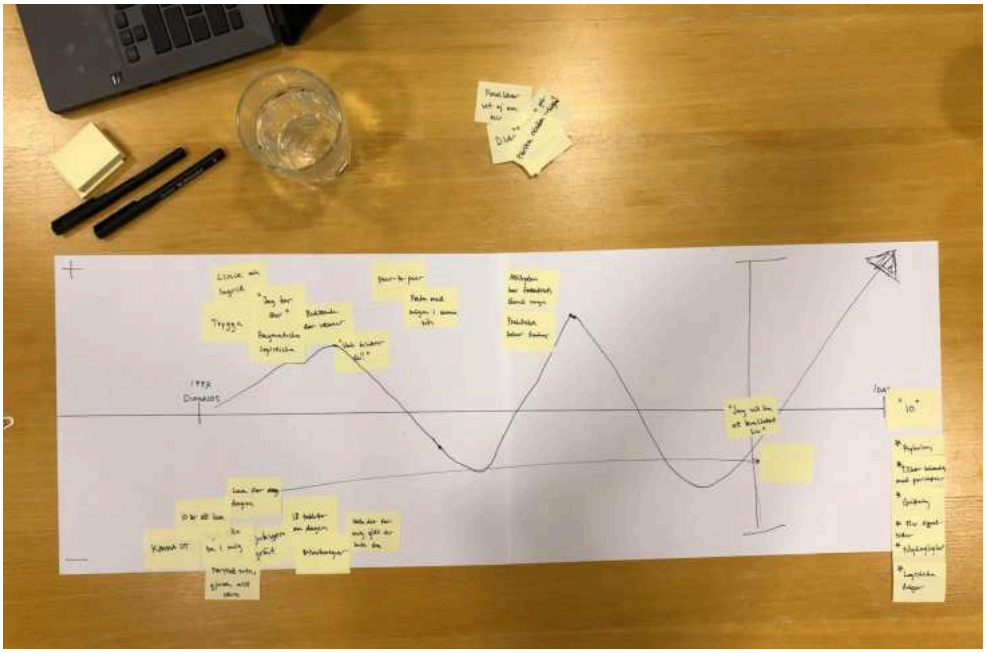


Figure 10. Co-created customer journey map

Takeaways

The map enabled the respondents to stay on specific experiences for a longer time. If an experience was defined and placed as a negative or positive experience, the respondent was asked to describe why, and how it affected them. This made defining challenges and feelings easier for the respondents and made the researchers gain a deeper understanding of how the events affected the respondents.

4.2.3. Cultural probe

A cultural probe is a method where researchers are not participating, which is used to collect data and information from tasks by the researchers (Stickdorn et.al, 2018). When using a cultural probe, participants are asked to document their experiences, through for example photos, texts, or artifacts. By letting participants describe their experiences without a researcher present, the data becomes unbiased and guides researchers across cultural boundaries to change their perspectives.

In the patient-group study, participants received a physical or digital diary, shown in Figure 11. The entire package of instructions and material was in a pocket-size, to allow the participants to always carry it with them. The idea was for them to be able to "capture the moment" and feelings connected to it, without having to wait until they came home.



Figure 11. The physical diary: a pen, a notebook and instructions for the task. See content of instructions in Appendix D.

The participants were instructed to write every day for two weeks. They were informed to: *Reflect about if your hiv has affected you today. Write about* ***what happened*** *and* ***how it felt****.*
To make the most out of the information gathered, follow-up interviews were held with the participants. Before the follow up interviews, the researcher processed the diary entries and prepared questions on entries and subjects that seemed important.

Takeaways

Unfortunately, only a few respondents wanted to write a diary. Some did not have the time or mental capacity, while hiv for others was such a small part of their everyday life that they felt they would return the diary empty. However, the diary worked great as a cultural probe for the few who followed through. Follow-up interviews were held to discuss certain diary entries which allowed the respondents to elaborate on the small notes they had made, and the researchers to ask follow-up questions. One respondent was surprised with how much they thought about hiv, having thought they did not think about it at all during their everyday life.

4.3. Analysis

The pre-study and most importantly the patient-group study resulted in a considerable amount of material that needed to be analyzed. To facilitate that, different methods were utilized such as empathy and system maps, clustering, and a why-how ladder.

4.3.1. Empathy maps

After each interview, an empathy map was used to synthesize and unpack information from the respondent. This was used to gain an initial better understanding, through processing what was said, to try to find the respondents' underlying thoughts. The empathy map used is displayed in Figure 12.

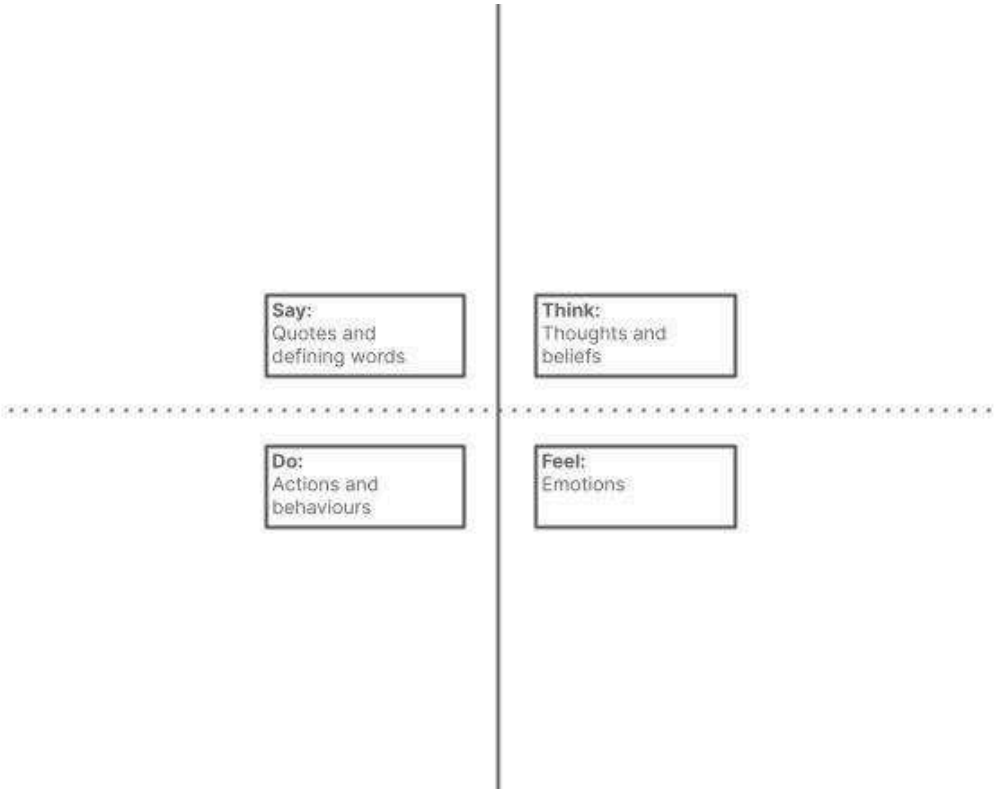


Figure 12. The empathy map used to unpack information from the patient interviews.

The empathy maps were conducted right after the interviews with the patients. After listening through the recorded interviews to make sure nothing was missed, important parts were highlighted and then transferred to digital post-its, which were then placed in the “say” and “do” quadrants. These were observed and noted directly from the respondents. The contents in the “think” and “feel” quadrants were inferred by the researchers, by asking “What might the respondent be thinking? What does this say about their beliefs?” (Stanford d.School, 2024). All the post-its were tagged for each respondent so it could be traceable, and the post-its that the researchers inferred in the “think” and “feel” quadrants were a different color to differentiate what was hard facts and what was interpreted. After the maps were completed, additional post-its were created for insights and new questions, phrased as “How might we?”. These were again a different color to differentiate them from the rest of the post-its.

As mentioned, in total 9 interviews were conducted and thus 9 empathy maps were created. It was important that the maps were conducted right after the interviews to catch certain emotions and interpretations that would later have been forgotten. A few maps were created the day after the interview and those took noticeably longer to complete. Apart from that, the empathy maps worked as a great tool to break down each interview and discuss what was being said, for example a lot of patterns or contradictions could be identified between or inside the different quadrants. Since a lot of insight was gathered in the maps, it facilitated the analysis tremendously.

4.3.2. System map

It became evident during interviews with the patients and the Venhälsan co-workers that many actors contributed to how the patient felt in some way. To get an overview of how the actors interact with each other and the patients, a system map was made, see Figure 13.

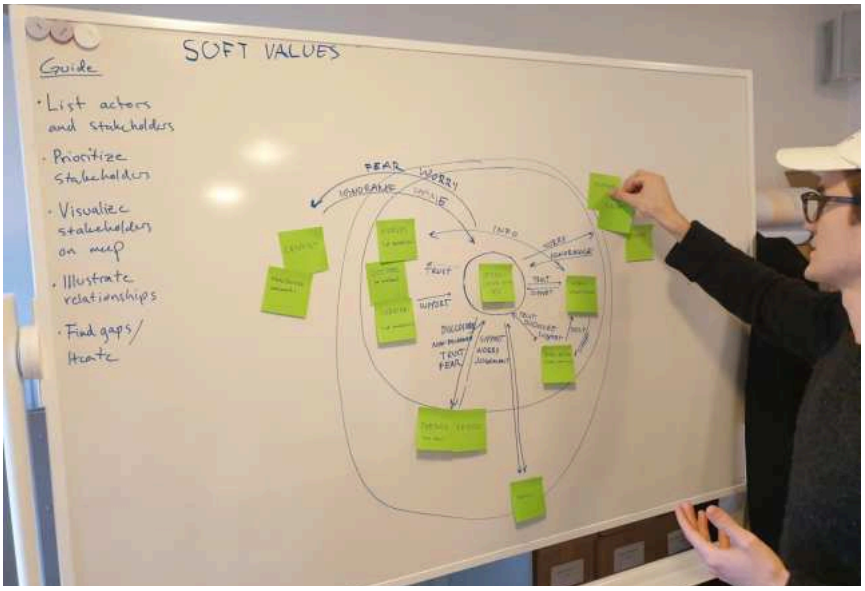


Figure 13. A system map with actors and soft value transfers.

Actors were placed at various distances from the patients, representing how peripheral or close they were to the patients. As shown in the picture, soft values, feelings or interactions between the patients and the actors were also mapped.

What became clear during the mapping was how there were many levels, or tiers of trust between the patients and different actors. Who the patients disclosed their hiv status to varied a lot. The people that the patients most often disclosed their hiv status to were, of course, Venhälsan, but also often patient organizations and peers living with hiv. Contexts where hiv is accepted and sort of the premise on which you interact. These were contexts where the patients often felt most safe, where there was an exchange of trust and support.

For some patients, this tier also involved friends and family. For some, however, these were actors who were not told about the patients' hiv, and that sometimes were the last to know. This was often the case for families with strong religious beliefs or that were less accepting of the gay community.

Lastly, the actors who most often caused discomfort for the patients were more distant friends, and colleagues, but also parts of the healthcare system that was not Venhälsan. Many patients witnessed of ignorance and bad treatment among other parts of the health care system.

4.3.3. Clustering

Clustering was started in parallel with the interview phase as interviews were conducted, and was completed when all the data was gathered and processed. It was done in many stages and with different amounts of interviews, focus, and content. The interviews were clustered separately for the pre-study, and the main study. The majority of the analysis work was made by saturating and grouping quotes and proofs from the interview stages, first in categories to get an overview of everything, and then later grouped around insights.

Pre-study

From the interviews in the pre-study, initial mapping of what was said and what insights were received was done for each interview. These were color-coded and tagged for each individual so that the information could be traced back to who said what.

As stated in the interview chapter, the information received from the co-workers at Venhälsan was mainly used as a way to understand the context in which the patients are treated, what most commonly is discussed or recurring in the interactions between patients and co-workers. The information was therefore only clustered in one iteration, together with the findings from other interviews in the pre-study, to find recurring themes as inspiration for the needs study of the patient group.

Main study

The data from the respondents' interviews was first unpacked in an empathy map for each interview, as described in the earlier chapter.

The first iteration of clustering contained data from the first four respondents' interviews. The data was mapped similarly to the co-workers, with dividing the data into categories. With the data in categories, a second, more analytical clustering was made, where quotes and information were clustered and connected to different statements.

The third round of clustering was done with the data from patient interviews 5-7, in the same way as the first four. They were held separately to try to avoid bias from the previous clustering and to see if any new themes or categories would arise. After the categorical clustering, a more analytical one was done in the same way as for interview 1-4.

The clustering took time and energy due to the high amount of data to process. Post-its were often moved around and insights were created and dissolved along the process, but in the end the clustering resulted in good insights to base further work on.

4.3.4. Why-how ladder

On top of clustering, a version of a why-how ladder was created to synthesize the material further to see if any new insight could be gained or any new needs could be discovered. Several problem areas were phrased as an answer to “What if people living with hiv´s psychosocial wellbeing are dependent on …”. These were then connected to each other with arrows, the arrow going up answering the question “Why?” and the arrow going down answering the question “What does it lead to?”, see an example in Figure 14. In this way, a complex situation was visualized and structured, see Appendix E for the full ladder. Insight from the interviews and challenges for how to solve the issues could also be added into the why-how ladder.

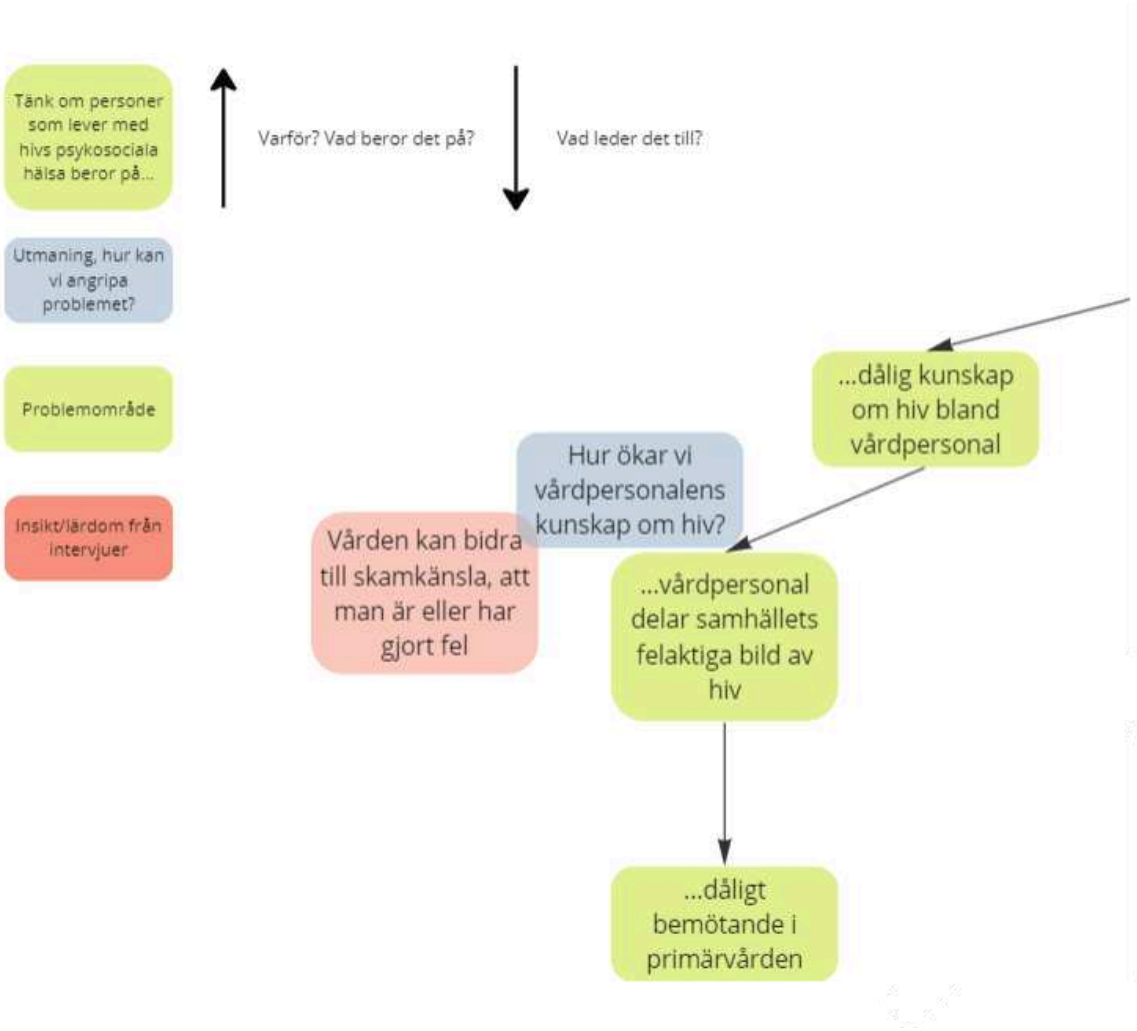


Figure 14. Fragment of the why-how-ladder, with explanations for the colored boxes.

After completing the ladder, the complexity of the issue was visualized in a clear way, and how different problem areas relate to each other. It was also clear that every problem came from the issue that hiv was a deadly disease with a death sentence in the 80s, and that it still is portrayed as that, or that that is societys' perception of it. Several new "how might we …?"-questions were phrased in regard to the problems that led to further insights and that was used during the ideation phase.

4.4. Insights

The last step in the main study was to phrase insights from the study. The insight chapter had two main functions: it compiled information for the continuation of the project for the students, but was also handed to Venhälsan as a concise study of the needs of their patients, see Appendix F for deliverable to Venhälsan.

With the information extracted through the empathy maps, stakeholder maps, the different clustering sessions and the why-how ladder, many insights were found. These insights involved the stakeholders on different levels, in different constellations, and were often

connected to a specific context. While many more insights were found, 11 major insights are presented in this chapter, which are made up of a number of smaller insights. Many of the used quotes were translated from Swedish to English. The original quotes can be found in Appendix G.

Insight 1: Enacted stigma towards people who live with hiv is based on an understanding of the disease from the 80's and 90's

While hiv has gone through major changes since its spread in the 80's, the same depiction of the disease very much lives on today. As stated in earlier chapters, there is in some contexts still a consensus of hiv being something that only affects people from the gay community, who have a promiscuous lifestyle, or people who inject drugs. As this is far from the truth, this creates a dissonance for people living with hiv. As one respondent stated:

- *"First and foremost I want people to know that it is not contagious. Not everyone is a drug addict or prostitute. It is enough to meet someone once. You don't have to be a bad person for that, that you live immorally* [not that prostitutes or drug addicts are bad people or that they are immoral]. *I can't infect someone."* - Respondent 13, man aged 47

This brings up the question of contagiousness, as there were many testimonies from the respondents of being met as contagious, even though a well treated hiv disease is not. This often appeared in subtle gestures, such as people joking about not wanting to share a coffee mug, or more extreme examples such as:

- *"I was out with my colleagues. Everyone knew that I had hiv. One colleague accidentally drank from my glass, and started crying. I had to comfort her and explain that it would not infect her. She worked in health care, so she should have known that."* - Respondent 13, man aged 47

Another aspect of hiv being depicted as something that only spreads among highly sexually active gay people or people who inject drugs, is the question of blame and shame. That many of the respondents felt ashamed for getting hiv, often connected to them "not being careful enough". This results in a culture of blaming people who got hiv for being infected. As one respondent said:

- *"In some way it feels as if I have done something wrong, why did I not use a condom etc."* - Respondent 12, man aged 68

Generally, much of the enacted stigma toward people living hiv could be traced back to this outdated depiction of hiv. Should this depiction change into a more accurate one, many of the problems that people who live with hiv meet today should disappear.

Insight 2: Peer-to-peer support is atleast as important as professional help and support

While this insight may sound as critique of the support offered today at Venhälsan or other instances, this insight is more about lifting the subject of peer-to-peer support as something vital in someone who lives with hiv's relationship to hiv, and something that plays a different role from professional help.

While professional help is important, many of the respondents from the patient group witnessed of instances of peer-to-peer support that helped them greatly with their hiv status. This was often based on it being easier to talk to someone who lives with hiv, since they already knew about what it meant to live with hiv. This sort of "broke the barrier" in communicating and disclosing your hiv-status. As Respondent 12 expressed themselves when talking about a meeting with Posithiva Gruppen:

- *"Many express how important these meetings are, and to meet others. Some might not be so open so for them it is extra important to meet."* - Respondent 12, man aged 68

Or as Patient 6 talks about meeting people at Noaks Ark:

- *"Every time I come to NA and have meetings, people talk about different experiences, and then I relate a lot. Many think the same. It makes me feel better."* - Respondent 17, man aged 24

This automatically opens up for peers to have a great impact on helping others who live with hiv, since they might be the ones that are first disclosed about that persons hiv status, or atleast someone they can discuss their hiv with.

One respondent also explained that while peer-to-peer support is great, it might be less accessible if you don't have anything else in common with that person, and that the support was stronger if you shared other interests or other factors such as age.

Lastly, it is important to recognize that while professional help is great, it should not be compared to a peer-to-peer relationship, as it is not an equal situation. The person living with hiv might seek and receive a lot of help from the nurses and counselors at Venhälsan, but it is not a natural relationship. As one of the counselors framed it:

- *"We counselors become paid life partner."* - Respondent 2, counselor at Venhälsan

Insight 3: An hiv-diagnosis today and in the 80's create a similar life-crisis, even though a well treated infection today is harmless.

As discussed previously in Insight 1, there is still a dissonance between the portrayal of hiv that remains from the 80's and what an hiv-diagnose means today. Some of the respondents who were interviewed received their diagnose before successful treatment obviously viewed their diagnose as the start of the end. As one of the respondents wrote on their customer journey map, referring to getting their diagnose:

- *"May 1987: Dead."* - Respondent 16, man aged 74

Although this reaction was valid for that time, the reactions today can sound similar, even though a well-treated infection today is not lethal.

- *"My first question was: when do I die? That was how little I knew about the disease…"* - Respondent 18, diagnosed in 2012, man aged 40

or:

- *"I thought that I was going to die."* - Respondent 17, diagnosed in 2018, man aged 24

This reaction is yet again thought to be a result of the depiction of hiv from when it was lethal. Respondents who experienced the early stages of the hiv epidemic described how it back then had a greater part in the public discourse, unlike today where it is not discussed due to the accessibility and effect of the treatment available.

During the interviews, it became evident that an hiv-diagnosis is associated with a lot of blame. This blame displayed itself in many ways. Many respondents witnessed a culture of blame regarding how you got the disease. Whether it was the respondents blaming themselves or someone else, it was often brought up as a topic:

- *"I pieced it together and noticed that person had it. He had not tested himself for a while. It made me bitter."* - Respondent 18, man aged 40

Insight 4: After an hiv-diagnosis you are vulnerable and sensitive to impressions

This insight is closely related to Insight 3, as the shock from receiving a diagnosis often puts the patient in a vulnerable position, where different impressions can have a huge impact on the person's relationship with their hiv for a long time to come. This state presents both opportunities and risks which can affect the person's well-being.

As explained in Insight 3, many of the reactions to receiving an hiv-diagnosis was to view it as a failure, or as a source of shame:

- *"In the beginning it was tough, I felt a lot of shame, I felt useless, like a failure."* - Respondent 12, man aged 68

This affected how some of the respondents acted, or reacted to their diagnose. Some of the respondents felt alone or different after receiving their diagnosis, which could lead to them being more isolated than before it. That the feeling of being deviant caused them to be more isolated and lonely.

The main findings connected to this insight however, was regarding sensitivity following a diagnosis. That during the time after a diagnosis, you react stronger to different types of treatment. As one of the respondents expressed themselves in regards to being treated poorly during a medical visit:

- *"I am happy it did not happen right after my diagnosis, because it probably would have broken me."* - Respondent 18, man aged 40

However what was noted among some of the respondents, was that this state following a diagnosis also could be an opportunity to accept and cope with your disease. That good treatment following a diagnosis could benefit the well-being for people who live with hiv for a long time to come. As Respondent 16 explained when talking about sharing their hiv-status to their choir:

- *"All of a sudden I had 30 people supporting me. That was when i started to survive."* - Respondent 16, man aged 74

Insight 5: Intimate relationships are changed when you disclose your hiv-status.

What became evident during the interviews was that hiv affects the way you approach, form and think about intimate relationships. Whether it is just dating, disclosing, or having sex, hiv often affects the situation in one way or another. As Respondent 18 explained what dating with hiv in the beginning was like:

- "*In the beginning it was incredibly hard. It was the best contraceptive you could have."* - Respondent 18, man aged 40

What many of the respondents also expressed was that hiv was hard to hide, or at least that many witnessed of having a hard time hiding it, often because of an urge to disclose when engaging in new relationships. That hiding your hiv was a heavy burden to bear. This made disclosing your hiv information to someone a sign of trust, but also something that became imminent when becoming more intimate with someone. As Respondent 13 explains:

- *"If they become too close to me, I have to tell. If I meet a new partner I want to say it, of course. If I have known someone for three years I have to tell them."*
  - Respondent 13, man aged 47

Many respondents explained disclosing as sharing the load, that when you have disclosed your hiv-status to someone, you both share the responsibility and knowledge about the situation. However, this often altered the relationship in some way. Respondents explained disclosing as a sort of test, or something that changes the situation. As Respondent 14 explains:

- *"Hiv has affected how I date, as it constantly lies in the back of my head. We can be newly in love,* ***but the romance passes and quickly becomes reality*** [when disclosing about their hiv-status]*."* - Respondent 14, man aged 50

Insight 6: People who are living with hiv expect to be treated differently because of their hiv, based on previous/ others experiences.

Because of earlier, often negative experiences, people who are living with hiv experience anticipated stigma. They assume that society has an inaccurate and outdated understanding of hiv and who gets it, referring back to insight 1, which then reflects on their life and life-choices. Because of that, people who are living with hiv are forced to be careful and selective about who they disclose their hiv for as to not be treated differently. One respondent stated:

- "[Hiv] *affects how I interact with others. I am restrictive with letting people in. Don't want to be exposed to prejudices. I opt out of people that I would not have opted out of if I didn't have hiv."* - Respondent 13*,* man aged 47

while another one said:

- *"There is always an assessment* [regarding disclosure] *that I have to make, at least in new contexts. A fear of being judged."* - Respondent 12*,* man aged 68

These expectations can lead to a feeling of isolation. There is a common saying that one bad comment needs ten good comments to neutralize the bad one, and this seems to apply to the stigma of hiv as well. One bad experience often becomes the expectations, not the least because others bad experiences are taking into account as well as one respondent brought up:

- *"The internalized* [stigma] *is there, even if I have not been exposed to something bad I know it happens and there are so many stories from other people that have been exposed and then I internalize their stories and fear."* - Respondent 12, man aged 68

Insight 7: People who are living with hiv expect to be treated differently in a healthcare setting because of their hiv, based on previous/ others experiences.

This insight is very similar to insight 6, but focuses on the healthcare setting. Many of the respondents that testified about stigmatization in healthcare settings had stories from at least ten years ago:

- *"Went there* [to the dentist] *and they had on space-suits. She* [the dentist] *did a sloppy job and there was a terrified nurse next to me with a black garbage bag that they threw all the instruments in that they had used on me. Then she told me they did not have any more appointments for me and that I had to look for another dentist."* - Respondent 13, man aged 47

and

- *"... and I told the paramedics about my hiv and then I hear one of them tell the other that they would give me the shot intramuscularly because it would be so bloody if I got it in my vein."* - Respondent 12, man aged 68

Few of the respondents have talked about incidents in recent years, and some have none, but they still mention that they think about their hiv in healthcare settings:

- *"Hiv is not present in my everyday life, but I think about it when I take a blood test, I tell the nurse."* - Respondent 19, woman aged 39

This is an example of when enacted stigma becomes anticipated stigma, since many of the respondents, as in insight 6, took other people's experiences into mind. Additionally, the patients are already in a vulnerable position and if they have to answer questions about how

they attained hiv, it only reinforces the feeling that they have done something wrong, as one of the nurses explained:

- *"In my experience it has become better, but they* [people living with hiv] *still get the question "how did you get your hiv?" that only people not working with hiv asks."* - Respondent 5, nurse at Venhälsan

The respondents also talks about little knowledge about hiv and how medical staff thinks everything is connected to their hiv:

- *"When I go to smaller health care centers (vårdcentral) it feels as if they are less knowledgeable. They try to connect everything that happens with me to my hiv."* - Respondent 18, man aged 40

Migrants who have come to Sweden might also bring anticipated stigma with them from other countries, as one respondent experienced from a non-swedish doctor:

- *"You have yourself to blame* [for your hiv] *because you have chosen to live this life* [being gay]*."* - a non-swedish doctor to respondent 15

Insight 8: People living with hiv understand that they are not contagious, but are afraid of infecting other people with hiv.

As stated, there is a common misconception that hiv is infectious, even with antiretroviral therapy, and that the person living with hiv has to inform other people even though they are U=U. This almost forces the person living with hiv to disclose their status to other people because they want to make it "correct" from the other person's point of view. It is probably also to avoid making the other person angry if they find out later, because they think a person living with hiv have to disclose it.

The internalized stigma shows itself when many respondents talked about their sex-life:

- *"It* [being intimate] *was very hard in the beginning. It was almost the best contraceptive I have ever gotten."* - Respondent 18, man aged 40

and the fear is still there even after years of being U=U:

- *"It is in the sexual regards it* [hiv] *has been and is a problem. I have sex in a way so that I can not transmit. It is important to not transmit."* - Respondent 16, man aged 74

Almost every respondent stated that they tell their sex-partner about their hiv, even temporary ones. One theory is that they want to resign some of the responsibility and share it in case of a potential transfer, which supports the insight that they still feel that they could transmit even though they know they can not.

Insight 9: "Survivors" have different needs and challenges than people that receive their diagnosis today.

"Survivors", referring to people that received their hiv before effective treatment, face a whole set of different challenges in their everyday life. Firstly, they often carry trauma from the 80s and 90s and what hiv was then, see insight 1. On top of the emotional baggage, they are also affected practically today, for example, many of them prepared to die and quit their jobs and sold their homes when they received their diagnosis:

- *"I'm going to die 95/96 anyway. I quit* [my job]*."* - Respondent 16, man aged 74

or

- *"It affects how you think about money, tomorrow, savings,* [you] *should not save for your pension, or an apartment, you live for the day."* - Respondent 14, man aged 50

This leads to them having to deal with this later in life, which is much harder, and many of them need help with practical things like a pension and housing.

Furthermore, hiv adds a worry about their physical health when they are starting to get older, as respondent 12 said:

- *"I'm getting older and I see other hiv-positive end up with things* [illnesses]*."* - Respondent 12, man aged 68

The worry might also be because they have either been very sick before they received treatment for their hiv, or they worry about the side effects of having been on antiretroviral therapy for so long:

- *"Some drugs have different side effects that you can't escape and I know that and I have to live with that."* - Respondent 15, man aged 45

Insight 10: Hiv is a big thing both for the person that has it, but also for the people close to the person living with hiv.

For most people living with hiv, it is a big part of their life, and many ponder if they should tell the people close to them or not. Many think it is hard to get really close to someone without telling them about their hiv, and it is a sign of trust to tell someone. As respondent 13 stated:

- *"You have your chosen family."* - Respondent 13, man aged 47

Moreover, people close to the person living with hiv wants to be trusted with the disclosure, and wants to take part of the other person's life. If not, they might feel disheartened that the person living with hiv does not want to open up to them:

- *"I told my sister and she was mad because I had told another friend before I told her."* - Respondent 16, man aged 74

Disclosing your hiv can be hard both for the person living with hiv, but also the one finding out about it. However, this is mitigated if the person being disclosed to has prior knowledge about what hiv is:

- *"I was pretty knowledgeable* [about hiv]*, so it wasn't very dramatic*." - Respondent 20, man aged 57, partner to someone living with hiv

Insight 11: Venhälsan offers a bigger support apart from the medical aspects for people living with hiv, compared to the rest of the healthcare.

During the interviews, all respondents said they felt safe and taken care of at Venhälsan, something that really differentiates Venhälsan from other healthcare places, as respondent 13 put themselves, referring to visiting another health care center:

- *"I didn't feel as safe, they weren't unsafe, but I feel more safe at Venhälsa, considering my sexuality. It just feels easier."* - Respondent 13, man aged 47

Venhälsan has a very good reputation and the patients expect them to have updated information and knowledge:

- *"It feels good* [going to Venhälsan] *because they are used to it, they have seen it before."* - Respondent 18, man aged 40

Consequently, Venhälsan has a lot of influence and can have a big impact since they have the trust of the patient group, which is something that can be utilized.

5. Concept development

This chapter explains the process of concept development. Ideation, with and without involving stakeholders, the generation of major ideas and concepts, creation of trigger material to receive feedback from respondents, and the development of a final concept is presented.

5.1. Ideation

The ideation stage mainly consisted of a workshop involving staff at Venhälsan, working together to create ideas from gathered insights from the analysis. It also involved brainstorming without actors, building on the ideas generated during the workshop.

5.1.1. Workshop with staff

Before structuring the workshop, the 11 gathered insights were presented to two counselors at Venhälsan. The purpose of this presentation was to extract a few questions to be discussed during the workshop at Venhälsan, since there would not be enough time to present all 11 in detail. After discussing with the counselors, the following six insights were brought to the workshop:

- Insight 2: Peer-to-peer support is atleast as important as professional help and support

- Insight 3: An hiv-diagnosis today and 40 years ago create a similar life-crisis, even though a well treated infection today is harmless.

- Insight 4: After an hiv-diagnosis you are vulnerable and sensitive to impressions

- Insight 9: “Survivors” have different needs and challenges than people that recieve their diagnosis today.

- Insight 10: Hiv is a big thing both for the person that has it, but also for the people close to the person living with hiv.

- Insight 11: Venhälsan offers a bigger support apart from the medical aspects for people living with hiv, compared to the rest of the healthcare.

The workshop with the staff at Venhälsan was done during their 1 hour lunch break, and thus was adjusted to fit their pressed schedule. In total, there were 6 participants, representing all of the professions that the hiv-patients meet: 2 counselors, 2 nurses, 1 assistant nurse, and 1 doctor. They were assigned seats in two different groups, making sure the different professions were divided.

The workshop started with a quick introduction of the project and the work thus far, and then followed a brief of the insights that had been chosen beforehand by the counselors, see Figure 15.



Figure 15. Background information about the project being presented before the ideation workshop.

The insights were presented with the aim to bring new perspectives to the co-workers, priming them for the following ideation session. To make the discussion of the insights more concrete, following the presentation of the insights, 6 different “How might we?”-questions were presented as a base on which the ideation would build:

- **How might we** facilitate disclosing your hiv status?
- **How might we** make receiving an hiv-diagnose less dramatic?
- **How might we** support patients during the time after their diagnosis?
- **How might we** update the understanding of hiv in society?
- **How might** Venhälsan make peer-to-peer support more available?
- **How might we** make other health care clinics feel more like Venhälsan?

Each of the teams were allowed to pick two of the questions to discuss, as there was not enough time to discuss them all. After initially ideating individually for a few minutes, the groups placed their questions and ideas on a wall, and began discussing. They took turns

presenting their own ideas, and building on each others. After 15 minutes, both groups gathered, and took turns presenting their questions and ideas to the other group, inviting them to collaborate and discuss, while the moderators took notes about what was said. See Figure 16 for group discussion, and Appendix F for the different clusterings.



Figure 16. The two groups discussing ideas about two questions.

To finalize the workshop, each participant was handed a placard on which to write. They were instructed to create a vision for 2034 (10 years ahead), and write a headline about something new happening at Venhälsan, that would increase the psychosocial well-being for people living with hiv. This exercise would build on what had previously been discussed during the workshop, but this time without the hinders that can be found today. A more wishful thinking. This exercise created many interesting and innovative goals. See front-page in Figure 17, or Appendix H for all of the front-pages.



Figure 17. Newspaper scope in 10 years

Takeaways

The workshop went according to plan and received positive feedback from the participants. The time schedule was met without it feeling rushed. As stated, the group was mixed with professions, but also with gender, age, and length of work experience which created nuanced discussions. The format with two smaller groups of 3 worked really well, and everyone easily had a say in the discussions, which was also something that the participants mentioned after completing the workshop.

As mentioned, the participants had a few minutes to brainstorm ideas on their own before they had around 15 minutes to discuss their ideas in the smaller groups, and this was plenty of time for them to concretize all of their ideas. Many of the ideas were very realistic and practical, so the moderator asked follow up questions and nudged them into a freer mindset. For example, one participant put “be available for questions” on a post-it, and the moderator then asked “how can you be available?” to facilitate discussion and ideation. During the bigger group discussions in the end, they were able to build on each others’ ideas without much interference from the moderators.

After the workshop was complete, all notes and post-its were processed and the ideas and thoughts were put into Miro. Most ideas were quite realistic and feasible for Venhälsan to

implement straight away, which was to be expected. All ideas are presented in Appendix F. After going through them, the ideas could be bundled into 5 main ideas or categories of ideas:

1. Spreading information to different stakeholders in different ways.
2. Make peer-to-peer support more available through patient organizations.
3. Involve people close to the person living with hiv to either support the person living with hiv, or the person close to them.
4. Make the contact between patients and Venhälsan easier.
5. Spread Venhälsans work and model of work to other instances of care.

5.1.2. Brainstorming

After the workshop with the staff at Venhälsan, another ideation session was carried through within the research group. In addition to the "How might we?"-questions presented to the staff at Venhälsan, a few new questions were added after discovering some important areas that did not get covered during the workshop:

- **How might we** make it less lonely to live with hiv?

- **How might we** make sure that an hiv-diagnosis does not affect one's sex-life?

- **How might we** expand the "safe-zone" for people living with hiv?

All of these questions were then brainstormed for 10 minutes each, which was plenty of time. The swiftness of ideation was a little surprising, since a lot more time had been put aside for it, but it was noticeable that ideas ran dry after about 7 or 8 minutes, and the ideas that were created during the remaining time were forced. Some ideas were very similar to ideas that the staff at Venhälsan had created during their workshop, while some were more visionary or bigger, see Appendix F for all ideas.

5.2. Concept generation, first iteration

After the workshop with co-workers at Venhälsan and the brainstorming session, the ideas were clustered, combined, and tweaked to create new major ideas. The following chapter will present some of those major ideas and which idea were selected for further development.

5.2.1. Major ideas

The major ideas that were created for this thesis were deliberately chosen to be more complex than some of the ideas that Venhälsan could easily implement on their own, for example a mailbox in the waiting room for questions. Some of the most promising ideas are presented in this chapter.

Venhälsan certification

The first major idea was to create a Venhälsan Certification for different healthcare institutions. This was both to spread the knowledge that Venhälsan possesses, but also to ease the worry for people living with hiv when they seek medical help somewhere else, inspired by hbtqi certifications. This could be done in several ways, but some ideas were to let Venhälsan hold lectures for the entire wards, or to have some Venhälsan-representatives that had completed training in every ward.

Venhälsan app

The second major idea was to create an app, gathering all the information and resources that you would want to have access to when living with hiv. This could both support the person living with hiv after they receive their diagnosis, but also follow them through their life, similar to a period-tracker app, or an hormonal tracker app. Some functions could be a chat with the staff at Venhälsan, quick access to the chart of your virus-levels, FAQ, contact with other people living with hiv and so on.

Information campaign

The next major idea was created to spread information about hiv, with an aim to update the picture of hiv from the 80's and 90's. To engage more people, this could be created as an info-center or pop-up center around the city, or at the entrance at Sös, ensuring many different people would go by it. This could be seen as an alternative way of spreading information instead of the more traditional use of media.

Help with disclosure

Another major idea was to help the person living with hiv to disclose their diagnosis to a person close to them. This could be facilitated by for example a template for a conversation the person living with hiv could follow, or an activity that could be done together, leading up to the disclosure. The aim with this idea was to support the person living with hiv, give them tools, and create a safe space for disclosing.

Post-diagnosis care package

The last major idea was created to support people living with hiv right after their diagnosis and entailed a care-package. This kit could contain information that is important right after a diagnosis, more tangible things like a dosette box or an emotion-journal, or other resources. The kit could be designed in many different ways and incorporate many different aspects from the other concepts, such as a game for disclosure for example. The aim with the kit was to find a way for the newly diagnosed person to continue working on coping with their diagnose at home, when they are not at Venhälsan.

5.2.2. Major idea selection

All major ideas were discussed within the project group, as well as with supervisors at Sös Innovation, and lastly with a counselor at Venhälsan. This was to take into consideration that the idea needed to fit a thesis project, the idea needed to fit into a healthcare setting, and lastly fit how Venhälsan works and what they can put into the concept.

The first major idea, with Venhälsan Certification, held a lot of potential to create a positive change in the bigger picture. People living with hiv would not be affected directly, but after some time the certification could create a great change in the healthcare setting. Despite this, the idea was hard to implement since Venhälsan currently does not have any time or money to educate other instances since it is not part of their mission. It would also require a lot of time and effort from Venhälsan to implement the certification.

The second major idea, a digital app, could include many different smaller ideas. Similar to Venhälsan Certification, it was a costly idea since the app had to be developed and maintained, and Sös does not currently have an app platform. It would also be hard to finalize the app for this project, and a lot of work would be left for future development. Nevertheless, the idea held potential for subjects that could be included on Sös webpage.

The third main idea, an infocenter, was similarly to the Venhälsan Certification, not going to affect the people living with hiv directly, but hopefully decrease the stigma about people living with hiv. Nonetheless, the reach would only be so big since it was stationary, and depend on how long it will be up for. It was a visionary idea that again would be hard to implement in this project.

The idea to help people living with hiv to disclose it to people close to them would directly affect the target group. Nevertheless, it was an idea that did not hold any direct connection to Venhälsan, apart from them being the dispatcher. It was also an idea that was easily included into other ideas.

The major idea that was finally chosen to move forward with was the idea of a care-package. This idea would fit into Venhälsans current work, while at the same time not add a lot of extra work for them. Additionally, it would be relatively cheap and easy to implement in the public sector. The kit could also intervene at a moment where everyone who is living with hiv have similar needs, and it is a time where intervention can be very effective. The counselors at Venhälsan thought this idea would fit them best, and there was also great potential for the care-package to be completed within the frame of this thesis.

5.3. Concept generation, second iteration

This chapter explains the development of the final concept, with further involvement of experts and co-workers at Venhälsan through interviews, and the use of trigger materials in interviews with respondents living with hiv. Lastly, the process behind the design is

explained. Additional stakeholders that were interviewed in this chapter can be found in Table 3.

Table 3. Respondents from the second iteration of concept generation.

ID	Time [min]	Location of interview	Profession	Description
Respondent 21	60	Sös	Counselor	Internal medicine and intensive care unit at Sös
Respondent 22	60	Sös	Psychotherapist	HBTQ-hälsan at Sös

5.3.1. A state of crisis

As the counselors at Venhälsan described that receiving an hiv-diagnose often puts the patient in a state of crisis, it became important to take into consideration different recommendations when treating people in a crisis.

Elin Svedjetun, licenced healthcare counselor at the internal medicine and intensive care unit at Södersjukhuset, who has worked with people in crisis for 20 years was interviewed to discuss what needs a person could have in a state of crisis, that could be addressed within the boundaries of the package. According to Elin, during a time of crisis you need help, support, counseling and the opportunity to think out loud, but also to normalize and validate. As Elin put it: *"It is normal to react to an un-normal situation"*. She also pointed out that it is important to help the person regain control over the situation. To give them information regarding what lies ahead, what the situation means and who they can turn to for aid. Lastly she said that: *"It is important to tell them that it will not always feel the way it does now"*.

In Region Stockholms' guidelines regarding treating people who are in a state of crisis (2023) they point out the importance of giving out written information as a complement to the verbal information patients get from their health care providers: "*Written information is very valuable and good to provide. The person you meet may quickly forget or may not be able to take in what you've said at that moment"*.

5.3.2. Themes

Developing a kit with several parts, chapters or "themes" enabled solutions that could cover many problems that were identified in the pre- and main study. Therefore, the insights from the Analysis chapter were put together into six themes which would be processed in the package.

Although the insights covered much more, the package would focus on aiding the patients during the period after their diagnosis, when the patient might be in a state of crisis, and gather them into one place. Every person deals with the situation differently, so the idea with

the kit was to work with questions that were shared by as many as possible. Dividing the kit into themes would let the person choose to read and process different modules that they are interested in or feel that they need to process. The themes would be put in the order it seems best to process them in, but again, the user can choose to process them in any order they want. One important thing to remember is also that the kit would not replace the psychosocial support that Venhälsan offers at the clinic, but act as a support for when the person diagnosed with hiv gets home, where new needs of support appear.

The themes were created from the gathered insights from the needs study. The content and ideas were inspired by the main-study and existing material from Venhälsan and Patient organizations.

Introduction and journal

To make the patient understand what the care-package is, there would be an introduction containing the background, purpose and structure of the package. To allow the patient to reflect on the package, to write down thoughts that might arise during the themes and to give them an incentive to stay active during their treatment, the introduction would contain a journal. The journal would be their place to reflect, and could also act as a conversation starter when visiting Venhälsan.

Theme 1: About hiv

The first theme would process some of the information one would want right after diagnosis to cope with the potential emotional crisis that might happen. As interviews have stated, the person diagnosed with hiv gets a lot of information at Venhälsan, but it can be hard to remember everything by receiving only verbal information. This theme would repeat a lot of what has been said by the nurses and doctors.

As the staff at Venhälsan mentioned during interviews, it was important to keep the focus “here and now” when receiving an hiv-diagnose, and to keep streams of information to the patient as concrete as possible. The content of theme 1 was therefore supposed to matter-of-factly present the person diagnosed with hiv with information that is correct and necessary for them to know in the beginning. The solution became to offer the patient a brief summary of the core aspects of what hiv is: that it is a chronic infection, what the infection does, how the infection acts when it is untreated versus treated, how it is treated, how it is not transmitted versus transmitted, about antiretroviral therapy, but also about the legal obligations that you have when your infection is not yet treated. The information would be gathered from credible sources, and combined with existing information material already provided by Venhälsan.

Theme 2: Venhälsan

The second theme would be centered around Venhälsan and what one needs (and wants) to know about the ward. This was to introduce the person diagnosed with hiv to the healthcare setting they will be in, and to give an introduction and overview of the treatment they will receive. Similarly to theme 1, this theme is predominantly informative.

What stood out about Venhälsan during the interview stage was that it was a safe space for many patients, where they were acknowledged and treated well. This is particularly important in the early stages of an hiv treatment, since the patient is at their most vulnerable. This theme would work to create a sense of belonging, familiarity, and continuity for the person living with hiv towards Venhälsan. The goal is to make them feel that they have a team of professionals supporting them and being there for them.

As mentioned in theme 1, it was important to provide the patient with tangible information about the treatment. This involved subjects such as the structure of the treatment and appointments, what type of medication they will receive and what some common side effects are. It could also be important to establish a routine around taking the medication. The purpose was to give the patient a good enough overview to allow them to feel in control, while at the same time not make them feel overwhelmed from the information or the long time spans connected to hiv (since the infection is chronic). As the counselors at Venhälsan once again said: “It is important to focus on here and now”.

Ideas that were decided for this theme was to start out with something along the lines of “Meet the team” which would be pictures, names, and contact information to the nurse, doctor and counselor assigned to the person diagnosed with hiv. This is to let the patient know that Venhälsan has control of the situation, and to early on establish Venhälsan as a safe space. The theme would also provide the patient with an overview of the treatment through a 1-year timeline, which would show the general visits one has at Venhälsan during that period, what staff they will meet and what procedures will be done. On top of this, the timeline would also display a general virus curve, to demonstrate to the patient that their infection will go down.

Theme 3: Knowledge

Theme 3 was developed to help the person diagnosed with hiv with their potential worry about the infection. The worry was identified from the interviews and insight to spring from ignorance or misconceptions about hiv and the point of the theme was therefore to educate the person living with hiv, and potentially people who are close to them. As stated in the insight chapter, much of the depiction of hiv among the general public is based on what the infection was in its early stages, before active medication. This understanding has created many misconceptions about hiv, how it is transmitted, what a life with hiv is like etc. This is often the root cause of the hiv stigma, and affects the people who live with hiv greatly.

To address this, the idea was to talk about truths and myths about hiv in a fun and light way through creating "flashcards" with different statements about hiv. The statements could be common misconceptions about hiv, frequently asked questions, myths about the infection and facts that are good to know. The statements would be on one side of the card, and the other side would explain if it is true or false, and why that is. Flashcards are also a very common and effective study technique, and will be easier to digest than just written information. Through actively recalling information, researchers claim that you are 150% better at retaining information compared to passive studying (Karpicke et.al 2008). To make the statements more powerful, the idea was to take actual statements and questions from real people who live with hiv. In this way, the risk of projecting stigma on the person playing is reduced, and the questions might become more relevant.

In addition to the flashcards, there could be links to other trustworthy resources that Venhälsan recommended such as Folkhälsomyndigheten, Posithiva Gruppen, Noaks Ark, or 1177, if the person diagnosed with hiv wanted to dive deeper.

Theme 4: Recognition

For many of the respondents, the hiv diagnosis resulted in them feeling alienated, which for some still lasts today, many years after their diagnosis. Much of the anticipated and internalized stigma they experience resulted in them feeling different from people who do not live with hiv, and this can result in them living more isolated. This is where peer-to-peer support often is brought up, as a means to deter the feeling of isolation or alienation. Peers play a large positive role in other people who live with hiv's lives, as they often share similar experiences. So, much of the positive impact a peer can have is to *recognize*, and therefore theme five was about recognition.

To connect this to the care package, ideas were discussed about how to allow the reader to take part of someone else who lives with hiv's life. This could be to either read about their experiences with hiv and topics connected to it, tips about how to cope with the diagnosis, or more mundane things such as jobs, family, dreams. Different ways of showing to someone newly diagnosed that hiv often results in a normal life.

The solution that was brought forward was a series or portraits of people living with hiv. These portraits would contain a picture, some formalities and a text written by a real person living with hiv. With an emphasis on informality, the portrait would be shaped as a sort of letter, sent from someone who lives with hiv to someone newly diagnosed. This informal format would hopefully play on a more personal level than work as an informative text.

On the topic of recognition and peer-to-peer support, it was suitable to bring up patient organizations, and their efforts to facilitate the interactions between people who live with hiv. An idea was therefore to somehow include them in the theme, either through reference or by more active involvement.

Theme 5: Disclosure

One of the more complicated themes that was present throughout the project was disclosure. Co-workers at Venhälsan, researchers, patient organizations and respondents living with hiv all seem to have different views and experiences with disclosure about hiv, but there were some patterns. Firstly there was a consensus among health care co-workers and patient organizations to not advocate for any type of disclosure, because of the risks it could bring if it was not received well. Associate Professor Maria Wikander at Karolinska Institutet also explained that disclosing diseases with less stigma, such as diabetes is more advocated for in psychiatric treatment, but that the same can not be said for people living with hiv.

During the interview stage, disclosure was a big topic. Many of the respondents viewed disclosure as a big risk, as many experienced great anticipated stigma. It also became clear that it was a big step in a relationship with someone to disclose your hiv status. What was interesting was that there were very few, if any stories of disclosure resulting in a bad experience. The only exception was when disclosing their hiv status to a new sexual partner, where the reactions often were poor. Most respondents also compared disclosing your hiv status to coming out as gay. This comparison drew some attention to what is recommended when coming out as gay, since it was explained as a similar situation. UMO, a Swedish website about sex, health and relationships for youths, gives these tips about coming out (UMO, 2024):

- Tell a few people at a time, to begin disclosing to someone you trust and who you think will react well.
- Write and tell, that if it feels easier it can be recommended to send a letter.
- To say it as if you expect a positive reaction.
- To say it as if it was something obvious.
- Reach out to peers or patient organizations to ask what tips they have.

An interview with Magnus Karlsson, psychotherapist and licensed healthcare counselor at HBTQ-hälsan at Södersjkhuset, was held on the topic of coming out, and disclosing your hiv status, as many of the respondents compared the two. Magnus talked about the increasing acceptance of gay people in society, problematizing the fact that it is expected of someone being disclosed to react positively. That this has a tendency of not giving the person opportunity to think their situation is troublesome. Magnus therefore pointed out that if you want to tell someone about your hiv, it is important to not only disclose your status, but also what that means for you, how you are dealing with it and what needs you have connected to your situation. He also spoke about structuring the way you disclose, to set boundaries for yourself. What am I obliged to tell? What do I want to tell? To let the person disclosing choose where the conversation will go, and where it will end. Disclosing can require talking about more than once. Lastly he wanted to prepare the person disclosing about any reactions they might get. "*You're so observant that you draw conclusions that may be a bit off or that you know nothing about, that rarely is hostile",* that you should approach telling someone about your hiv with an open mind.

The user study showed that there was a benefit in disclosing to someone you feel comfortable with, that is not a new sexual partner, and that the risk of being met in a bad way is minimal if the person already is somewhat close. Ideas for this theme therefore revolve around facilitating disclosure in some way, as many respondents explained that it could be difficult to find an appropriate time or setting for disclosing to someone. Ideas discussed were for instance: playing a game with different questions leading up to disclosure, handing over an artifact explaining the patient's hiv status or reading a book together, ending up in disclosure, similar to the book *Something I want to tell you,* written by Arnesen in collaboration with Noaks Ark (Noaks Ark n.d.), directed at children aged 7-9 years old to read together with a parent living with hiv, ending up with the question "Do you have something you want to tell someone?" where the parent discloses their status.

Theme 6: Acceptance

Throughout the interviews with people who live with hiv, there were many who brought up negative feelings regarding getting the diagnosis. Many witnessed of harsh feelings towards themselves, such as shame, blame, guilt, while sometimes also including others, blaming them or feeling anger towards them. While the first three themes focus on mitigating the impact of an hiv diagnosis through knowledge and defusing the situation, theme 6 was more about acknowledging the feelings that are there, and how to cope with them.

To gain a better understanding of how to accept living with these feelings, Agnes von Schreeb, a PTP psychologist was interviewed. She referred to Acceptance and Commitment Therapy (ACT) as a common tool used in these situations. As Agnes described it: "*ACT is about letting yourself feel the emotions you feel, without acting on them. The best way to reduce the intensity of a feeling is to feel it, and to practice feeling it while for example going to work"*. Agnes also referred to literature by Russel Harris M.D., who explains the practice of ACT as: "*In ACT, there is no attempt to try to change, avoid, suppress, or control these private experiences. Instead, clients learn to reduce the impact and influence of unwanted thoughts and feelings, through the effective use of mindfulness* (Harris, 2006).

A study at IAU University in Tehran in collaboration with the patient organization Tehran Positive Club was done using ACT for people living with hiv, who suffered from mild to severe depression connected to their diagnosis (Fazeipour et.al, 2018). The group was divided into a group who received eight, sixty-minute ACT sessions, and a control group who did not receive any therapy. The results clearly showed that the people going through the ACT treatment showed lower levels of depression, while the control group did not. The study assumption that ACT is an effective tool to reduce depression among people living with hiv was confirmed with 95% confidence using ANOVA, and therefore proves that it is an effective tool to combine with feelings and hiv.

To put this in the context of a care package, there were some ideas that were discussed. Agnes continued by explaining that a good way of viewing it is to not allow the negative feelings to be "in your face", but instead try to carry you with them on your shoulder. To

refocus the feelings, or placing them somewhere other than right in front of you. Here some ideas could be for the patient to reflect on their feelings, write them down and imagine that the paper is now the place where these feelings live. Another approach was to stand in a room and look in one direction while feeling all the negative feelings, then turning into another direction and projecting all the positive feelings that way.

The idea that seemed most approachable for different kinds of people were to write down ones feelings to help the person diagnosed with hiv to reflect and figure out what they were really feeling, and then depending on how much commitment they were ready for put that letter in a envelope or for instance fold it into an origami figure.

5.3.3. Trigger material

Two types of trigger material were created to trigger a response from the patient group about the different ideas, the first being trigger materials to discuss the themes with respondents, and the second a question box that was put in the waiting room at Venhälsan.

Feedback interview with respondents

After the first round of ideation with the different themes, it was important to get the opinion from the respondents to make the solutions as relevant as possible. To make this happen, the respondents from the interview stage were asked to participate in a digital interview, where the ideas would be presented, and where they would be given the opportunity to give feedback and express their opinions. A total of three interviews were held with Respondent 13, 17 and 18.

For the first theme, the respondents were presented with the topics that would be discussed. This involved what hiv is, what it does to your body, how the infection is when treated or not treated, how it is transmitted, how the treatment works and also briefly about the legal obligations that applies before the patient is undetectable. They were then asked if they would like to add anything to the list. The respondents were pleased with the topics, and emphasized that they wanted brief, easily understood information. One respondent requested some statistics about treatment and hiv, since he wanted to level his expectations with reality. Another respondent also asked about general tips about what actions you can do yourself, such as stop smoking or keeping an active lifestyle.

The second theme had more visual trigger materials, and was presented in two parts, the first part referring to the idea to “Meet the team”, with the presentation of staff at Venhälsan. The visual trigger is shown in Figure 18.

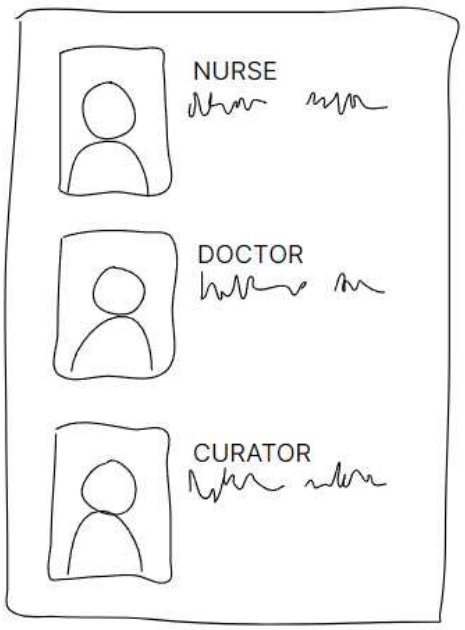


Figure 18. Visual trigger for introducing the staff at Venhälsan.

All respondents reacted positively to seeing the faces of staff at Venhälsan. One respondent explained that they felt unsafe during a time when they met different staff every time they visited Venhälsan, and explained the importance of continuity for them.

The second trigger was an outline of the timeline discussed in the previous chapter. The timeline contained different time stamps for visits at venhälsan, and what the visits were about. There was also a curve to represent a general hiv virus curve. The timeline is displayed in Figure 19.

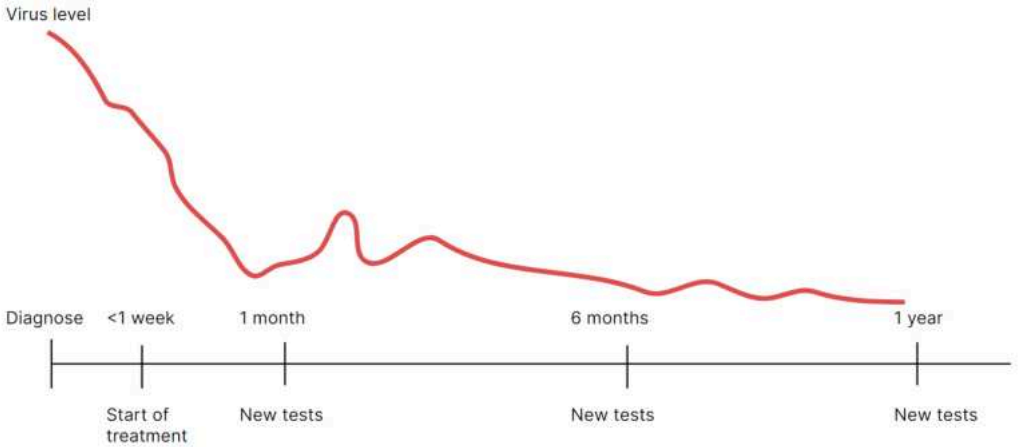


Figure 19. A curve of a general case of hiv virus levels after start of treatment.

While this curve did not represent the actual curve of the patient, the respondents still believed that it gave them hope to see that the virus often reaches undetectable levels after a month. That *"I am atleast not heading towards dying"* as Respondent 18, aged 40, said. One

respondent claimed that they were more calmed by viewing the CD4-curve, that is the immune system, rather than the virus curve.

For the third theme, the respondents were shown an example of the structure of the flashcards, and were explained to that the statements would be from real people living with hiv. The visual trigger is shown in Figure 20.

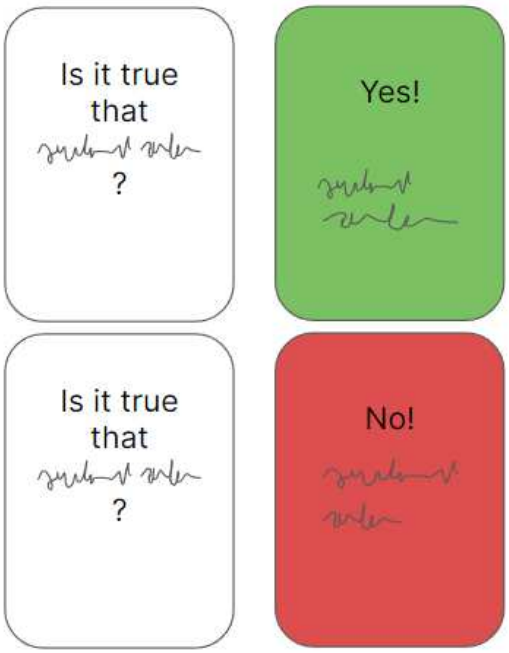


Figure 20. Visual representation of the structure of the flashcards.

The respondents witnessed of having many questions about hiv that felt "stupid" or "silly", and therefore reacted positively to having similar questions on the flashcards. This in combination with the playful format of flashcards was praised by the respondents. As Respondent 18 put it: "*It is a serious disease, but everything about it does not have to be dead serious"*. The respondents also reacted positively to the fact that the questions would spring from real people who lived with hiv, since it would probably make a newly diagnosed person feel more normal with having similarly "silly" questions.

For the fourth theme, the idea of the portraits of people living with hiv was explained to the respondents, see Figure 21. The portraits were described as letters from people living with hiv to newly diagnosed people, and would contain information about their hiv, how long they have lived with it, what it felt like getting it and what it is like living with it today. It would also depict their everyday life, with questions about occupation, interests and dreams. The respondents all reacted positively, and emphasized on the importance of not feeling different. That a "normal" person can also get hiv. It was therefore important for the respondents to have a wide array of portraits of different people. Another important aspect that was brought up was that you as a newly diagnosed person want to see that everything eventually will be ok, and that you can live a normal life despite having hiv. This would then be a good way of "toning down" the seriousness around living with hiv.

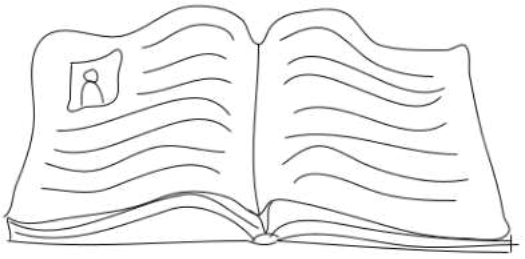


Figure 21. A sketch of a portrait.

The fifth theme, about disclosure, presented some ideas about how the disclosure could be facilitated, for example with a game or an artifact, see Figure 22. Respondents repeated how contextual the disclosure is, while also being cautiously optimistic about a game to help the person diagnosed with hiv if they felt that they needed an extra push. All of them agreed that the disclosure to friends and family should be light-hearted, while the disclosure needs to be serious if they tell a sexual partner. They also mentioned that it could be good to have material with information for the person they are telling, so they do not have to answer all the questions the other person might have.

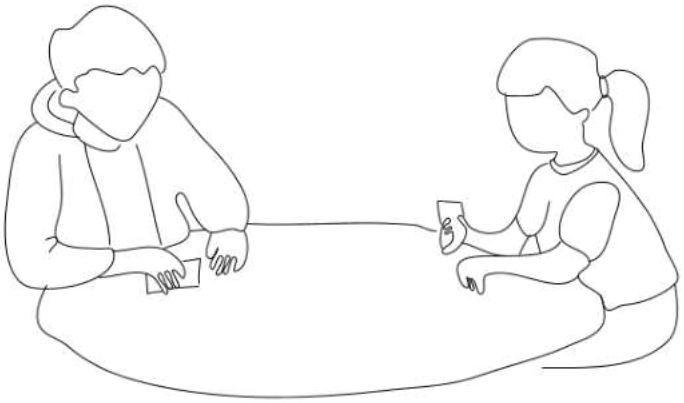


Figure 22. Simple sketch for Theme 5.

For the last theme about acceptance, the respondents were presented with some pictures describing how one could write a letter and put it into an envelope or fold it into origami, see Figure 23, to deal with the potential strong emotions. All of them were very positive about the idea of writing about their feelings. One mentioned that it had been good for them to dwell on their feelings over and over after their diagnosis, while another mentioned that they already write a diary and really think it helps them manage their feelings. The feedback regarding the origami folding was both good and bad. One respondent preferred to just use an envelope, while another thought the idea was very nice and that they would have really benefitted from the idea.

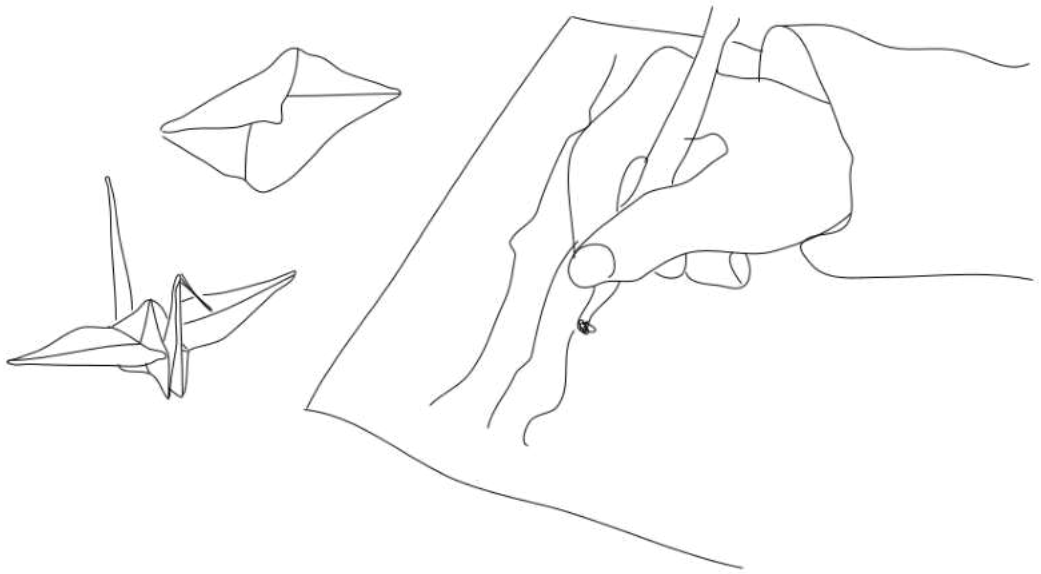


Figure 23. Explaining figures for Theme 6.

The general feedback about the package was very positive. One respondent mentioned that the reflection journal was very good to bring back to Venhälsan.

Question box

To involve more people living with hiv and make sure the package would actually help people diagnosed with hiv, a question box was put in the waiting room at Venhälsan, see Figure 24. This was to gather material for the different themes in the package in a light and easy way. The text on the front and back reads “Think back! Are you living with hiv? Take a note, answer the question, and put it in the box.” with some information about the project and what the answers were going to be used for. The colorful notes on the side had different questions on them, corresponding to different themes.



Figure 24. The question-box put in the waiting room at Venhälsan.

For Theme 4: Ignorance, the respondents expressed during their interviews in chapter 4.2, that it was important that the questions on the flashcards were from actual people living with hiv, to help with the feeling of loneliness and alienation, and to see that other people had similar or even "more stupid" questions than them. The blue note reads "What "stupid" questions did you have when you received your hiv diagnosis?", see Figure 25. The formulation of "stupid" questions were discussed if it was necessary, but the formulation was to try to make it clear that it was not just though-through, serious questions that could be written down. Apart from the question-box, the previous respondents were also asked if they remembered that they had any.

The pink note reads "Have you told anyone about your hiv? Describe how it played out." and was to gather material for the theme Disclosure. Since it is such a tricky subject, and very contextual, the idea is that if a lot of different stories are told, the person diagnosed with hiv can get inspiration for how they practically can proceed, while also reading about real stories and reactions and therefore set their expectations. Together with the tips, they can then decide if, how, and when they want to disclose their hiv.

The answers to the yellow note could be used for both the theme about Ignorance and About hiv, and reads "What did you wish that you knew when you received your hiv-diagnosis?". As stated, these answers could be used to make sure all relevant information is brought up in the first theme, and also if the flashcards could be complemented.

The last note, the purple one, reading “What helped you the first time after your hiv-diagnosis?”, was not meant for a specific theme, but to gain insights from other people living with hiv apart from the interviewed respondents. The idea was that this could complement the whole package, and thus take into account more peoples’ voices to be more collaborative and focused on the real situation.

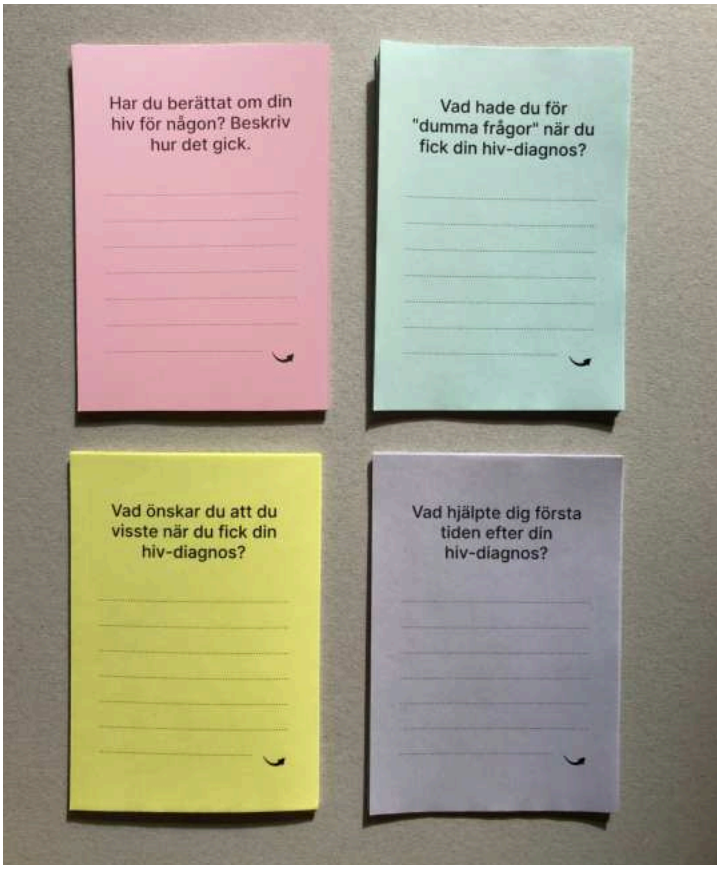


Figure 25. Notes to answer and put in the question-box.

The question box was left in the waiting room for 20 days before it was gathered and processed. There were 12 answers, mostly equally divided between the different questions. Some answers were more useful than others, while others were a bit more off topic or could not be used for the package, as seen in Figure 26. Taking into consideration how many patients go in and out of Venhälsan in a day, 12 answers showed that the question box did not create a huge interest.

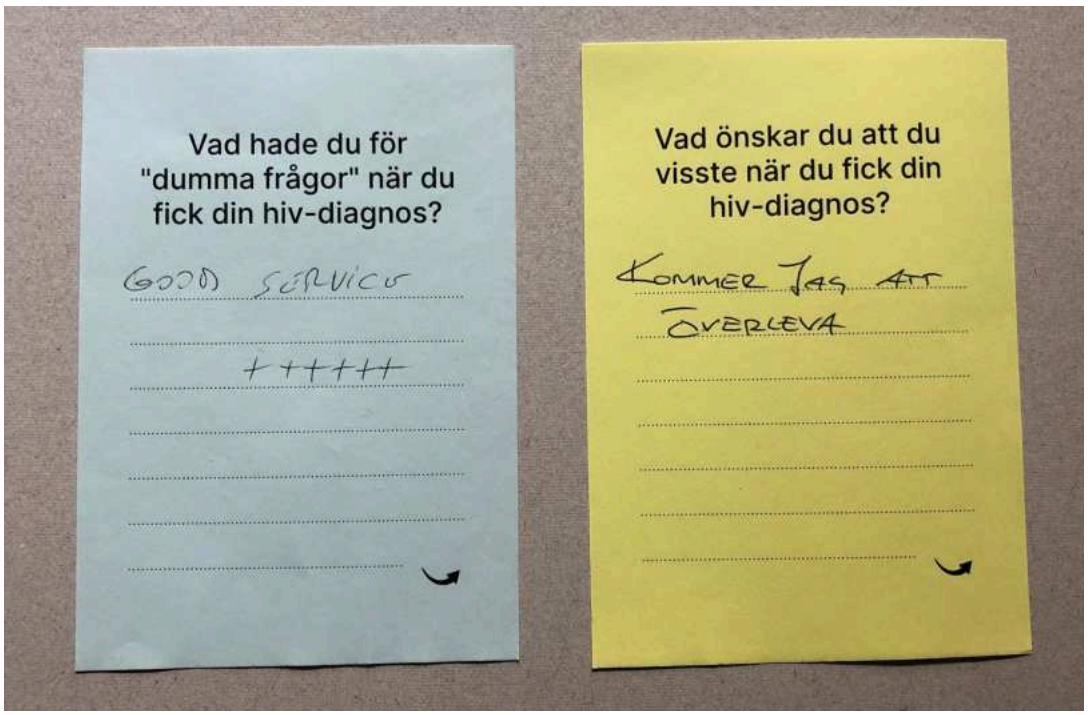


Figure 26. Two answered question cards reading “Good service” and “Will I survive?”, one not so useful and one useful.

5.3.4. Package design

Alongside the research and creation of the content for the package, the design was determined by several iterations.

It was decided that the package should be in cardboard due to its’ versatility and cost-effectiveness, it is also a sustainable and recyclable material. Moreover, it was important that the themes could be treated separately, to give the newly diagnosed person the opportunity to focus on some themes or skip others if they feel the need for that, as explained in chapter 5.3.2. That meant being able to see all themes at the same time, and being able to take each theme out separately without having to lift or move the others.

Inspiration was taken from existing cardboard-box-solutions, and was evaluated through creating physical and digital mockups. The physical mock-ups were created fast to quickly get a sense of size and proportions, see Figure 27. The mock-ups explored different ways the themes could be separated and viewed, and how the package could be opened. The size was kept small as it would be easy to transport the package, for example put it in a bag or pocket of a coat. In this way, some chapters could be processed outside of the home, and the package could easily be transported to Venhälsan during visits. The content for the themes were created in parallel with the physical mock-ups to get a sense of how much space was needed.



Figure 27. Physical mock-ups

The digital mock-ups were then more detailed, and were used to visualize ideas more clearly, see Figure 28.

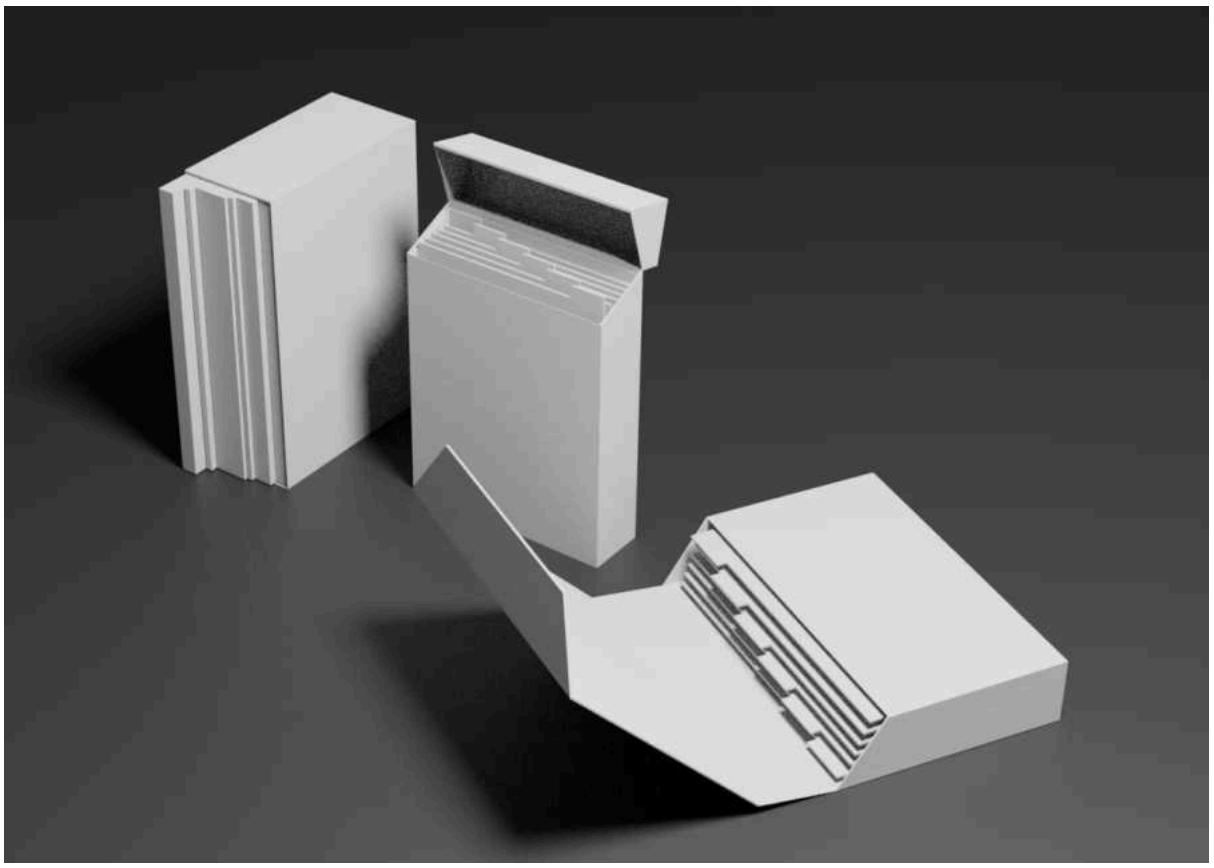


Figure 28. Digital mock-ups of three different package design ideas.

As for the graphic design, the idea was that the package should balance between being serious, while also being light-hearted. As the package was meant to process during a time of crisis, as explained in chapter 5.3.1, everything needed to be easy to read and follow a logical structure. The themes should be easy to distinguish, with different colors and symbols that differentiate them. To still create cohesion, the same font and font size was used throughout the package, and the outside format of every theme was the same.

6. Final concept

This chapter presents the final design which features an Hiv Handbook, created as a complement to the already existing service that Venhälsan offers after an hiv-diagnosis. As the main study showed, the period after an hiv-diagnosis can be tough, and even though the newly diagnosed person goes to regular appointments, see 4.1.2 Interviews with healthcare co-workers at Venhälsan, the time in between those can be confusing and lonely. The handbook includes six themes that are relevant for the newly diagnosed person to process. At the same time, the handbook allows the newly diagnosed person to be prepared before the appointments, and healthcare co-workers at Venhälsan can use the handbook as support for their work.

6.1. Final design

The final design features a physical package containing six different themes, see Figure 29. The purpose of the handbook is to go through topics that are important to think about when receiving an hiv-diagnosis, and to guide the newly diagnosed person at home in between the visits at Venhälsan, acting as a complement to the care they are given at Venhälsan.



Figure 29. The final design of the Hiv Handbook

The patient is first met with an introduction to the handbook. This introduction explains the purpose and structure of the themes, and goes on to list them, as shown in Figure 30.



Figure 30. Introductory text, explaining the purpose and content of the handbook.

The handbook also features a journal. The journal acts as a place of reflection, where the patients can write down their thoughts about the different processed themes in the handbook and bring to Venhälsan to discuss their reflections with their counselors, doctors and nurses. To facilitate this, all themes include some sort of reflection assignment.

As explained in the previous chapter, all themes will be stored separately, to enable the user to process them one by one. The themes will therefore be stored in smaller packages, distinguished by the use of a number per theme. See Figure 31.



Figure 31. Smaller package for each theme.

Each theme also has a corresponding pattern to distinguish them from each other, see Figure 32.

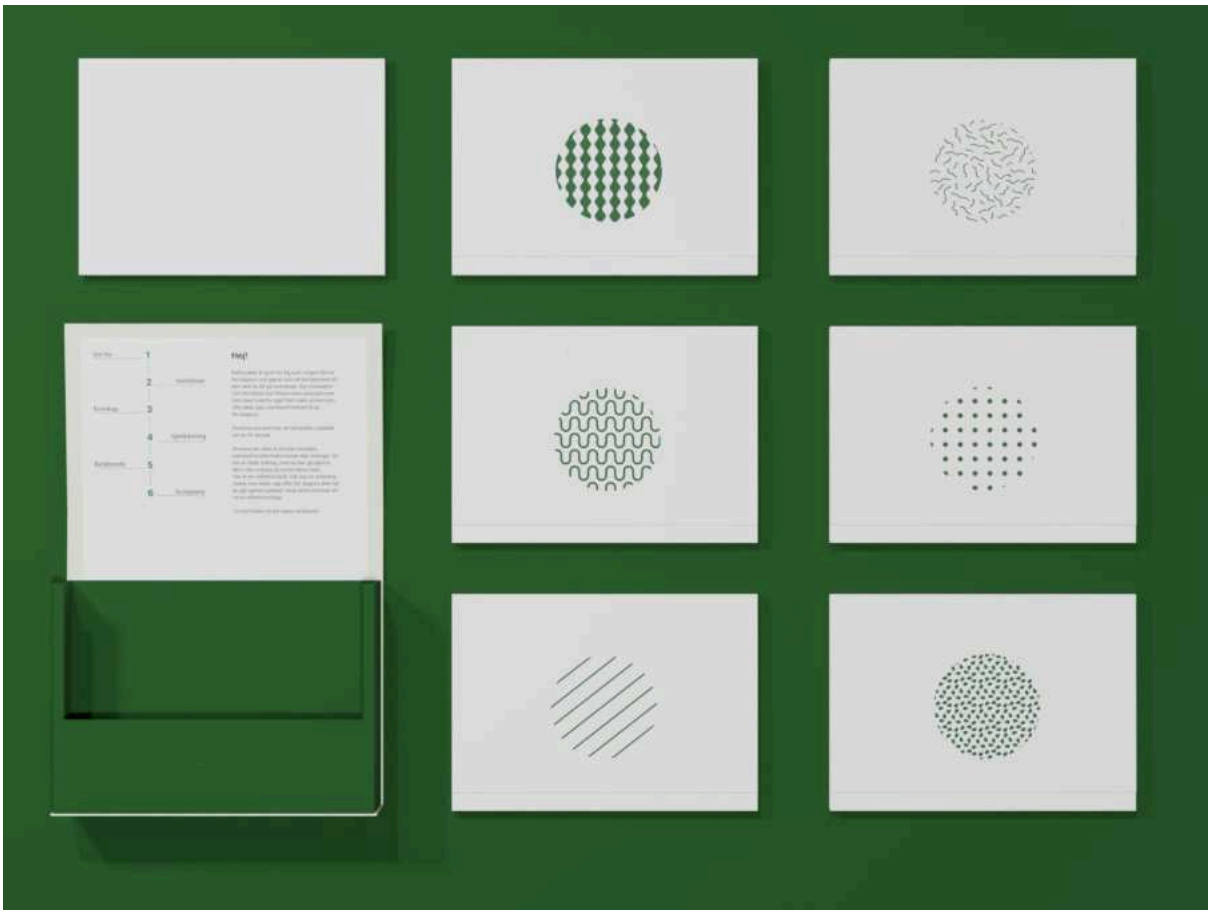


Figure 32. Each theme layed out.

The contents of the themes vary in structure, but all share a similar A5-size. See all first pages of the themes in Figure 33.

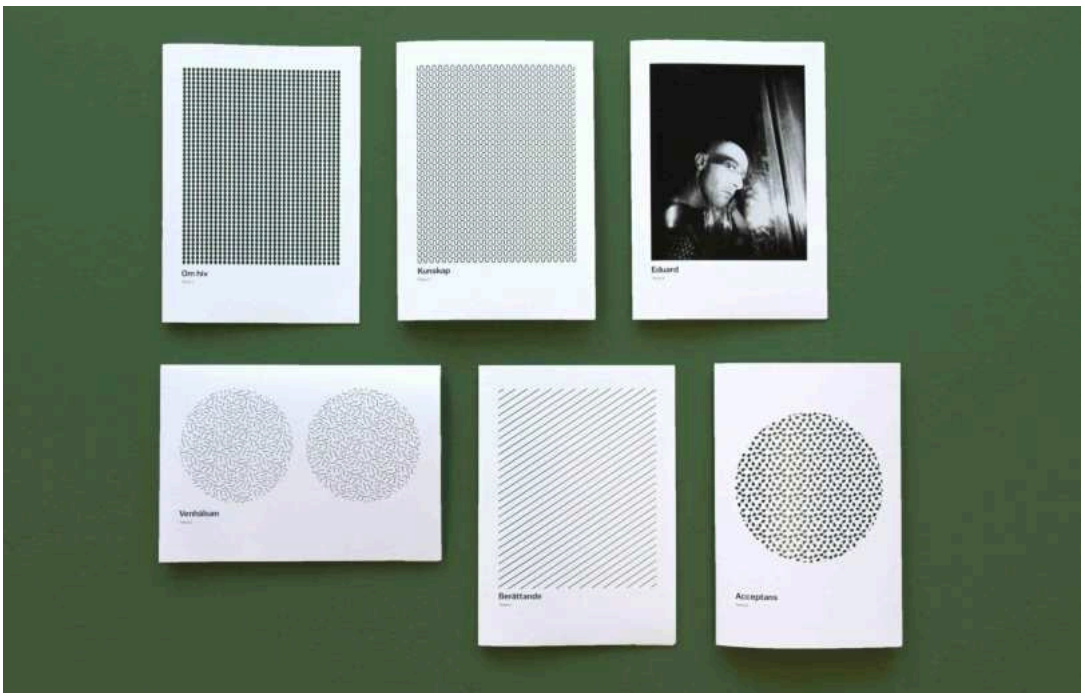


Figure 33. The front-page of each theme.

Theme 1: About hiv

The first theme presents the newly diagnosed person with information about hiv in a clear and concise way, see Figure 34. As explained in chapter 5.3.1, it is good to receive repetitive information in different ways, both orally and written, when in a state of crisis. The aim of presenting information this way is to reduce the risk of the patient looking at information from non credible sources, to make sure they receive the right information. If the patient wants to delve into more information, trustworthy sources are listed at the end of the theme as QR codes to scan.

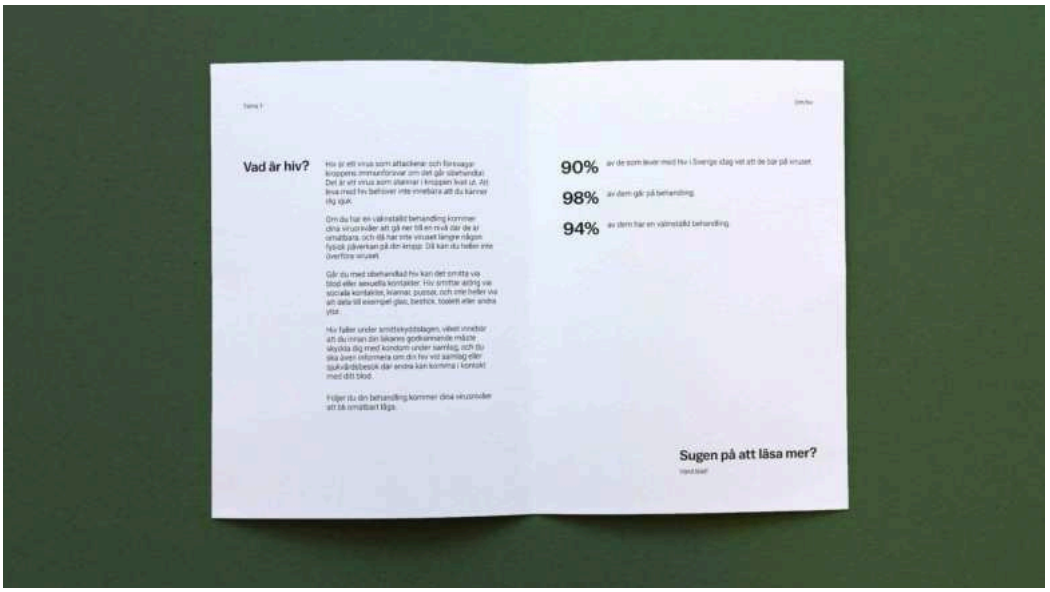


Figure 34. Theme 1, information about hiv.

Theme 2: Venhälsan

Theme 2 introduces Venhälsan and the care they give. After reading a short introduction about Venhälsan, the patient is presented with the different professions they will meet during their visits at Venhälsan. Each profession is presented with a picture and a small text about their responsibilities towards the patient, see Figure 35. This is a way for the patient to familiarize themselves with their caregivers, and to contribute to creating a feeling of safety around Venhälsan, as explained in chapter 5.3.2.



Figure 35. Venhälsan presented.

The theme also includes a page which provides an overview of the treatment the patient will receive during their first four months at Venhälsan, see Figure 36. This is explained with a text explaining “What will happen now?” and a graph displaying the different visits the patient will have to go through at Venhälsan, when they will happen, and also displaying a general case of how the hiv virus reacts to ART. This is further explained with a text. The purpose of this is to give the patient an overview, to regain a feeling of control which is sometimes lost in a crisis, as explained in chapter 5.3.1.

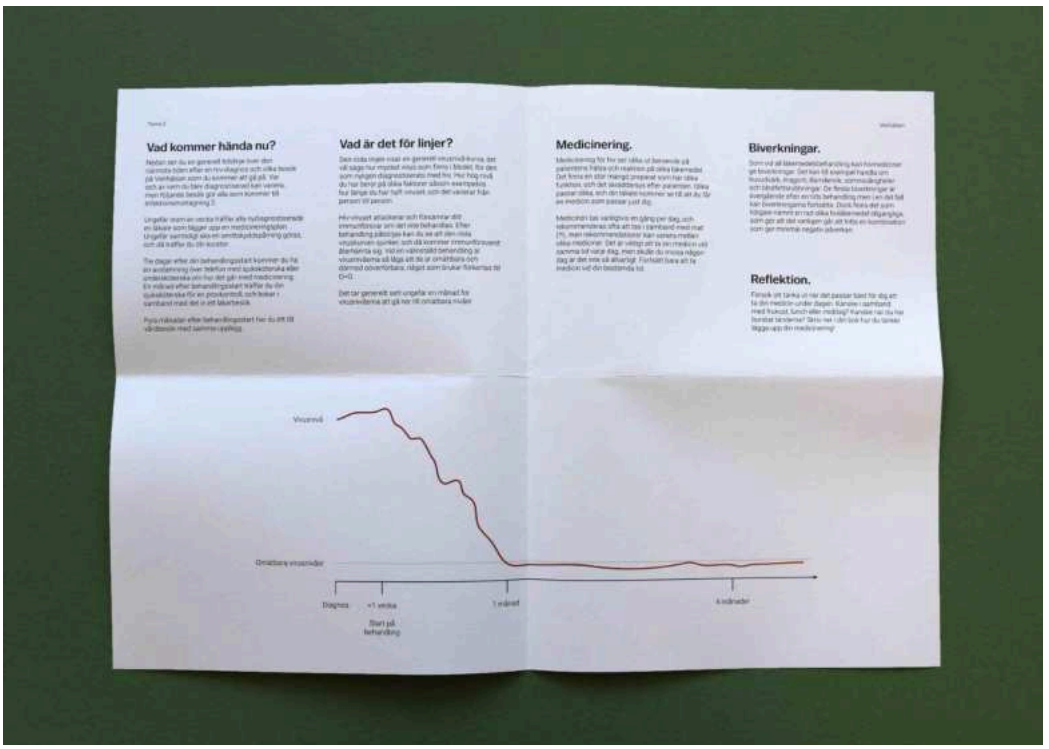


Figure 36. Information and a graph about the coming time.

Together with the treatment overview, this theme will also present the newly diagnosed person with information about what the ART does and what side-effects they might experience from their medication. This is to once again work towards reducing the patients' worries regarding their health and their hiv diagnosis.

Theme 3: Knowledge

In theme 3, the person diagnosed with hiv will be presented with cards with statements and questions regarding hiv from other people living with hiv, see Figure 37 and 38. The idea is to normalize questions and thoughts that the person diagnosed with hiv might have, but do not dare to ask. This is again to give the newly diagnosed person more information, but also address “stupid” questions. As the respondents living with hiv mentioned during the feedback session, seeing that other people living with hiv share experiences and questions with them can help the patient cope with their diagnosis.



Figure 37. Theme 3 introduction.



Figure 38. The question cards, question and answer.

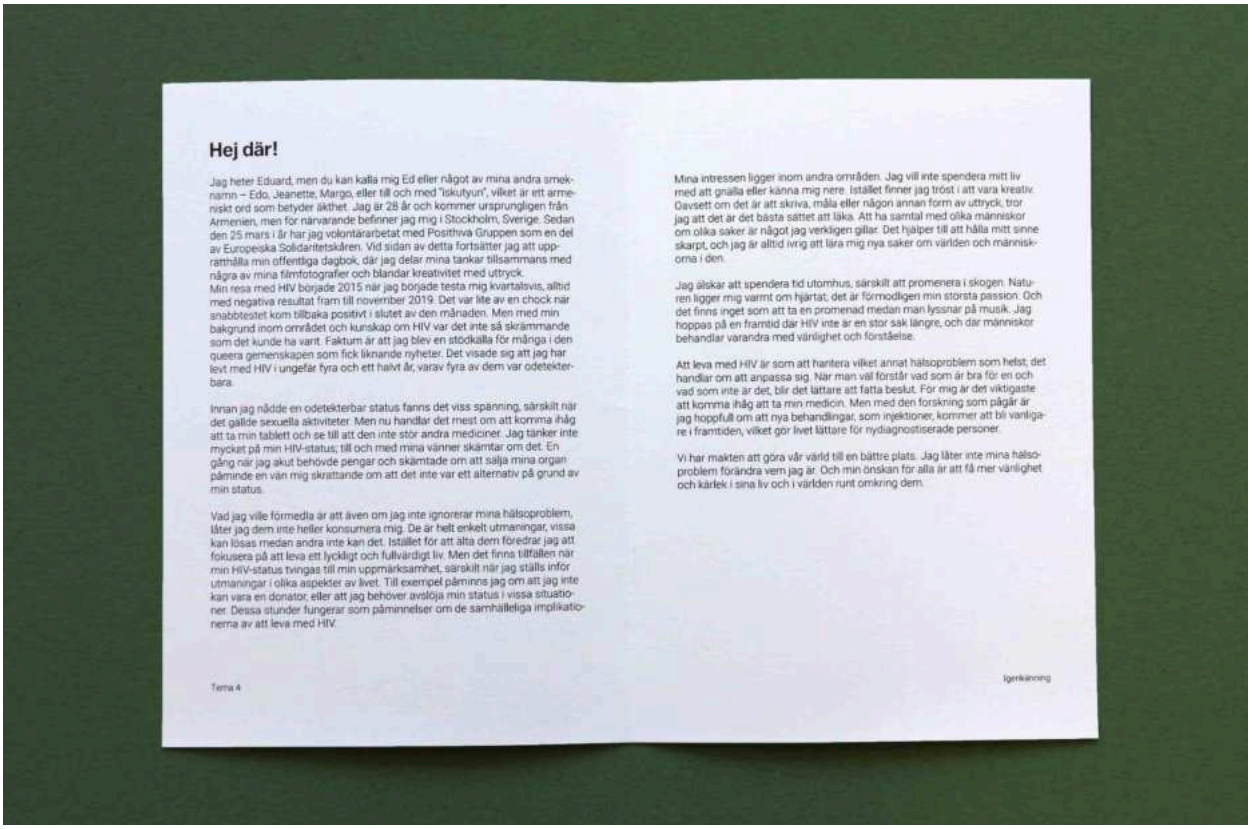
Theme 4: Recognition

This theme presents many different portraits of people living with hiv and contains a short introduction of them, their age, occupation, hopes and dreams, interests, future plans and so on, see Figures 39 and 40 for one of the portraits. The portraits are written sort of as a letter to the newly diagnosed person, but will not be mainly about hiv. The respondents reacted positively to reading about different normal people living with hiv to combat the stereotypes or preconceptions of people living with hiv that the patient might have.

For example purposes, meet Eduard:



Figure 39. Eduard.



Hej där!

Jag heter Eduard, men du kan kalla mig Ed eller något av mina andra smeknamn – Edo, Jeanette, Margo, eller till och med "iskutyun", vilket är ett armeniskt ord som betyder äkthet. Jag är 28 år och kommer ursprungligen från Armenien, men för närvarande befinner jag mig i Stockholm, Sverige. Sedan den 25 mars i år har jag volontärarbetat med Posithiva Gruppen som en del av Europeiska Solidaritetskåren. Vid sidan av detta fortsätter jag att upprätthålla min offentliga dagbok, där jag delar mina tankar tillsammans med några av mina filmfotografier och blandar kreativitet med uttryck.
Min resa med HIV började 2015 när jag började testa mig kvartalsvis, alltid med negativa resultat fram till november 2019. Det var lite av en chock när snabbtestet kom tillbaka positivt i slutet av den månaden. Men med min bakgrund inom området och kunskap om HIV var det inte så skrämmande som det kunde ha varit. Faktum är att jag blev en stödkälla för många i den queera gemenskapen som fick liknande nyheter. Det visade sig att jag har levt med HIV i ungefär fyra och ett halvt år, varav fyra av dem var odetekterbara.

Innan jag nådde en odetekterbar status fanns det viss spänning, särskilt när det gällde sexuella aktiviteter. Men nu handlar det mest om att komma ihåg att ta min tablett och se till att den inte stör andra mediciner. Jag tänker inte mycket på min HIV-status; till och med mina vänner skämtar om det. En gång när jag akut behövde pengar och skämtade om att sälja mina organ påminde en vän mig skrattande om att det inte var ett alternativ på grund av min status.

Vad jag ville förmedla är att även om jag inte ignorerar mina hälsoproblem, låter jag dem inte heller konsumera mig. De är helt enkelt utmaningar, vissa kan lösas medan andra inte kan det. Istället för att älta dem föredrar jag att fokusera på att leva ett lyckligt och fullvärdigt liv. Men det finns tillfällen när min HIV-status tvingas till min uppmärksamhet, särskilt när jag ställs inför utmaningar i olika aspekter av livet. Till exempel påminns jag om att jag inte kan vara en donator, eller att jag behöver avslöja min status i vissa situationer. Dessa stunder fungerar som påminnelser om de samhälleliga implikationerna av att leva med HIV.

Mina intressen ligger inom andra områden. Jag vill inte spendera mitt liv med att gnälla eller känna mig nere. Istället finner jag tröst i att vara kreativ. Oavsett om det är att skriva, måla eller någon annan form av uttryck, tror jag att det är det bästa sättet att läka. Att ha samtal med olika människor om olika saker är något jag verkligen gillar. Det hjälper till att hålla mitt sinne skarpt, och jag är alltid ivrig att lära mig nya saker om världen och människorna i den.

Jag älskar att spendera tid utomhus, särskilt att promenera i skogen. Naturen ligger mig varmt om hjärtat; det är förmodligen min största passion. Och det finns inget som att ta en promenad medan man lyssnar på musik. Jag hoppas på en framtid där HIV inte är en stor sak längre, och där människor behandlar varandra med vänlighet och förståelse.

Att leva med HIV är som att hantera vilket annat hälsoproblem som helst; det handlar om att anpassa sig. När man väl förstår vad som är bra för en och vad som inte är det, blir det lättare att fatta beslut. För mig är det viktigaste att komma ihåg att ta min medicin. Men med den forskning som pågår är jag hoppfull om att nya behandlingar, som injektioner, kommer att bli vanligare i framtiden, vilket gör livet lättare för nydiagnostiserade personer.

Vi har makten att göra vår värld till en bättre plats. Jag låter inte mina hälsoproblem förändra vem jag är. Och min önskan för alla är att få mer vänlighet och kärlek i sina liv och i världen runt omkring dem.

Figure 40. Eduards' letter.

Theme 5: Disclosure

Since the disclosure of ones' hiv is personal and contextual, as explained in chapter 5.3.2, this theme presents some information about disclosure and how one can approach it, while emphasizing that it is on the person living with hivs' terms. As Magnus Karlsson mentioned, it is good to think about boundaries and what the person diagnosed with hiv wants to achieve with the disclosure. To facilitate this, this theme also includes a list of questions the patient should ask themselves before deciding to disclose to a person, displayed in Figure 41.



Figure 41. Theme 5: Disclosure.

Theme 6: Acceptance

The last theme features an exercise the person diagnosed with hiv can do to start reflecting on their own feelings, since “the best way to reduce the intensity of a feeling is to feel it”, as Agnes von Schreeb put it in her interview. An introduction to the theme is shown in Figure 42.



Figure 42. Theme 6 introduction.

They will guide themselves through a mindfulness exercise and then write about any negative feelings that were brought up, see exercise manuscript in Figure 43. Springing from Acceptance and Commitment theory, this exercise aims to make the patient aware of their feelings, to start treating them, and discussing them with the co-workers at Venhälsan.



Figure 43. The manuscript for the mindfulness session.

6.2. The Hiv Handbook in a service system

As the Hiv Handbook is meant to act as a complement to an already offered support and care service at Venhälsan, it was important to view how the handbook would fit in the service, and how it would affect the experience of both co-workers at Venhälsan and the patients.

A service blueprint of the service over the four months following the patient getting their diagnose at Venhälsan was made, and can be found in Figure 44. The blueprint involves processes, the different actors, and props, which in this case only is represented by the handbook. It was based on Venhälsans' timeline for handling newly discovered hiv-diagnoses which was presented in chapter 4.1.2. The blue texts are new interactions and processes connected to the handbook.

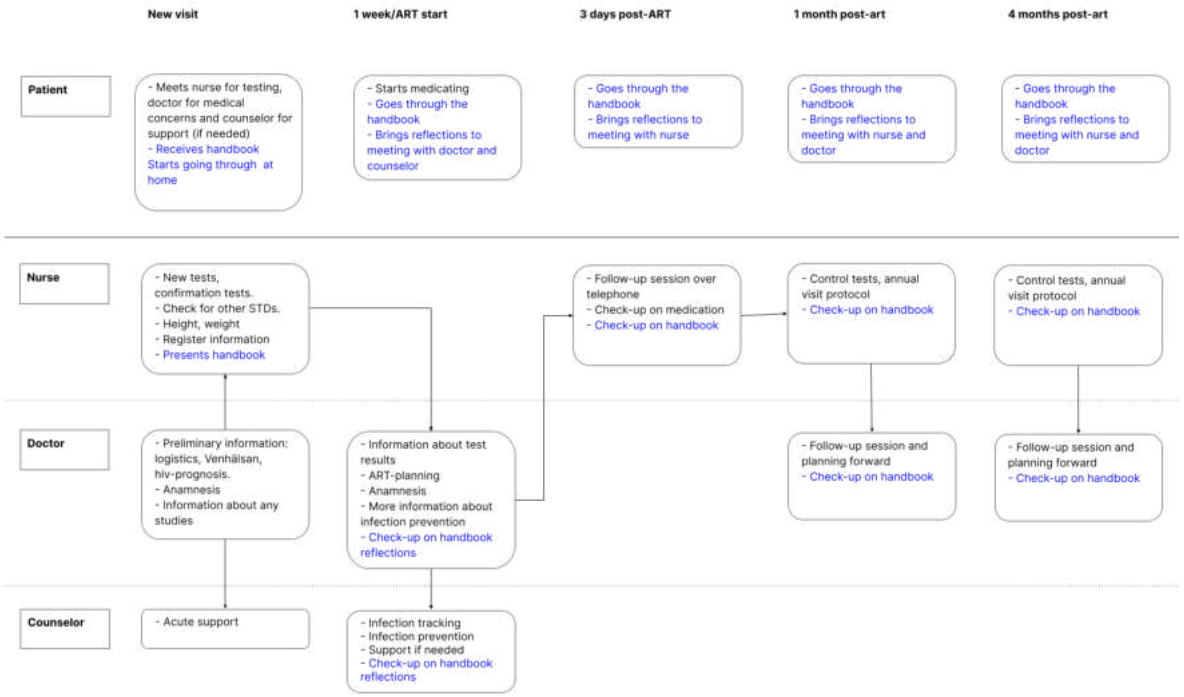


Figure 44. Service blueprint of an hiv-patients' care at Venhälsan, after introduction of handbook.

The main aim of the handbook is to act as a complement to the care given at Venhälsan. This means that the handbook itself has two main objectives:

- To act independently with aiding the patient through treating the handbook content, doing the exercises and reading the different texts provided in the handbook.
- To act as a base of conversation for the patient when meeting the nurses, doctors and counselors at Venhälsan. This would be based on the reflective part of the handbook, and going through notes written in the provided journal.

When handing over the handbook to the patient, the nurse briefly explains the content and structure of the handbook through showing the inside of the carton, and reads the instructions shown in Figure 30 together with the patient.

This uses the patient's trust towards Venhälsan, as explained in Insight 11, and should increase the likeness of the patient processing the handbook material. During this first period after the diagnosis, where the handbook aims to support the patient, the patient frequently visits Venhälsan. The visits have different purposes, and with different actors. Similarly, the handbook has different themes and covers topics that some are more medical, and some psychosocial. This makes it so that during the visits at Venhälsan, the different professions can discuss the themes that are most related to their profession. Should the patient bring up topics that are more connected to other professions, the co-workers can refer to the most suited colleague. As themes 1: About hiv, 2: Venhälsan (which is focused on medication and structure of treatment), and 3: Ignorance, are more focused on the medical aspect of hiv, they should primarily be discussed with a nurse or doctor. Similarly, as theme 4: Recognition, 5:

Disclosure and 6: Acceptance are more focused on psychosocial subjects, they should primarily be discussed with a counselor.

Already one week after the patient is handed the handbook, they meet with their doctor and a counselor. Through asking about the patients' experience of the themes in the handbook, this quickly gives Venhälsan an opportunity to learn more about the patient, to tailor the treatment to their needs.

The handbook does put further requirements on the co-workers at Venhälsan, as their service towards the patient today does not include any artifacts that are directly handed to the patients. This takes time from each visit, as the co-workers should ask the patients about their progress with the handbook, and discuss the reflections that might be brought up by the patients.

7. Discussion

This chapter discusses the project in overarching subjects that does not necessarily correspond to a certain chapter. Finally, further work and development of the handbook and its role in Venhälsans' care is discussed, and the research questions are discussed and evaluated.

Ethical considerations

While considerations were taken, it did not become a big topic of discussion throughout the project. Every step of the project, the methods used, the phrasing of different texts were always checked by counselors at Venhälsan, which reduced the risk of projecting any stigma towards the respondents. One respondent who took part in several rounds of interviews and the cultural probe was relieved when they realized that they would not be interviewed further, as thinking about their hiv made them uncomfortable. Despite that, the respondent was grateful for being involved in the project.

The respondents living with hiv

As mentioned and shown in Appendix A, the respondents from the patient-group consisted of 7 men and 1 woman, of varying age. Looking at all people living with hiv in Sweden, the respondents are not that representative of the patient group, since it in Sweden is a very heterogeneous group, as 51% of all hiv transmissions occur during heterosexual intercourses. As all male respondents were either gay or bisexual, this did not give an accurate perspective of the Swedish patient group. However, Venhälsans targets MSM and transgender people, and even though Infektionsmottagningen 2 caters to everyone, the group consists of 89% MSM, and the respondents could be seen as representative for the context of this project. A discussion can be made about how the needs would change for other health clinics, such as Karolinska Sjukhuset in Huddinge, where the target group is more varied than at Venhälsan. There, an intersectional perspective could have proven more effective than at Venhälsan, since the majority of the respondents were white middle- or upper-middle-class men.

Due to the stigma around hiv, a big challenge of the project was to find respondents willing to take part of the study. This became evident during the recruitment process when the people who responded to the posters were generally out-going and extroverted, which could pose a challenge as they did not represent the entire patient group. However, recruiting respondents through using the counselors and nurses at Venhälsan provided insights in the lives of people living more isolated and introverted, which strengthened the research phase tremendously. Four of the respondents saw the advertisement for the interviews and got in contact by themselves, while four of the respondents were asked to participate by the counselors or nurses.

Approach

In many instances where Service Design is discussed, it is based on organizations and customers, looking at the different people, processes and props involved. As the care of someone living with hiv at Venhälsan is mostly about psychosocial aspects and intangible services (as the care at Venhälsan is service-dominated), the focus of the project naturally went more towards studying the people and processes involved. The design process did not involve that much immersion in the patients' journey through a Venhälsan visit, and therefore did not pay that much attention to any artifacts or props that are involved in a visit. However, from speaking to co-workers at Venhälsan, there seemed to be very few, if any, props apart from the healthcare setting. An evaluation of the handbook would therefore be interesting, as it is an implementation of a "product" in a service-oriented process and environment.

Evaluating the project from a Service Design perspective, one can compare the process with the six main principles of Service Design that were stated in the Methodology chapter. The project was a success in working human centered and collaboratively throughout the main study and ideation stage, building on constant feedback from different actors. One can also argue that the work was done iteratively through the constant feedback, with several rounds of interviews and feedback sessions. Much of the study was also done in a real healthcare setting, apart from the previously mentioned absent immersion methods. Lastly, the project was shaped from a holistic mindset, as the study involved as many main actors as possible, despite that the end result was a product for the individual.

Going back to Vinks' assumptions, there are a few comments to be made about their correlation to the project. Starting with discussing the value of a design process rather than outputs, it became apparent that it was valuable for Venhälsan to be involved in the project. Not only did they bring up ideas during tutoring sessions and the workshop, but in connection to the workshop, co-workers started discussing how they with a Service Design approach can identify small problems or opportunities which they themselves can easily address.

As stigma is a social construct, Vinks' discussion of social structures being the materials of design, and that actors are entangled in the systems they design became relevant during the project. A goal with the project became to look at the attitudes towards people living with hiv, to try and reshape the understanding of hiv. To balance informing the people living with hiv about the negative movements and problems that exist, while also trying to make them feel good about themselves, to spread a more positive attitude of hiv. This also connects back to Manzinis' understanding of Social innovation, that people can be viewed as bearers of opportunities rather than problems.

Methodology

Since the project followed a service design methodology, it was important to involve the different stakeholders as much as possible, and for the students to immerse themselves into the different settings, which proved to be difficult. One reason for this is that the context is a

healthcare setting, which can be a delicate and vulnerable setting for the patients. In the early stages of the project, there were plans to carry out observations and shadowing during appointments but this was disregarded by the counselors and management at Venhälsan. Because of the vulnerable situation, the healthcare co-workers were not comfortable with asking the patients about shadowing when they arrived at their appointment. If the project would have been longer, one solution to this was to ask patients if they would approve of shadowing or observations during their next appointment, but since most appointments are 6 months apart that was hard to realize.

Some methods had to be reworked since no one living with hiv wanted to participate. The original plan was to conduct a workshop with the respondents living with hiv, similar to the one with the healthcare co-workers at Venhälsan, where the respondents could give feedback on some of the major ideas. However, not one person showed interest in doing a workshop in a group setting, which resulted in the online feedback sessions one-on-one instead. The online session served the same purpose, but it would have been interesting to see the respondents interact with each other and what ideas and feedback that could be generated. This also spoke against some ideas that were brought up throughout the project that revolved around recruiting people living with hiv to engage in group activities, as it was difficult to get willing participants.

The interviews conducted online posed some challenges, as it was for example hard to analyze body language and intonation on a video call. As the needs study required the interviews to enter areas that could be uncomfortable for the respondent, this made it harder to navigate what was sensitive and not.

Result discussion

It became evident after the why-how ladder and the insights that most of the factors that affect people living with hiv negatively today are societal and institutional, or spring from societal or institutional factors, for example the outdated understanding of hiv from the 80's and 90's. The outdated understanding is the root cause of every other issue, and if that was addressed, many other issues would be solved as well. While creating ideas, some were on societal and institutional levels, such as the Venhälsan certification and information campaigns, while others were on an individual level, such as the care-package or the Venhälsan-app. Addressing the main issue, the outdated understanding, has potential to have a larger societal impact, however for the scope of this project, it would require too much time to construct, and it would take a lot of time before the respondents at Venhälsan would be positively affected by it. The final solution, the handbook, is much closer to the respondents, and has more potential to increase their psychosocial well being right now, and within the frames of the project.

Commenting on this projects' contribution to the UN Sustainable Development goals, hiv and aids is directly mentioned in target indicator 3.3.1, which aims to end the aids epidemic. This appears to only include the medical side of living with hiv, which does not relate to the

Swedish context. For a Swedish context, the 95-95-95-95 goals would be a more important aim than the SDG indicator 3.3.1, as it is a goal that targets psychosocial well-being, rather than medical.

However, the handbook could cause issues for the structure of the care at Venhälsan. An optimistic view is to see the handbook as a base on which a visit at Venhälsan can build. Discussing everything from the perspective of the handbook. A less optimistic view is to see the handbook as something that takes time from already short visits at Venhälsan. As explained in the pre-study, many of the co-workers wished for longer patient visits during the period after a diagnosis, as they thought they lacked time for some discussions. Here the handbook could cause a problem, if it ends up taking too much time from the ordinary care given at Venhälsan. It is therefore important to run tests with the handbook for future development, to see which themes have the most impact, and to review if any should be changed or even discarded to benefit the care.

An optimistic view of the presented solution is to make the psychosocial support offered at Venhälsan more available. As stated in the introduction, there is only a small number of patients who utilize the support offered by counselors at Venhälsan, and also that only 15% of people living with hiv in Sweden visit patient organizations. The use of psychosocial support is less than the need for it, as many more witness of negative feelings due to their hiv status. As themes 4-6 in the handbook heavily focuses on emotions and psychosocial aspects, it could raise questions that can be discussed with the counselors, and thus the handbook could make the support offering at Venhälsan more used.

Research questions

In order to conclude the project, the research questions defined in chapter 1.2 are discussed below.

RQ1: How does Venhälan provide psychosocial support for their patients?

RQ1 was mainly addressed in the pre-study, with interviews with co-workers at Venhälsan and actors from patient organisations and other healthcare wards.

Venhälsan offers their patients psychosocial support in most steps of their care. Whether the patient meets a nurse, doctor or counselor, they are always offered time for discussing psychosocial aspects of their diagnosis. Venhälsan cooperates with patient organisations to keep track of people living with hiv in Stockholm, to make sure they receive the care they need.

RQ2: ***How do people living with hiv experience existing psychosocial support and what are they lacking?***

Through involving the patient group throughout the project, their experience with different types of support could be reviewed.

People living with hiv that are treated at Venhälsan are generally happy with the psychosocial support they receive. They receive direct support from co-workers through the different visits, but also from the safe environment that is created at Venhälsan. As many patients witness, Venhälsan co-workers have great knowledge of hiv, and therefore create an environment that the patients feel normal and appreciated in, providing further support. Throughout the main study, it became evident that this type of environment is wanted in the rest of the healthcare system, and therefore there is a need to spread knowledge about hiv across the healthcare system.

Much of the support that is targeted on psychosocial aspects, such as counselors at Venhälsan or similar support solutions at patient organisations are not widely used. What became evident during the main study was that it was hard for people living with hiv to engage in group activities, to show themselves to other people living with hiv. This is despite the fact that peer-to-peer support was appreciated in the instances it was used. To make this support accessible, ways of introducing peers to each other have to be developed.

> **RQ3:** ***How might Venhälsan modify their work with supporting patients during the period after a diagnosis?***

The developed solution aims to support the newly diagnosed patient in contexts outside of Venhälsan, acting as a complement to the direct psychosocial support already offered at Venhälsan. It aims to support the patient directly, through the patient processing the material within the handbook, but also act as a base of conversation during visits at Venhälsan, which could contribute to increasing the well-being of the patient.

Apart from the handbook, Venhälsan was provided with a number of ideas they can develop, test and implement in their care themselves. This was handed over as a collection of insights and ideas from the workshop and brainstorming sessions.

Future work

To move forward with developing the handbook, there is much work to be done. While some themes were completed within the frame of this project, some (mainly Theme 3: Ignorance and Theme 4: Recognition) were not. To make those themes have more substance, they should contain more questions and portraits in order to be able to be tested. As there was only one gathered portrait and four flashcards made, it can be hard to test the themes properly. In order to see if the handbook has a positive impact, it needs to be tested, and the themes need to be completed. As previously mentioned, this would lead to a better understanding of what impact each theme has, which could lead to some being improved, replaced or removed.

The handbook must also be tested by co-workers at Venhälsan, as previously discussed. Introducing this artifact in a service-dominated context needs work, and the handbook should be tested thoroughly through many types of visits at Venhälsan, to see how it affects the care.

Lastly, the testing of the handbook should be done with the target group: people who just got their diagnosis. Respondents in this project varied in age, but the most newly-diagnosed person received their diagnosis in 2018. Although this is quite recent, the perspective of someone who has lived with hiv for 6 years is very different from someone just diagnosed. In order to test and verify the contents of the handbook, it should be tested on someone who just got their diagnosis.

8. Conclusion

This report presents a Masters' thesis project in Industrial Design Engineering at the Royal Institute of Technology in Stockholm. The project was a Service Design project conducted by two students, with supervision and Service Design guidance by Sös Innovation, on the behalf of Venhälsan and Infektionsmottagning 2 at Södersjukhuset in Stockholm. As stated in Chapter 1.2, the purpose and overall aim of the project was to contribute to increasing the psychosocial well-being of people living with hiv, treated at Infektionsmottagning 2.

To approach the purpose, a thorough literature study was done on both medical and psychosocial aspects of hiv, to map what previous research had been done on the subject. Using a Service Design methodology, a main study was carried out through using collaborative, human-centered methods to review existing problems, needs and solutions. This led to a delivery of several insights and ideas to Venhälsan, for them to implement themselves.

The final solution offers newly diagnosed patients a physical Hiv Handbook to act as a complement to the support that is already offered at Venhälsan. Processing the handbook, the patient goes through six themes that were developed, which contains topics and exercises that were deemed important to process during the period after receiving a diagnosis.

The conclusion of this project is that by implementing the handbook in their care, Venhälsan can offer patients psychosocial support in new contexts where they were not acting before, and therefore further increase the psychosocial well-being of their patients.

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Appendix A - Respondents

The interview respondents can be found in Table A.1 to Table A.3.

Table A1: List of respondents from the pre-study.

ID	Time [min]	Location of interview	Profession	Description
Respondent 1	60	Venhälsan	Counselor	Working at Venhälsan
Respondent 2	40	Venhälsan	Counselor	Working at Venhälsan
Respondent 3	49	Venhälsan	Nurse	Working at Venhälsan
Respondent 4	60	Venhälsan	Nurse	Working at Venhälsan
Respondent 5	60	Venhälsan	Nurse	Working at Venhälsan
Respondent 6	41	Venhälsan	Doctor	Working at Venhälsan
Respondent 7	47	Digital	Counselor	Working at Infektionsmottagning 2 at Karolinska sjukhuset
Respondent 8	42	Digital	Lecturer and associate professor at KI	Maria Wiklander
Respondent 9	38	Digital	Operations manager at PG	Daniel Suarez
Respondent 10	43	Digital	Team leader for NAs' social support	Emma Olsson
Respondent 11	43	Digital	Sociologist	Kenneth Fjällborg

Table A2: List of respondents from the patient-group study.

ID	Time [min]	Location of interview	Description	Other
Respondent 12	51	Digital	Man, age 68	Living with hiv
Respondent 13	42	Digital	Man, age 47	Living with hiv, trigger material
Respondent 14	60	Venhälsan	Man, age 50	Living with hiv, CJM
Respondent 15	59	Venhälsan	Man, age 45	Living with hiv, interpreter needed
Respondent 16	60	Venhälsan	Man, age 74	Living with hiv, CJM
Respondent 17	40	Digital	Man, age 24	Living with hiv, trigger material
Respondent 18	60	Venhälsan	Man, age 40	Living with hiv, CJM, trigger material
Respondent 19	49	Digital	Woman, age 39	Living with hiv
Respondent 20	23	Digital	Man, age 57	Partner to one respondent living with hiv

Table A3: List of respondents from the second iteration of concept generation.

ID	Time [min]	Location of interview	Profession	Description
Respondent 21	60	Sös	Counselor	Working at internal medicine and intensive care unit at Sös
Respondent 22	60	Sös	Psychotherapist	Working at HBTQ-hälsan at Sös

Appendix B - Interview guides

The following appendix contains all interview guides for the different stakeholders.

Interview guide for the nurses/doctors at Venhälsan

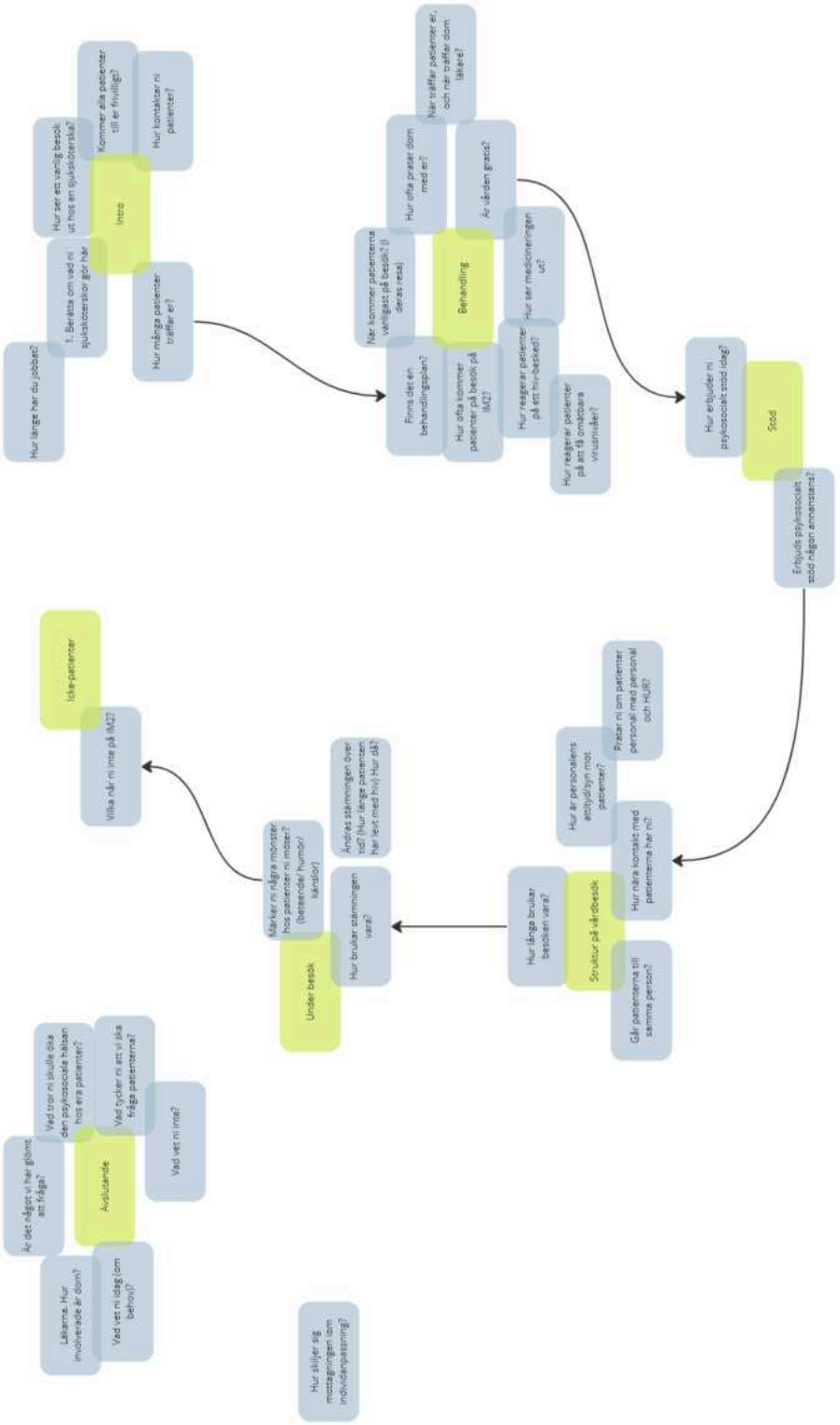


Figure B.0.1: Interview guide for nurses and doctors at Venhälsan.

Interview guide for the counselors

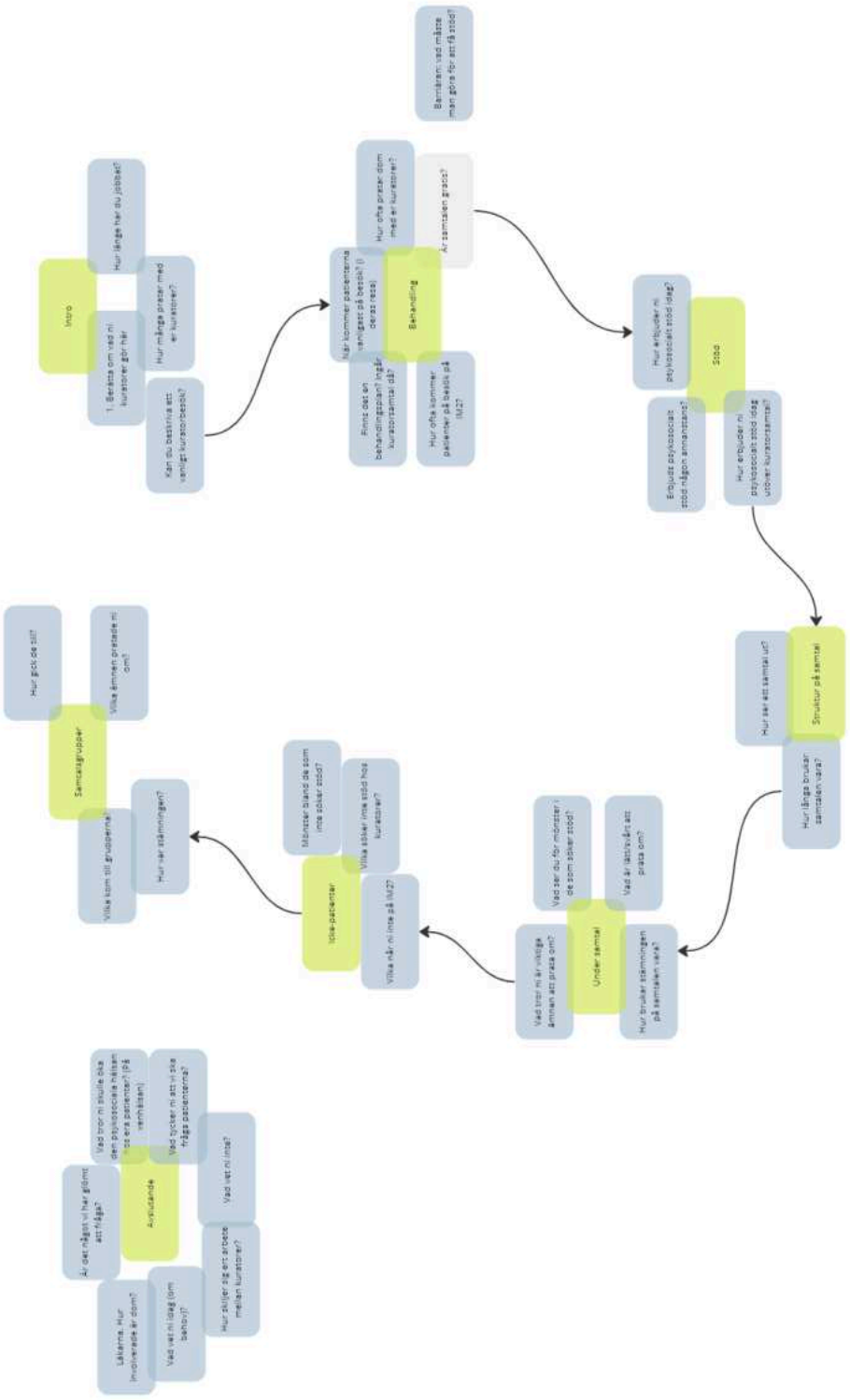


Figure B.0.2: Interview guide for counselors at Venhälsan.

Researchers

Maria Wiklander

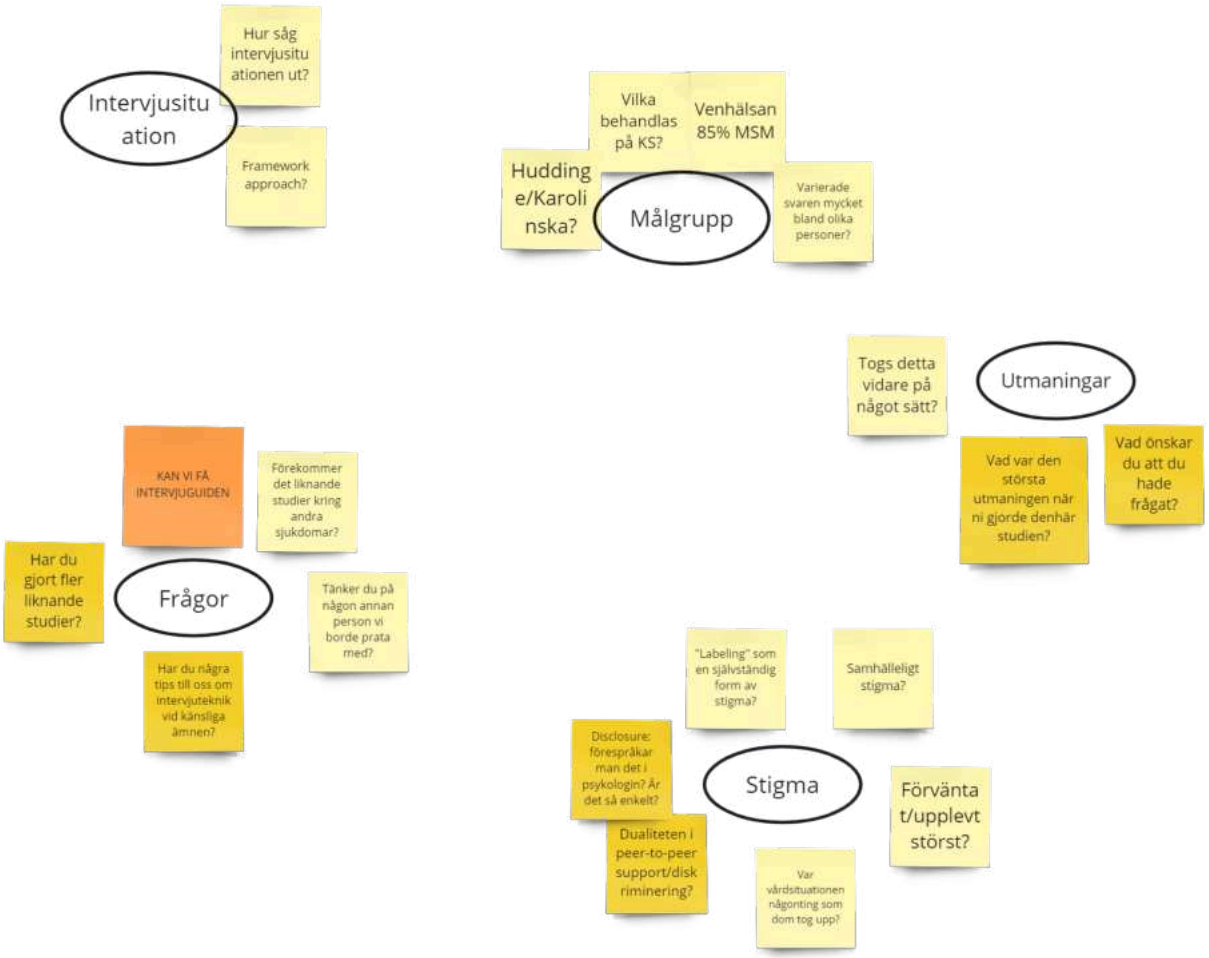


Figure B.0.3: Interview guide for Maria Wiklander.

Organizations, PG and Noaks Ark

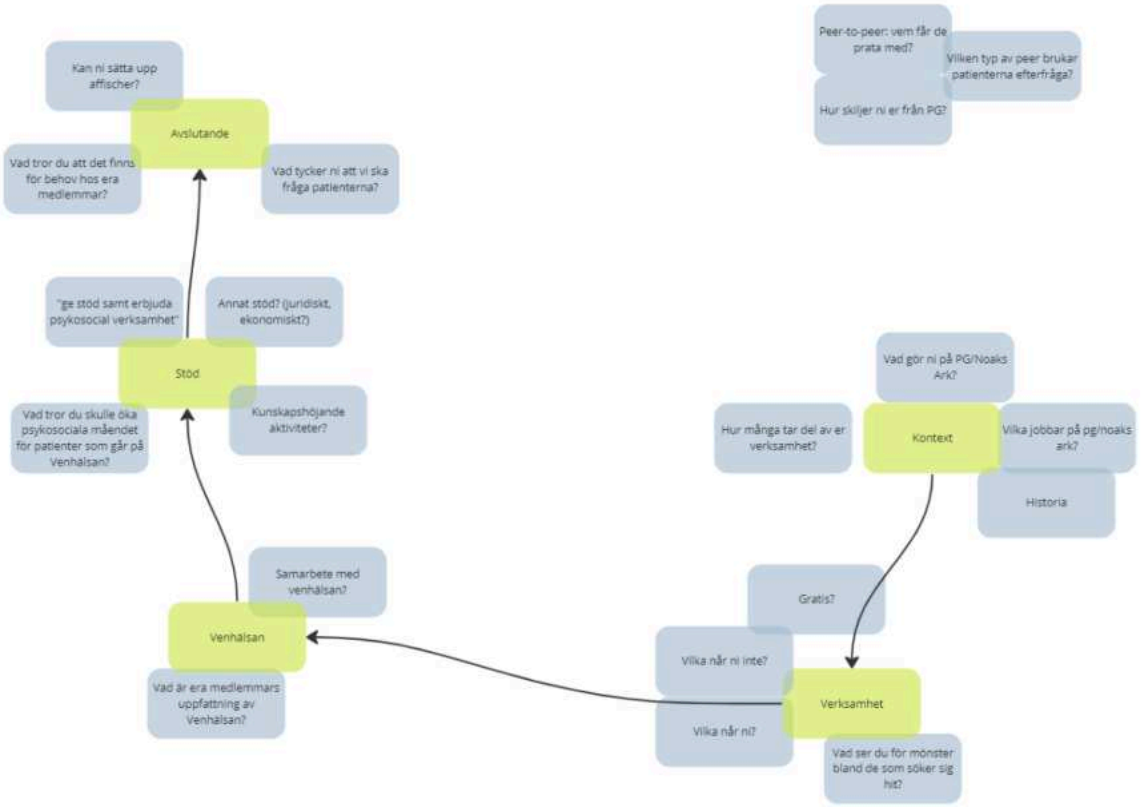


Figure B.0.4: Interview guide for patient organizations.

People in close contact with people living with hiv

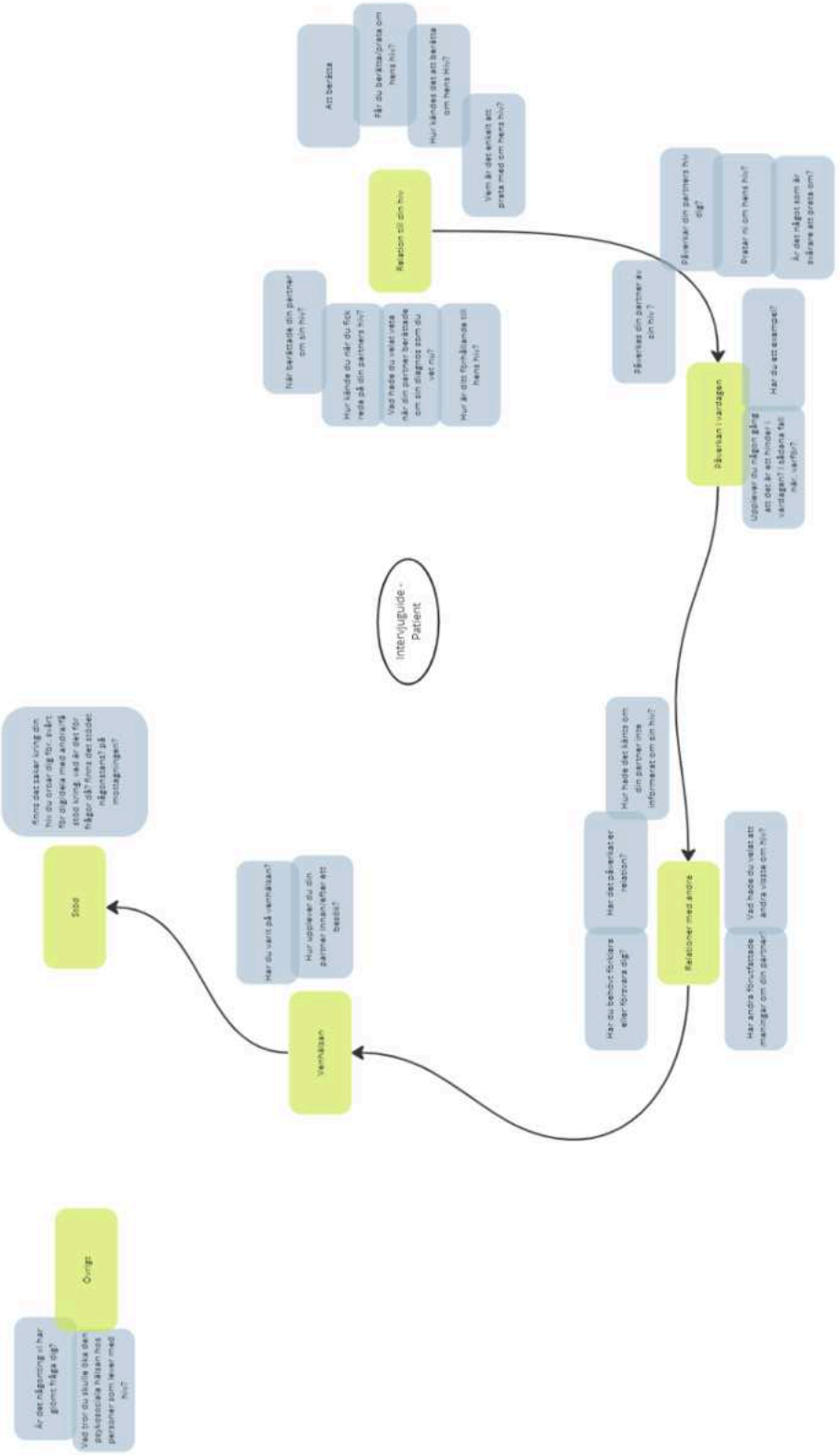


Figure B.0.5: Interview guide for partner to patient at Venhälsan.

Respondents living with hiv

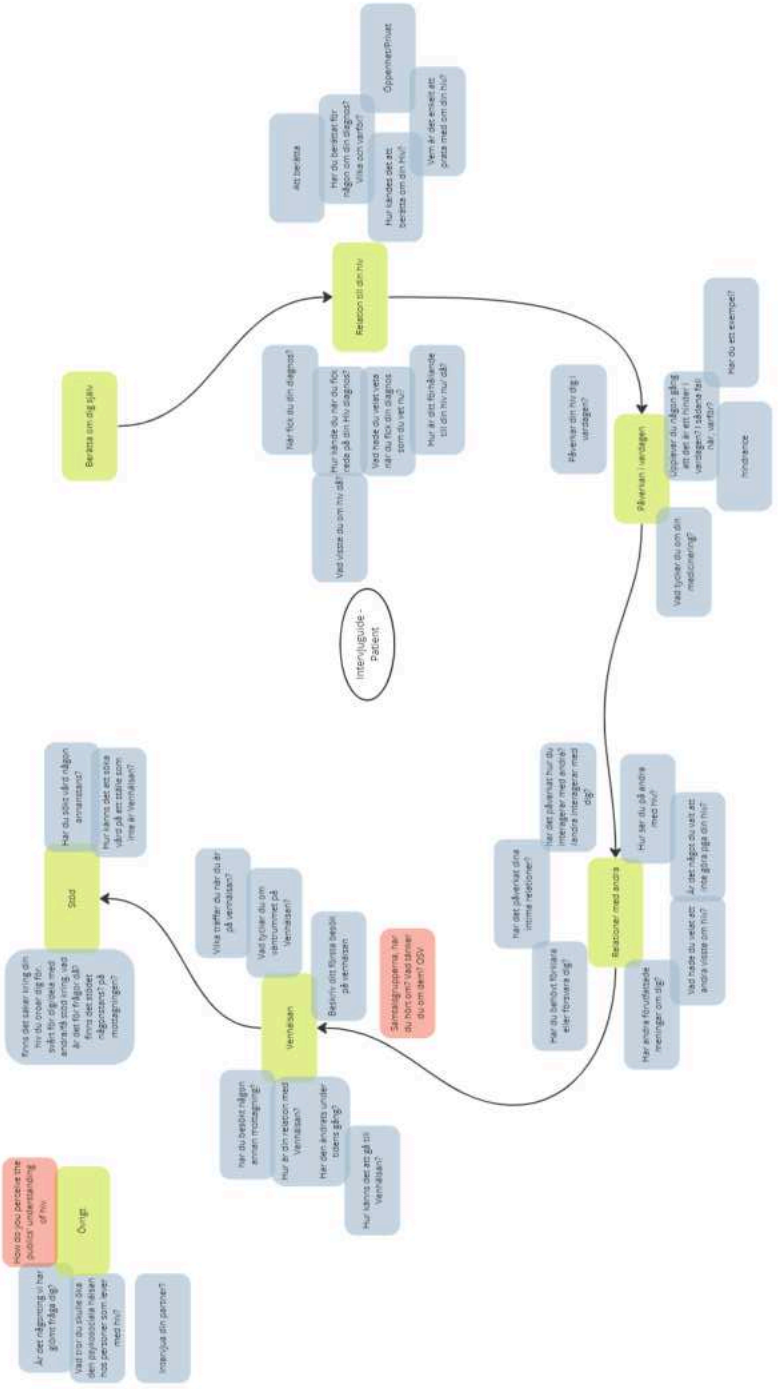


Figure B.0.6: Interview guide for patients at Venhälsan.

Appendix C - Posters

This Appendix presents the posters used at Venhälsan to get respondents, both in swedish and english.



Figure C.0.1. Poster in Swedish.

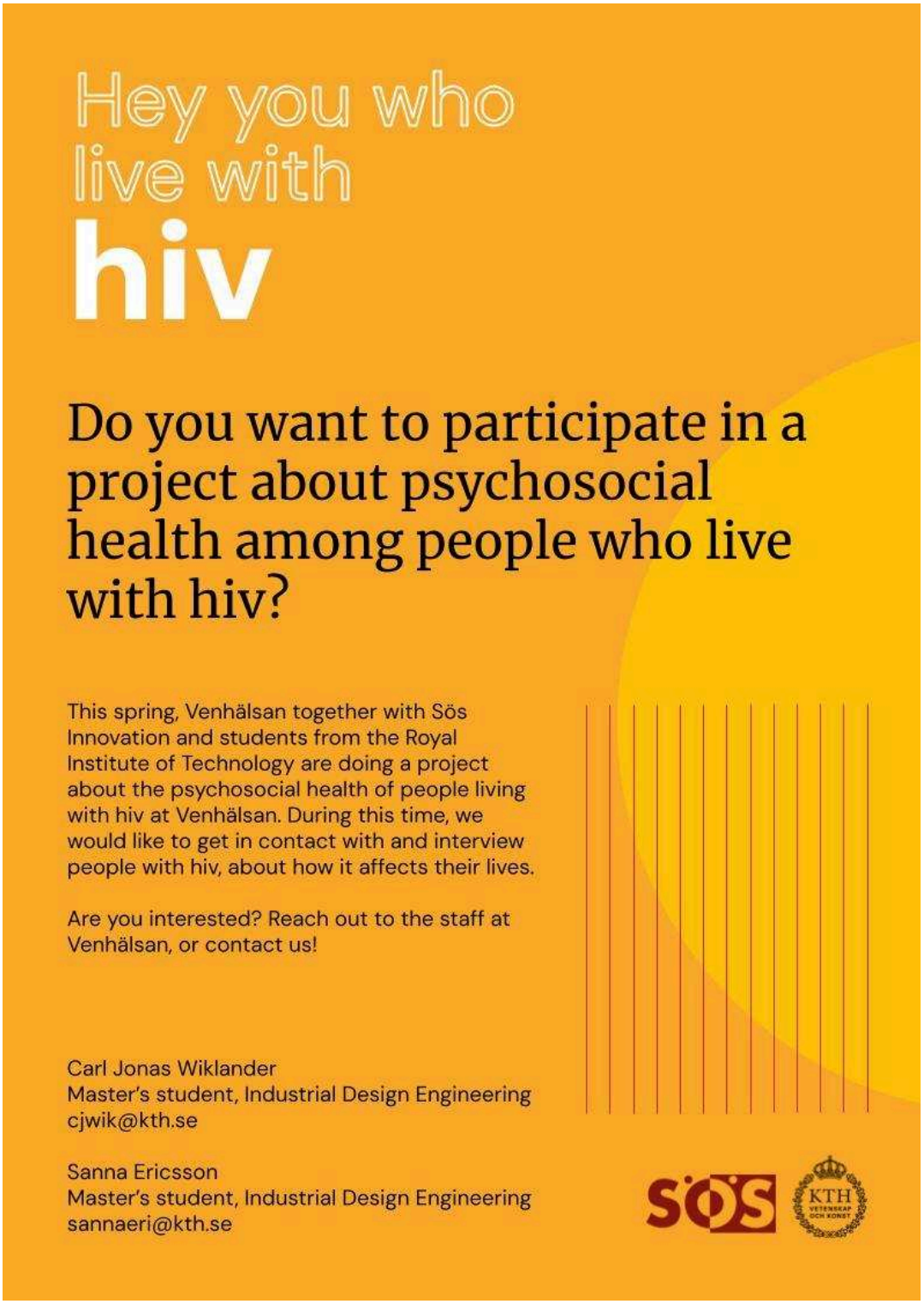


Figure C.0.2. Poster in English.

Appendix D - Cultural Probe

Figure D.0.1 and D.0.2 presents the instructions for the diary.

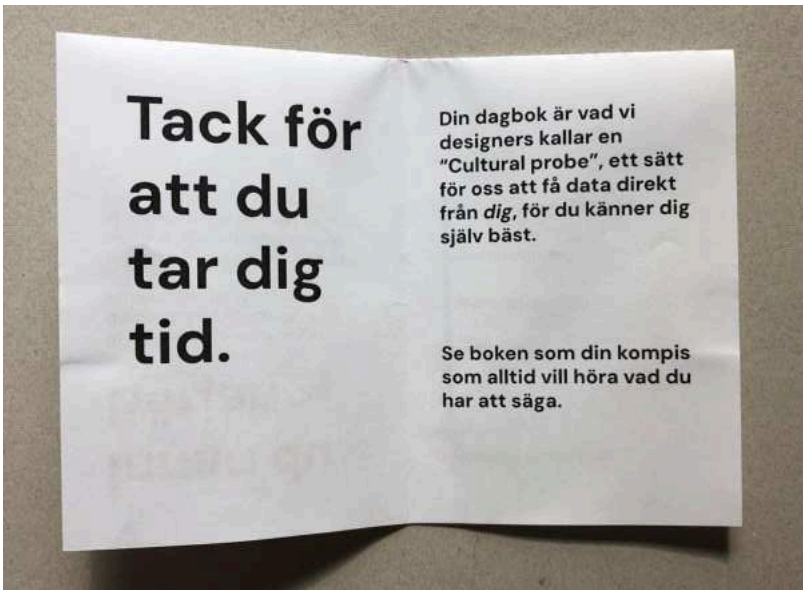


Figure D.0.1. The first pages of instructions.

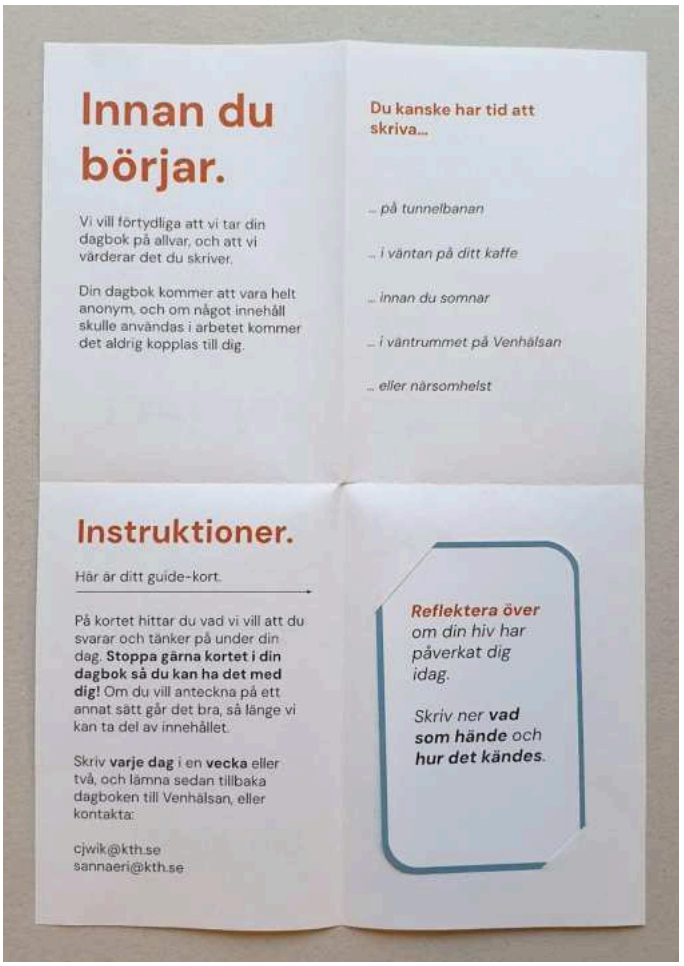


Figure D.0.2. The second pages with instructions..

Appendix E - Why-how ladder

Figure E.0.1 presents the Why-how ladder.

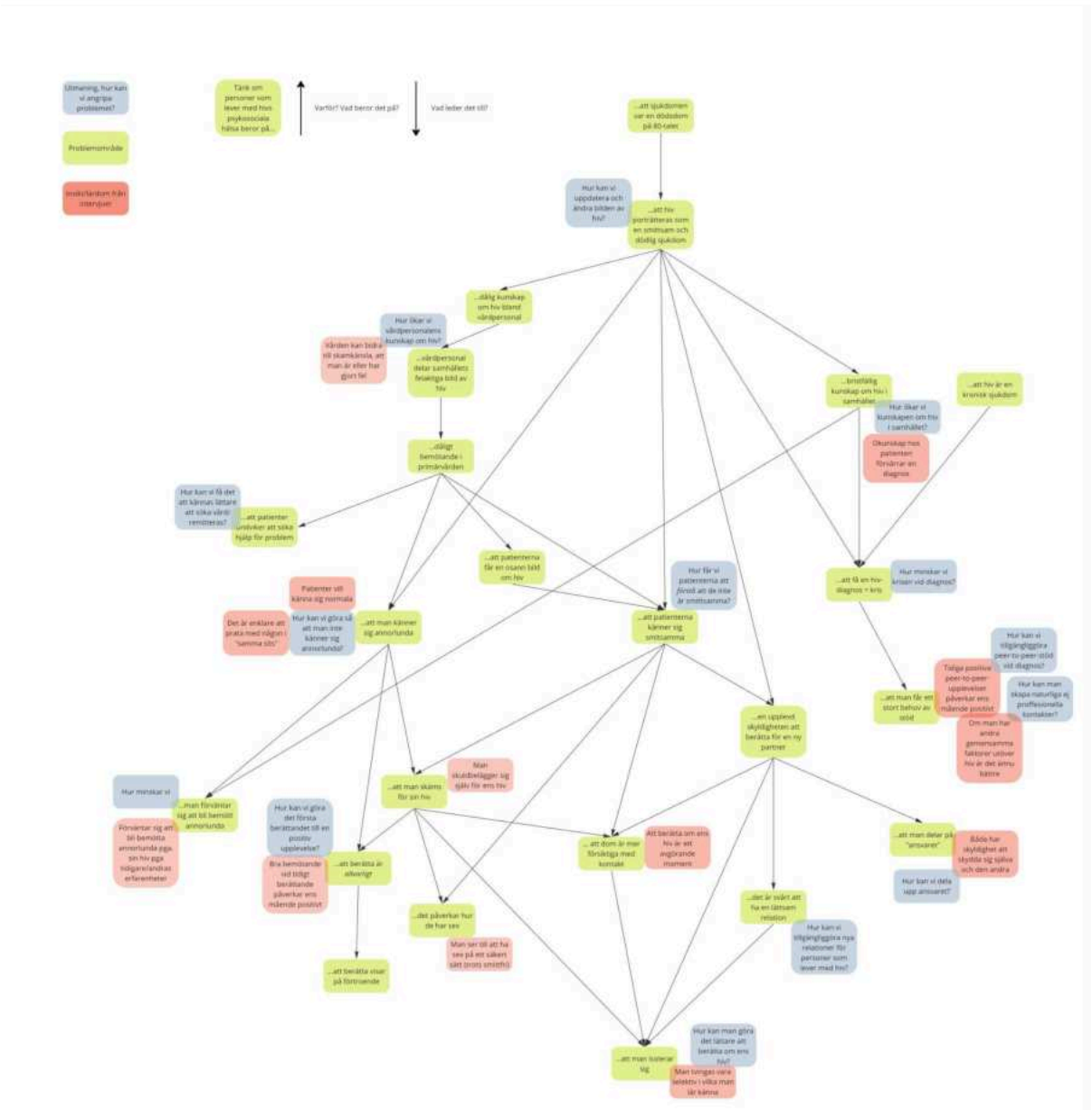


Figure E.0.2. Why-how ladder.

Appendix F - Material to Venhälsan

The following appendix presents the single pages of the material Venhälsan were given.



Detta är en sammanställning av projektet som behandlat frågeställningen:

Hur kan vi öka det psykosociala måendet hos personer som lever med hiv, och som följs på Infektionsmottagning 2 på SöS?

Här presenteras

1. Insikter
2. "Hur kan vi ...?" frågor
3. Ideér

Materialet har framtagits genom en litteraturstudie och intervjuer av personal på Venhälsan, forskare, personer som jobbar på PG och Noaks Ark, närstående till en person som lever med hiv, externa specialister, och självklart personer som lever med hiv. Bland personalen på Venhälsan genomfördes 6 intervjuer med 2 kuratorer, 3 sjuksköterskor och 1 läkare. Intervjuerna varade mellan 45-60 minuter.

Bland patientgruppen intervjuades 8 personer mellan åldrarna 18-75 år. Även dessa intervjuer varade mellan 45-60 minuter, och några genomfördes på plats på Venhälsan, medan andra genomfördes digitalt. Intervjuerna var semi-strukturerade och fokuserade främst på sex olika områden

- Den egna relationen till sin hiv
- Berättande om sin hiv-status
- Hiv i vardagen
- Hiv i samband med relationer till andra människor
- Venhälsan och andra vårdmiljöer
- Stöd

Efter litteraturstudien och alla intervjuer sammanställdes och grupperades all information för att hitta teman och mönster, och det skapade de insikter som kommer presenteras.

Insikter

I följande kapitel presenteras alla insikter som förstudien gett upphov till.

1. Upplevt stigma mot personer som lever med hiv baseras på en bild av sjukdomen från 80/90-talet

Folk associerar hiv med att det på 80-talet var en dödsdom

En hiv-diagnos innebär att folk får föreställningar om att man har en specifik livsstil likt dem som fick sjukdomen på 80-talet

Det saknas empati mot personer som lever med hiv, då det finns en kultur av skuldbeläggning för diagnosen

Det finns en uppfattning om att hiv är smittsamt

Patienter tvingas vara försiktiga och selektiva i vilka de berättar för, för att undvika att bli behandlade annorlunda

"Först och främst vill jag att folk ska veta att det inte är smittsamt. Inte alla är knarkare eller prostituerade. Det räcker med att man träffar någon en gång. Man behöver inte vara en dålig människa för det, leva omoraliskt. Det smittas inte. Jag kan ju inte smitta någon."
- Respondent 2

"Det känns på nåt sätt som man har gjort något som är fel, varför använde jag inte kondom osv"
- Respondent 1

"Var ute med kollegorna som alla visste att jag hade hiv. En råkade dricka ur mitt glas och började gråta. Jag behövde trösta henne och förklara att det inte smittade. Hon jobbar i vården."
- Respondent 2

2. Peer-to-peer stöd är minst lika viktigt som professionellt stöd

Vissa känner sig ensam eller avvikande med sin diagnos

Personer som lever med hiv känner sig mer normala när de träffar andra som lever med hiv

Det är enkelt att prata med någon i samma sits

De som lever med hiv har stor potential att hjälpa andra som lever med hiv

Professionellt stöd är fortfarande vård, man är ej jämlikar

"Det är enklare att prata med andra med hiv. När jag va med i föreningen [PG] pratade vi om allt annat utom det, då kände jag mig normal. Då behövde jag inte tänka så mycket på det."
- Respondent 2

"...kuratorerna blir som en avlönad livskamrat."
- Personal 2

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3. Diagnos nu och på 80-talet ger liknande krisreaktion, trots att sjukdomen är ofarlig vid välinställd behandling

Bilden av hiv på 80-/90-talet hos patienten förvärrar att få en hiv-diagnos

Det pratas inte om hiv i samhället iom. att det inte är "farligt" på samma sätt som förut

Man skuldbelägger för ens hiv (antingen sig själv eller någon annan)

"Maj 1987: Död"
- Respondent 5, diagnos 1987

"Min första fråga: "När dör jag?" Det var så lite jag visste om det."
- Respondent 7, diagnos 2012

"Jag pusslade ihop och märkte att den personen hade hiv. Personen hade inte testat sig på typ ett bra tag. Det gjorde mig bitter."
- Respondent 7

"Fick ingen information om hiv, kände mig dum. Jag var inte tillräckligt försiktig."
- Respondent 6, diagnos 2018

4. Efter diagnos är man sårbar och extra mottaglig för intryck

Att få en diagnos skapar ofta en krisreaktion

Det finns ofta starka negativa känslor kopplade till att få en hiv-diagnos

Extra känslig för dåligt bemötande nära diagnos

Bra bemötande vid tidigt berättande påverkar ens mående positivt

"I början [av diagnos] var det jobbigt, kände skam, värdelös, misslyckad."
- Respondent 1

"Jag är glad att det [dåligt bemötande i vården] inte hände direkt efter min diagnos för det hade nog knäckt mig."
- Respondent 7

"Då hade jag 30 stycken. Då började jag ta tag i att överleva igen."
- Respondent 5

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5. Intima relationer ändras när personer som lever med hiv berättar om sin diagnos

Att dölja sin hiv diagnos är en stor börda att hålla på

Personer som lever med hiv är rädda för andras okunskap om hiv

Det är svårt att komma någon nära utan att berätta om sin hiv

Att berätta om ens hiv är ett tecken på förtroende

Att berätta om sin hiv är ett avgörande moment i en relation

"Hiv har påverkat hur jag dejtar. Det ligger i bakhuvudet hela tiden, vi är liksom nykära men romansen blir verklighet när jag berättar."
- Respondent 3

"Minns att det blev en diskussion när jag berättade, han visste inte riktigt. "Jag behöver fundera". Det var tufft att höra."
- Respondent 2

"Mår bra kroppsligt men om jag träffar nån känner jag "oj nu måste jag berätta""
- Respondent 2

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6. Personer som lever med hiv förväntar sig att bli bemötta annorlunda pga sin hiv baserat på tidigare/andras erfarenheter

Personer som lever med hiv antar att andra har en felaktig, utdaterad bild av hiv och vem som får det

Patienter tvingas vara försiktiga och selektiva i vilka de berättar för, för att undvika att bli behandlade annorlunda

Ett dåligt bemötande väger tyngre än ett bra bemötande

"Påverkar hur jag interagerar med andra. Är restriktiv med att släppa in folk. Vill inte bli utsatt för fördomar. Väljer bort människor som jag inte hade valt bort om jag inte haft hiv."
- Respondent 2

"Finns alltid en bedömning som jag är tvungen att göra i alla fall i nya sammanhang. En rädsla för att bli bedömd."
- Respondent 1

"Det internaliserade [stigmat] finns där, även om jag inte blivit utsatt för nåt dåligt själv, så vet jag att det finns så många berättelser om andra som blivit utsatta och då internaliserar jag andras berättelser och rädsla"
- Respondent 1

7. Personer som lever med hiv förväntar sig att bli bemötta annorlunda *i vården* pga sin hiv baserat på tidigare/andras erfarenheter

Dåligt bemötande i vården hänger ofta ihop med felaktig bild av hiv från 80/90-talet

Vården kan bidra till skamkänsla, att man är eller har gjort fel (dom frågar hur man fick det ex.)

Man är redan utsatt i en vårdsituation, extra känslig för intryck

"Åkte dit, de hade rymddräkter. Hon slarvade, och så stog det en skräckslagen tandsköterska med en svart sopsäck och så slängde de alla instrumenten som de hade använt på mig och så sa hon "Vi har tyvärr inga fler tider för dig, du får söka en annan tandläkare"."
- Respondent 2

"När jag går på små VC känns det som om de är mindre kunniga överlag. De försöker dra allt som händer med mig till min hiv."
- Respondent 7

"Jag upplever att det har blivit bättre, men de [personer som lever med hiv] kan fortfarande få frågan typ "hur har du fått din hiv?" som bara de som inte jobbar med hiv frågar."
- Personal 5

8. Patienter förstår att de inte smittar men är rädda att smitta

Det finns en uppfattning om att hiv är smittsamt

Personer som lever med hiv agerar ibland som om de vore smittsamma

Personer som lever med hiv känner sig ibland tvungna att berätta om sin hiv

Man informerar för att avsäga sig lite av ansvaret vid eventuell smitta

Hiv påverkar hur man har sex trots att det inte smittar

"I början var det otroligt svårt. Det var nästan det bästa preventivmedlet jag fått. [avstod från sex]"
- Respondent 7

"Det är i det sexuella som det har varit och är ett problem. Jag har sex på ett sätt så att jag inte för över. Viktigt att inte föra över."
- Respondent 5

"Jag var rädd att min dotter inte skulle vilja att jag höll i barnbarnet."
- Respondent 1

9. "Överlevare" har andra behov och svårigheter än personer som får diagnos idag

Spår från diagnos på 80/90-talet påverkar dem fortfarande "praktiskt" idag

Hiv adderar en oro om fysisk hälsa hos personer som är äldre

"Jag ska ändå dö 95/96". Sa upp mig."
- Respondent 4

"Helt plötsligt skulle jag fortsätta jobba osv. Minst lika jobbigt som att få diagnos, men enklare att ta sig ur."
- Respondent 5

"[Hiv] Påverkar livet i hur jag tänker om pengar, morgondagen, sparande. Jag ska inte pensionsspara/ spara till lägenhet, blir något leva för dagen-tänk. Påverkar konsekvenstänket"
- Respondent 3

"Börjar bli äldre o se andra hiv positiva råka ut för saker [medicinska åkommor]"
- Respondent 1

"Vissa läkemedel har olika biverkningar som inte går att undgå och jag känner till det och måste samleva med det"
- Respondent 4

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10. Hiv är en stor sak både för de som får det, men även deras närstående

Att berätta om ens hiv är ett tecken på förtroende

Som närstående vill man bli litad på

Bra kunskap gör att berättandet inte blir så dramatiskt

"Man har sin valda familj."
- Respondent 2

"Jag berättade för min syster och då blev hon sur för att en annan kompis fått reda på det innan henne [om att berätta om hiv]"
- Respondent 5

"Jag var ganska insatt i det, blev inte så mycket dramatik över det.
- Närstående till Respondent 5

11. Venhälsan erbjuder ett större stöd utöver det medicinska för personer som lever med hiv jämfört med övriga vården

Venhälsan är en trygg plats för personer som lever med hiv

Patienterna vet att Venhälsan har mycket (rätt) kunskap om hiv och inte dömer

I en vårdsituation befinner sig patienter i en utsatt position

Vården kan skuldbelägga för ens diagnos

"Kände mig inte lika trygg, de va inte otrygga, men jag känner mig mer trygg på Venhälsan. Iom läggning osv. Det känns lättare."
- Respondent 2

"Kändes bra för dom är vana, de har varit med om det förr. "
- Respondent 7

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Hur kan vi?

Följande kapitel presenterar "Hur kan vi ...?" frågor som låg till grund för projektets idegenerering.

1. Hur kan vi underlätta att berätta om sin hiv diagnos?
2. Hur kan vi göra att få en hiv diagnos mindre dramatiskt för patienten?
3. Hur kan vi stötta patienter under perioden efter en diagnos?
4. Hur kan vi uppdatera bilden av hiv i samhället?
5. Hur kan Venhälsan tillgängliggöra peer-to-peer stöd för patienterna?
6. Hur kan vi få den övriga vården att kännas mer som Venhälsan?
7. Hur kan vi få det att kännas mindre ensamt att ha hiv?
8. Hur kan vi göra så att hiv inte påverkar ens sexliv när en lever med hiv?
9. Hur utökar vi patientens trygga cirkel? (personer som patienter har tillit till/har berättat för/vågar berätta för)

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Ideér

Detta kapitel presenterar alla ideér som skapades, både av personalen på workshopen på Venhälsan och av projektgruppen, som lösningar på "Hur kan vi ...?" frågorna.

1. Hur kan vi underlätta för att berätta om sin hiv diagnos?

- Personen som lever med hiv gör en övning/aktivitet som leder till berättande
  - Spelar ett spel
  - Läser en bok
- Personen som lever med hiv skickar något som berättar:
  - Ett kort
  - En broschyr
  - Länkar till en hemsida som berättar
- Venhälsan hjälper till med berättande
  - Kurator berättar för närstående
  - Person som ska diagnostiseras tar med närstående vid diagnos
  - Venhälsan erbjuder informationssamtal med närstående och läkare
  - Venhälsan ger person som får diagnos ett informationskit som hen kan ge vidare till närstående
  - Venhälsan bjuder in närstående
- Berättandemall, steg-för-steg
- Närståendefolder med information som personen som fått diagnos ger till närstående

2. Hur kan vi göra att få en hiv diagnos mindre dramatiskt för patienten?

- Personen får ta del av information innan diagnos (på plats eller hemma)
  - Bok/Broschyr som förklarar om hiv
  - Informativ film
  - Informativt spel
- Venhälsan hjälper till att förmildra yttre omständigheter i samband med diagnos
  - Diagnos ges endast på eftermiddag så patienten är ledig efter
  - Venhälsan hjälper till att se till att man får ledigt från jobb en tid efteråt, läkarintyg
  - Stöddjur
- Venhälsan förmildrar omständigheterna på plats vid diagnos
  - SSK/Läkare/Kurator i samma rum, visar direkt att Venhälsan bidrar med ett brett stöd
  - Extra långt vårdbesök
  - Obligatoriskt att ta med någon vid diagnos

3. Hur kan vi stötta patienter under perioden efter en diagnos?

- Avdramatisera hiv genom att hjälpa till att bygga upp förståelse för sjukdomen
  - Quiz man svarar på varje dag/vecka med viktiga frågor om hiv
  - Muntlig och skriftlig information från Venhälsan
- Strukturera upp fritiden
  - Program med läxor som bygger upp förståelse för hiv
- Ge patient lättillgänglig information
  - Bok/Broschyr med information patient kan läsa hemma
- **Digital lösning (app)**
  - Direktkontakt med Venhälsan (chatt eller dyl.)
  - Patient kan följa sin virus-/immunförsvarskurva i mobilen
  - 1177-chatt
- Venhälsan erbjuder peer-to-peer stöd
  - Aktivitetsgrupper
  - Samtalsgrupper
  - Patient får en mentor vid diagnos (som lever med hiv)
  - Pusha att ta kontakt med patientorganisation
- **Fysiskt paket**
  - Diskret dosett
  - Känslodagbok
  - Informationstext, "Såhär kan man göra med..."
- Tillgängliga FAQ (vanliga frågor). "Såhär går det till vid ny diagnos"
- Formaterade informationsbesök för nydiagnostiserade och närstående
- Jobba utvärderande med patienter: "Var det någonting du saknade i början?"
- Brevlåda i receptionen för "Dumma frågor"

4. Hur kan vi uppdatera bilden av hiv i samhället?

- **Informationscenter**
  - Hiv-museum
  - Monter på stan med lättillgänglig och attraktiv information
  - Monter vid Sös entré
- Informationsfilmer som skickas till olika aktörer
  - Vården
  - Skolor
  - Arbetsplatser
- Informationskapmanjer
  - TikTok-konto
  - Reklam i offentligheten
- Belysningskampanjer
  - "Rosa bandet"-liknande
  - Samarbeten med Konstnärer
- Hiv-kurs för olika aktörer
  - Vårdavdelningar
  - Skolor
  - Arbetsplatser där någon med hiv jobbar

5. Hur kan Venhälsan tillgängliggöra peer-to-peer stöd för patienterna?

- Nya typer av vårdmöten på Venhälsan
  - Boka "kompismöte" (träffa någon annan med hiv)
  - Dubbelkuratorssamtal, dvs. kurator träffar två patienter samtidigt
- Venhälsan sammanför personer som lever med hiv
  - Möte varje månad med de som blivit diagnostiserade
  - Samtalsgrupper
  - Blind dates
  - Brevlåda med brevkompisar
  - Vänskapstinder
- Använda gemensamma ytor på Venhälsan för att föra ihop folk
  - Spel i väntrummet
- Erbjuda kontaktpersoner
  - Mentorsprogram internt på Venhälsan
  - Fadder
  - Kontaktperson Patientorg.
- Aktiviteter (inspiration)
  - "Kropp- och själhelg"
  - Heart to heart
  - Ungdomsläger

6. Hur kan vi få den övriga vården att kännas mer som Venhälsan?

- **Venhälsan certifierar vårdcentraler och avdelningar** så att personer som blir remitterade från Venhälsan eller söker vård någon annanstans kan känna en liknande trygghet på den certifierade vårdplatsen.
  - Avdelningar
  - Vårdcentraler
  - Representanter på VC/Avdelning
- Venhälsan föreläser och utbildar andra vårdavdelningar
- Man kan rapportera dåligt bemötande i vården till Venhälsan
  - Venhälsan följer upp och kontaktar vårdpersonalen i fråga
- FAQ om hiv för vårdpersonal, sprid information om sjukdomen inom vården
- Sös personal får äta lunch med personal på Venhälsan
- Blockera blodmittemarkering vid remittering. Ta bort och se till att den inte kommer tillbaka
- Skriva i remisser att Venhälsan gärna svarar på frågor
- Venhälsan kan agera medlare mellan patient och övriga vården

7. Hur kan vi får det att kännas mindre ensamt att ha hiv?

- Venhälsan hjälper till att involvera närstående
  - Ta med närstående på vårdbesök
  - Lista en stödperson i samband med diagnos
- Låta patienter ta del av andra som lever med hivs liv
  - Bok med berättelser om hur det är att leva med hiv
  - "Porträtt" av personer som lever med hiv, beskrivningar, historier
  - Nyhetsbrev med berättelser (digitalt/fysiskt)
- Venhälsan kan hjälpa till med att hitta peers (se fråga 5)
- Venhälsan kan erbjuda ventilation
  - Anonyma chattar

8. Hur kan vi göra så att hiv inte påverkar ens sexliv när en lever med hiv?

- Jobba med olika informationsspridningskampanjer
  - "Sex med en person som har en välinställd behandling är det säkraste sexet"
- Förtydliga för patienter att de ej är smittsamma vid välinställd behandling
  - Digital lösning
    - Tillgängliggöra viruskurvan så de kan se att de ej är smittsamma
    - "Dummar frågor" om smittsamhet
    - "Smittfribevis"

9. Hur utökar vi patientens trygga cirkel (personer som patienter har tillit till/har berättat för/vågar berätta för)?

- Hiv-kurs för olika aktörer
  - Skolor där någon lever med hiv
  - Vården
  - Arbetsplatser där någon lever med hiv
- Anonym informationsspridning: "Någon på din arbetsplats lever med hiv, detta kan du tänka på..."
- Underlätta berättande (se fråga 1)
- Förhållningsguide till patienter: "Förvänta dig detta av en kollega"

Appendix G - Used original quotes

All quotes used were gathered from the interview stages, whether they were spoken or written in the customer journey map.

Original quote	**Used translation**
"Först och främst vill jag att folk ska veta att det inte är smittsamt. Inte alla är knarkare eller prostituerade. Det räcker med att man träffar någon en gång. Man behöver inte vara en dålig människa för det, leva omoraliskt. Det smittas inte. Jag kan ju inte smitta någon."	*"First and foremost I want people to know that it is not contagious. Not everyone is a drug addict or prostitute. It is enough to meet someone once. You don't have to be a bad person for that, that you live immorally* [not that prostitutes or drug addicts are bad people or that they are immoral]. *I can't infect someone."* - Respondent 13, man aged 47
"Va ute med kollegor. Alla visste att jag hade hiv. En råkade dricka ur mitt glas. Jag behövde trösta för att hon satt och grät. Förklarade att det inte smittade. Hon jobbade ändå inom vården. Borde ändå veta om smittor."	*"I was out with my colleagues. Everyone knew that I had hiv. One colleague accidentally drank from my glass, and started crying. I had to comfort her and explain that it would not infect her. She worked in health care, so she should have known that."* - Respondent 13, man aged 47
"Det känns på nåt sätt som man har gjort något som är fel, varför använde jag inte kondom osv."	*"In some way it feels as if I have done something wrong, why did I not use a condom etc."* - Respondent 12, man aged 68
"Många uttrycker hur viktiga mötena är och att få träffa andra, vissa i gruppen är inte öppna så för dem är det kanske extra viktigt att få mötas."	*"Many express how important these meetings are, and to meet others. Some might not be so open so for them it is extra important to meet."* - Respondent 12, man aged 68
[Interview was held in English] *"Every time I come to NA and have meetings, people talk about different experiences, and then I relate a lot. Many think the same. It makes me feel better."*	*"Every time I come to NA and have meetings, people talk about different experiences, and then I relate a lot. Many think the same. It makes me feel better."* - Respondent 17, man aged 24
"...kuratorerna blir som en avlönad livskamrat."	*"We counselors become paid life partner."* - Respondent 2, counselor at Venhälsan

"Maj 1987: Död."	*"May 1987: Dead."* - Respondent 16, man aged 74
"Min första fråga: "När dör jag?" Det var så lite jag visste om det."	*"My first question was: when do I die? That was how little I knew about the disease… "*- Respondent 18, diagnosed in 2012, man aged 40
[Interview held in english] *"I thought that I was going to die."*	*"I thought that I was going to die."* - Respondent 17, diagnosed in 2018, man aged 24
"Jag pusslade ihop och märkte att den personen hade hiv. Personen hade inte testat sig på typ ett bra tag. Det gjorde mig bitter."	*"I pieced it together and noticed that person had it. He had not tested himself for a while. It made me bitter."* - Respondent 18, man aged 40
"I början var det jobbigt, kände skam, värdelös, misslyckad."	*"In the beginning it was tough, I felt a lot of shame, I felt useless, like a failure."* - Respondent 12, man aged 68
"Jag är glad att det inte hände direkt efter min diagnos för det hade nog knäckt mig."	*"I am happy it did not happen right after my diagnosis, because it probably would have broken me."* - Respondent 18, man aged 40
"Då hade jag 30 stycken. Då började jag ta tag i att överleva igen."	*"All of a sudden I had 30 people supporting me. That was when i started to survive."* - Respondent 16, man aged 74
"*I början var det otroligt svårt. Det var nästan det bästa preventivmedlet jag fått."*	"*In the beginning it was incredibly hard. It was the best contraceptive you could have."* - Respondent 18, man aged 40
"Om de blir för nära mig måste jag berätta. Har varit öppen tidigare. Om jag träffar en ny partner vill jag säga det, självklart. Om jag känt nån i tre år måste jag berätta det."	*"If they become too close to me, I have to tell. If I meet a new partner I want to say it, of course. If I have known someone for three years I have to tell them."* - Respondent 13, man aged 47
"Hiv har påverkat hur jag dejtar. Det ligger i bakhuvudet hela tiden, vi är liksom nykära men romansen blir verklighet när jag berättar."	*"Hiv has affected how I date, as it constantly lies in the back of my head. We can be newly in love,* ***but the romance passes and quickly becomes reality*** [when disclosing about their hiv-status]*."* - Respondent 14, man aged 50

"Påverkar hur jag interagerar med andra. Är restriktiv med att släppa in folk. Vill inte bli utsatt för fördomar. Väljer bort människor som jag inte hade valt bort om jag inte haft hiv."	"[Hiv] *affects how I interact with others. I am restrictive with letting people in. Don't want to be exposed to prejudices. I opt out of people that I would not have opted out of if I didn't have hiv."* - Respondent 13, man aged 47
"Finns alltid en bedömning som jag är tvungen att göra i alla fall i nya sammanhang. En rädsla för att bli bedömd."	*"There is always an assessment* [regarding disclosure] *that I have to make, at least in new contexts. A fear of being judged."* - Respondent 12, man aged 68
"Det internaliserade finns där, även om jag inte blivit utsatt för nåt dåligt själv, så vet jag att det finns så många berättelser om andra som blivit utsatta och då internaliserar jag andras berättelser och rädsla."	*"The internalized* [stigma] *is there, even if I have not been exposed to something bad I know it happens and there are so many stories from other people that have been exposed and then I internalize their stories and fear."* - Respondent 12, man aged 68
"Åkte dit, de hade rymddräkter. Hon slarvade, och så stog det en skräckslagen tandsköterska med en svart sopsäck och så slängde de alla instrumenten som de hade använt på mig och så sa hon "Vi har tyvärr inga fler tider för dig, du får söka en annan tandläkare."	*"Went there* [to the dentist] *and they had on space-suits. She* [the dentist] *did a sloppy job and there was a terrified nurse next to me with a black garbage bag that they threw all the instruments in that they had used on me. Then she told me they did not have any more appointments for me and that I had to look for another dentist."* - Respondent 13, man aged 47
"Jag berättar om min hiv och ambulanspersonal säger att han inte vill ge sprutan i venen för att det blir så blodigt om de ger smärtstillande i venen. Det är inte så bra för skamkänslan, man känner sig konstig och annorlunda."	*"... and I told the paramedics about my hiv and then I hear one of them tell the other that they would give me the shot intramuscularly because it would be so bloody if I got it in my vein."* - Respondent 12, man aged 68
[Interview held in english] *"Hiv is not present in my everyday life, but I think about it when I take a blood test, I tell the nurse."*	*"Hiv is not present in my everyday life, but I think about it when I take a blood test, I tell the nurse."* - Respondent 19, woman aged 39
"Jag upplever att det har blivit bättre, men de kan fortfarande få frågan typ "hur har du fått din hiv?" som bara de som inte jobbar med hiv frågar."	*"In my experience it has become better, but they* [people living with hiv] *still get the question "how did you get your hiv?" that*

	only people not working with hiv asks." - Respondent 5, nurse at Venhälsan
"När jag går på små VC känns det som om de är mindre kunniga överlag. De försöker dra allt som händer med mig till min hiv."	*"When I go to smaller health care centers (vårdcentral) it feels as if they are less knowledgeable. They try to connect everything that happens with me to my hiv."* - Respondent 18, man aged 40
[Interview held in english] *"You have yourself to blame because you have chosen to live this life."*	*"You have yourself to blame* [for your hiv] *because you have chosen to live this life* [being gay]*."* - a non-swedish doctor to respondent 15
"I början var det otroligt svårt. Det var nästan det bästa preventivmedlet jag fått."	*"It* [being intimate] *was very hard in the beginning. It was almost the best contraceptive I have ever gotten."* - Respondent 18, man aged 40
"Det är i det sexuella som det har varit och är ett problem. Jag har sex på ett sätt så att jag inte för över. Viktigt att inte föra över."	*"It is in the sexual regards it* [hiv] *has been and is a problem. I have sex in a way so that I can not transmit. It is important to not transmit."* - Respondent 16, man aged 74
"Jag ska ändå dö 95/96". Sa upp mig."	*"I´m going to die 95/96 anyway. I quit* [my job]*."* - Respondent 16, man aged 74
"Påverkar livet i hur jag tänker om pengar, morgondagen, sparande. Jag ska inte pensionsspara/ spara till lägenhet, blir något leva för dagen-tänk. Påverkar konsekvenstänket."	*"It affects how you think about money, tomorrow, savings,* [you] *should not save for your pension, or an apartment, you live for the day."* - Respondent 14, man aged 50
"Börjar bli äldre o se andra hiv positiva råka ut för saker."	*"I'm getting older and I see other hiv-positive end up with things* [illnesses]*."* - Respondent 12, man aged 68
[Interview held in english] *"Some drugs have different side effects that you can't escape and I know that and I have to live with that."*	*"Some drugs have different side effects that you can't escape and I know that and I have to live with that."* - Respondent 15, man aged 45
"Man har sin valda familj."	*"You have your chosen family."* - Respondent 13, man aged 47

"Jag berättade för min syster och då blev hon sur för att en annan kompis fått reda på det innan henne."	*"I told my sister and she was mad because I had told another friend before I told her."* - Respondent 16, man aged 74
"Jag var ganska insatt i det, blev inte så mycket dramatik över det."	*"I was pretty knowledgeable* [about hiv]*, so it wasn't very dramatic*." - Respondent 20, man aged 57, partner to someone living with hiv
"Kände mig inte lika trygg, de var inte otrygga, men jag känner mig mer trygg på Venhälsan. Iom läggning osv. Det känns lättare."	*"I didn't feel as safe, they weren't unsafe, but I feel more safe at Venhälsa, considering my sexuality. It just feels easier."* - Respondent 13, man aged 47
"Kändes bra för dom är vana, de har varit med om det förr."	*"It feels good* [going to Venhälsan] *because they are used to it, they have seen it before."* - Respondent 18, man aged 40

Appendix H - Material from Workshop with Venhälsan

The following Appendix presents the material that Venhälsan created at the workshop, both the front pages and the ideas for the different “How might we …?” questions.

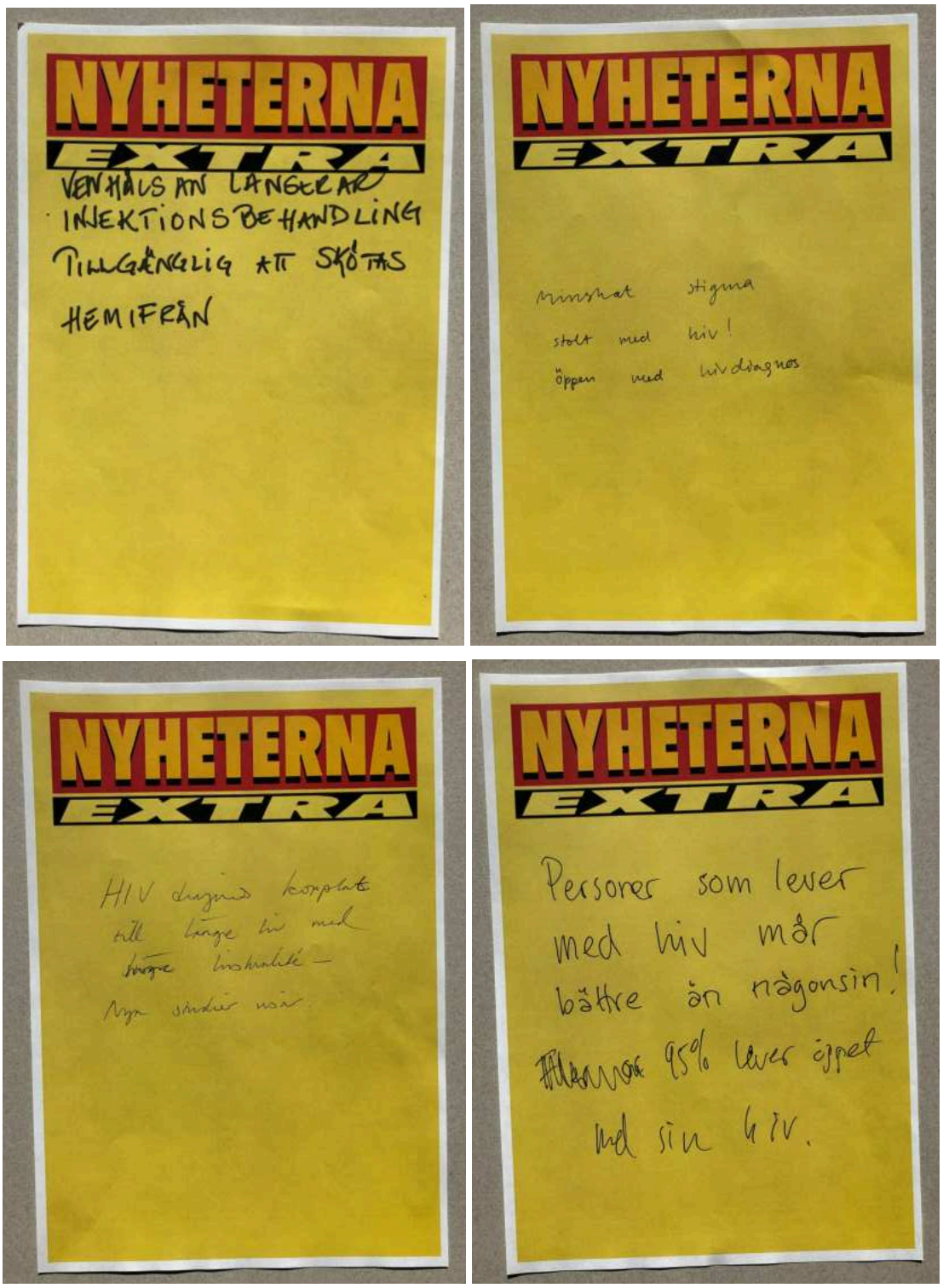


Figure H.0.1:4: Results from front-page-assignment.

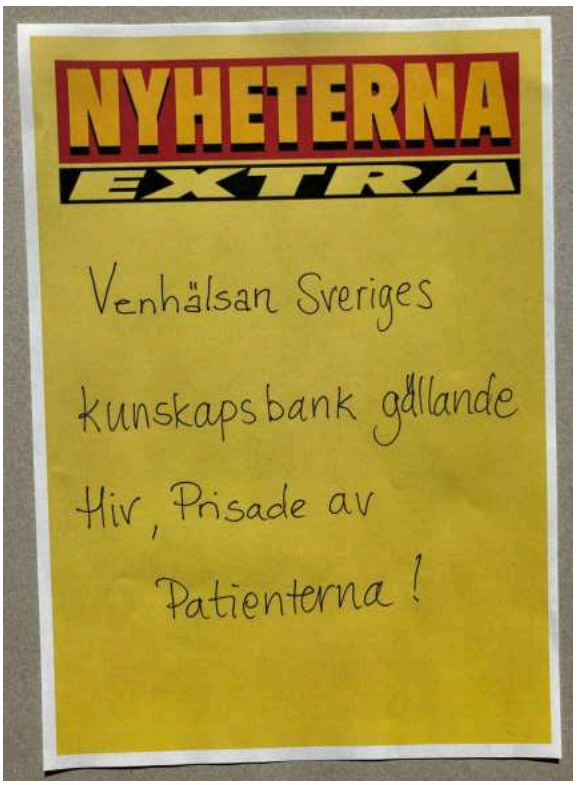


Figure H.0.5: Results from front-page-assignment.

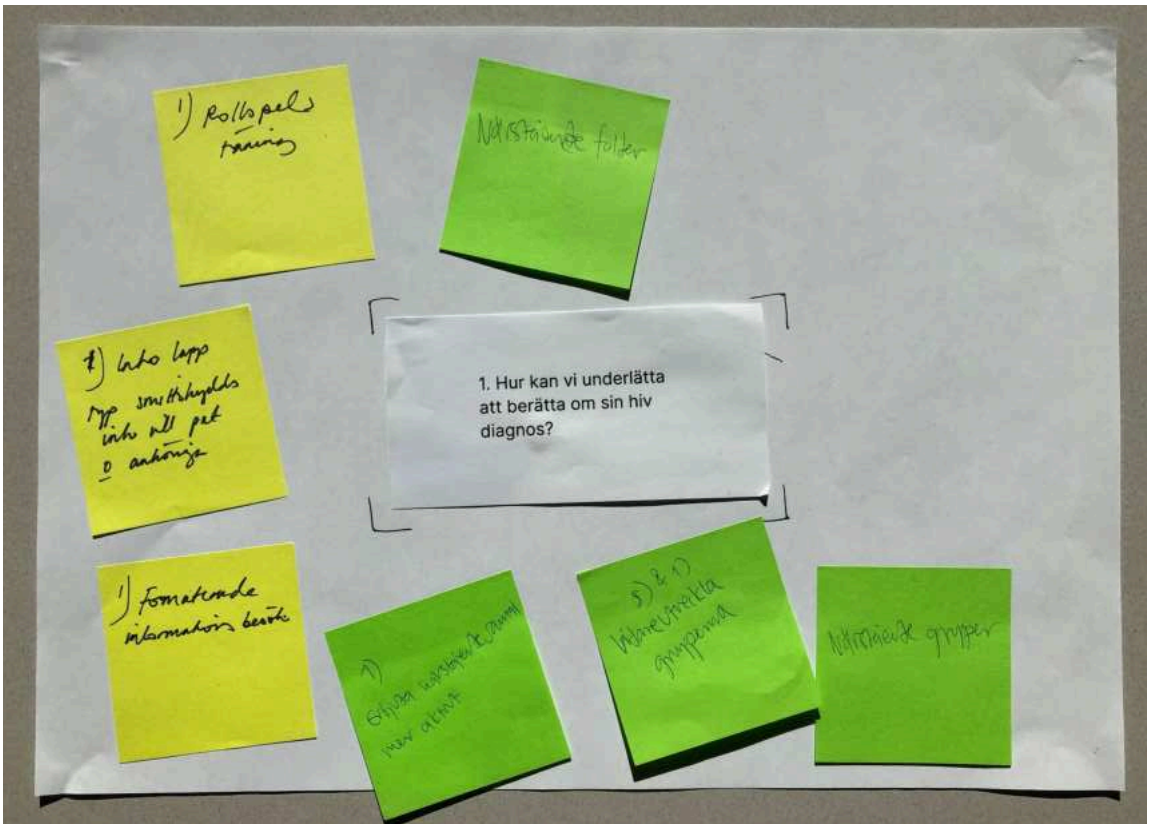


Figure H.0.6. The ideas for the question “How might we facilitate disclosing your hiv status?”.

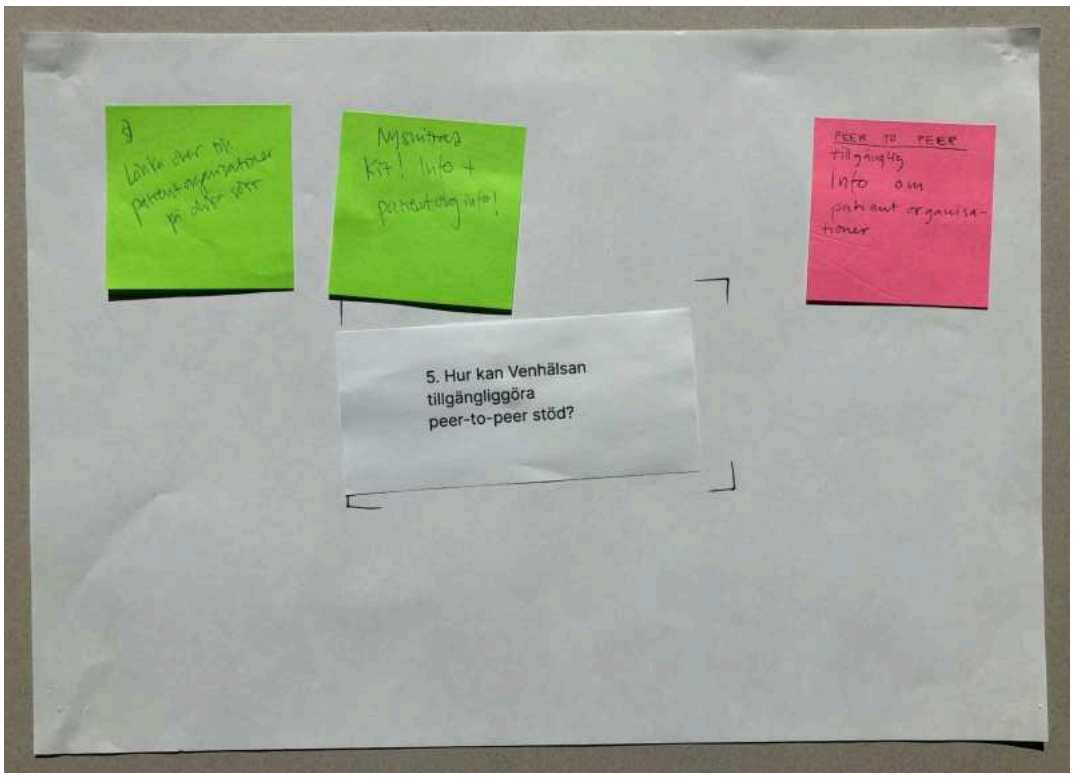


Figure H.0.7. The ideas for the question “How might Venhälsan make peer-to-peer support more available?”.

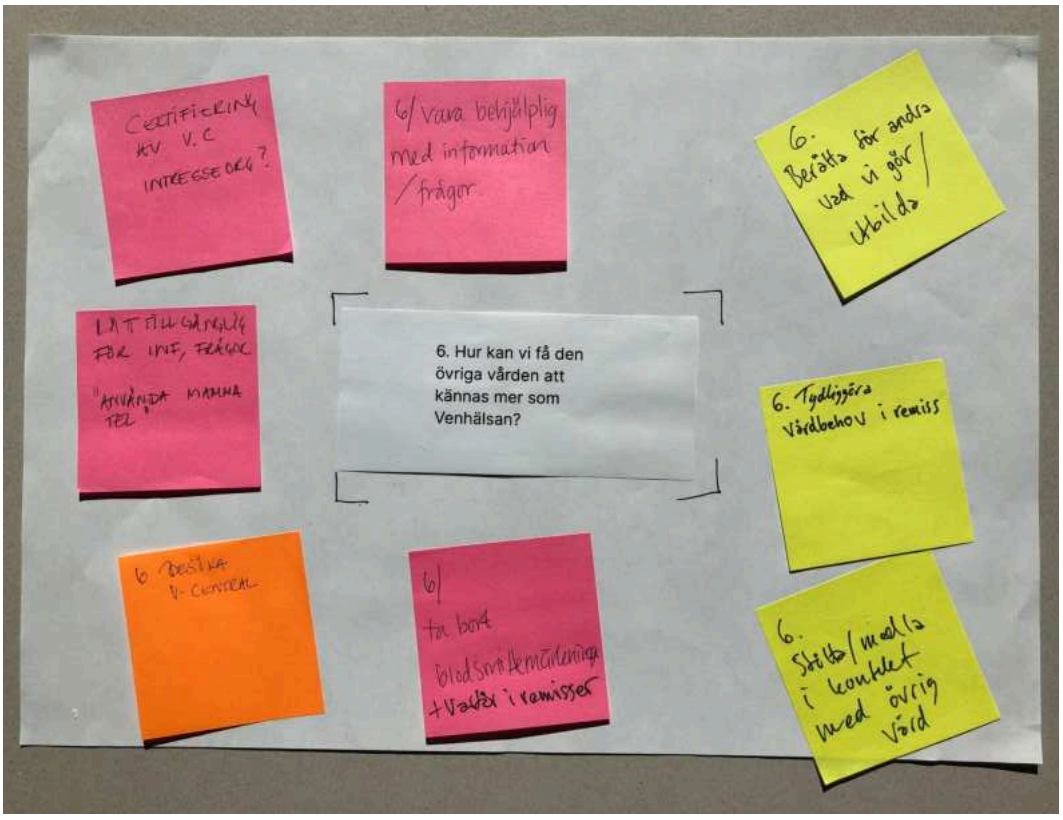


Figure H.0.8. The ideas for the question “How might we make other health care clinics feel more like Venhälsan?”.

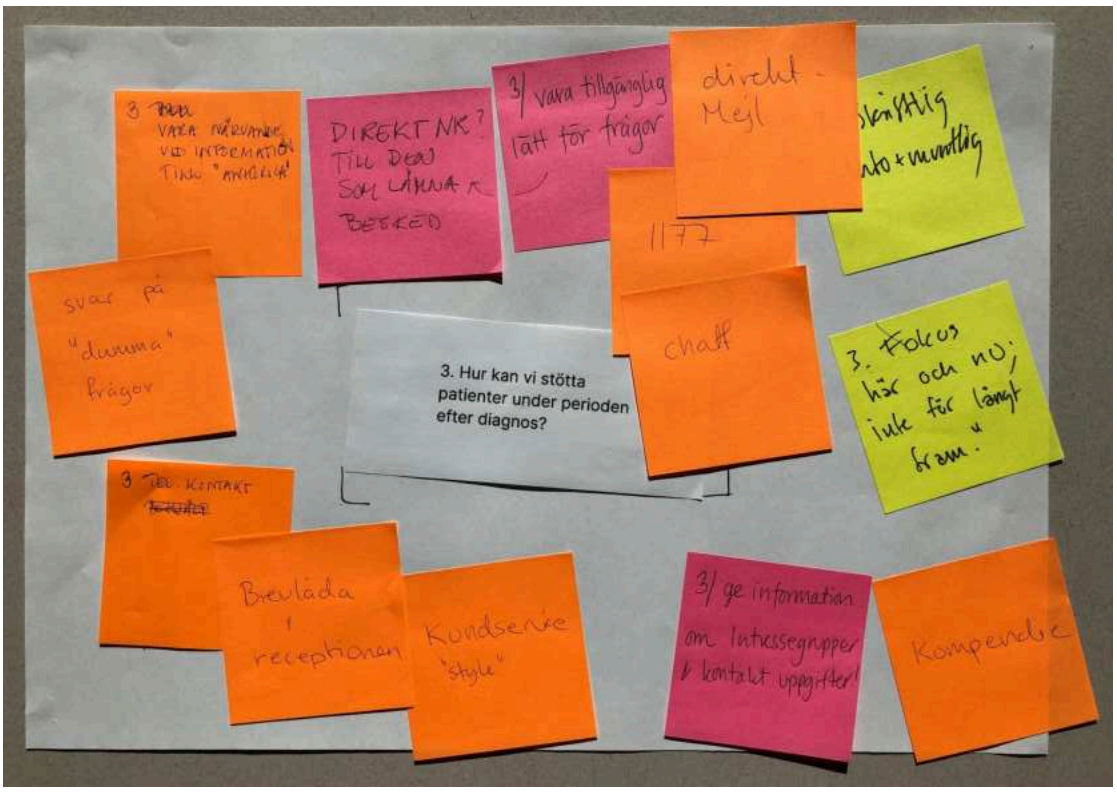


Figure H.0.9. The ideas for the question “How might we support patients during the time after their diagnosis?”.

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